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Note: The report is excerpted and as an exhibit here includes pages 1-14 (Introduction and contents), 203-13 (mask mandates and CDC authority), 276-80 (test supplies and accuracy), 296-99 (Biden Administration vaccine claims), 309-71 (vaccine mandates).

December 4, 2024



SELECT SUBCOMMITTEE ON THE
CORONAVIRUS PANDEMIC
— CHAIRMAN BRAD WENSTRUP —

**AFTER ACTION REVIEW OF THE COVID-19 PANDEMIC:
The Lessons Learned and a Path Forward**



Final Report of the

Select Subcommittee on the Coronavirus Pandemic
Committee on Oversight and Accountability

U.S. House of Representatives
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Congress of the United States

House of Representatives

SELECT SUBCOMMITTEE ON THE CORONAVIRUS PANDEMIC

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Dear Colleague:

It has been my distinct pleasure to lead the Select Subcommittee on the Coronavirus Pandemic for the 118th Congress. I was honored to be entrusted with a great responsibility: to investigate a once in 100-year pandemic and to prepare America for next time—and there will be a next time. This is a responsibility I took very seriously, and I believe that seriousness and teamwork has translated to much success.

Five years ago, on December 1, 2019, was what would eventually be the first confirmed case of COVID-19. After that, a pandemic devastated the world at nearly never before seen proportions, leaving millions dead and millions more concerned about long-term consequences.

COVID-19 was novel. The brightest scientists and medical experts were learning on the job to determine how to treat both the underlying disease and the second order side effects.

Since February 2023, the Select Subcommittee sought to produce a full after-action report to provide a road map of how we, in Congress, the Executive, and the private sector may better prepare for and respond to future pandemics. Throughout this process, the Select Subcommittee sent more than 100 investigative letters, conducted 38 transcribed interviews or depositions, held 25 hearings or meetings, and reviewed more than one million pages of documents from dozens of custodians. This work looks back on many events, comments, guidances, and other actions, to look forward. This is the single most thorough review of the pandemic conducted to date.

Most of you know me. You know I strive to work collegially, with our fellow Americans, to provide results for all of us. That is the same mentality I brought to my work as Chairman of the Select Subcommittee. During a time of intense partisanship, the Select Subcommittee had bipartisan consensus across multiple topics.

- 1) The possibility that COVID-19 emerged because of a laboratory or research related accident is not a conspiracy theory.
- 2) EcoHealth Alliance, Inc. and Dr. Peter Daszak should never again receive U.S. taxpayer dollars.
- 3) Scientific messaging must be clear and concise, backed by evidentiary support, and come from trusted messengers, such as front-line doctors treating patients.
- 4) Public health officials must work to regain American's trust; Americans want to be educated, not indoctrinated.

- 5) Former New York Governor Andrew Cuomo participated in medical malpractice and publicly covered up the total number of nursing home fatalities in New York.

In addition to these notable bipartisan successes, the Select Subcommittee developed extensive findings, some of which include:

- 1) The U.S. National Institutes of Health funded gain-of-function research at the Wuhan Institute of Virology.
- 2) The Chinese government, agencies within the U.S. Government, and some members of the international scientific community sought to cover-up facts concerning the origins of the pandemic.
- 3) Operation Warp Speed was a tremendous success and a model to build upon in the future. The vaccines, which are now probably better characterized as therapeutics, undoubtedly saved millions of lives by diminishing likelihood of severe disease and death.
- 4) Rampant fraud, waste, and abuse plagued the COVID-19 pandemic response.
- 5) Pandemic-era school closures will have enduring impact on generations of America's children and these closures were enabled by groups meant to serve those children.
- 6) The Constitution cannot be suspended in times of crisis and restrictions on freedoms sow distrust in public health.
- 7) The prescription cannot be worse than the disease, such as strict and overly broad lockdowns that led to predictable anguish and avoidable consequences.

Chairing the Select Subcommittee for the 118th Congress has been my honor. I said from the beginning, this work is the single most impactful responsibility I have undertaken in 12 years in Congress, and it has been. This work will help the United States, and the world, **predict** the next pandemic, **prepare** for the next pandemic, **protect** ourselves from the next pandemic, and hopefully **prevent** the next pandemic. Members of the 119th Congress should continue and build off this work, there is more information to find and honest actions to be taken.

The COVID-19 pandemic highlighted a distrust in leadership. Trust is earned. Accountability, transparency, honesty, and integrity will regain this trust. A future pandemic requires a whole of America response managed by those without personal benefit or bias. We can always do better, and for the sake of future generations of Americans, we must. It can be done!

Sincerely,



Brad Wenstrup, D.P.M.
Chairman

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V. Masks and Mask Mandates Were Ineffective at Controlling the Spread of COVID-19.

Much of the conversations around the earliest days of the COVID-19 pandemic surrounded inconsistent messaging and a lack of understanding around the virus in general. One area of policy that was riddled with contradictions was the use of face masks. Throughout the pandemic federal, state, and local governments had conflicting policies and rhetoric regarding wearing face masks.

In the very early days, public health officials urged the general public not to wear masks.⁷⁷³ That messaging was then replaced saying individuals should wear a mask, and then the American people were told they must wear a mask.⁷⁷⁴ The first recommendation for the public to wear face masks by the CDC was April 3, 2020.⁷⁷⁵ But before this, the WHO and CDC reported healthy members of the public at large should not wear masks, reserving them for those who were sick or most susceptible to the virus.⁷⁷⁶ On February 29, 2020, the U.S. Surgeon General tweeted, urging people not to buy masks and stated proper hygiene and a flu vaccine would be an adequate solution to the virus.⁷⁷⁷

During a *60 Minutes* interview on March 8, 2020, Dr. Fauci similarly stated "when you're in the middle of an outbreak, wearing a mask might make people feel a little bit better and it might even block a droplet, but it's not providing the perfect protection that people think that it is."⁷⁷⁸ This was consistent with the CDC and WHO's guidance, which in late March 2020 recommended using masks if one was sick or caring for a sick individual.⁷⁷⁹

But these sentiments were reversed just about a month later. On April 3, 2020, the CDC issued guidance recommending non-medical face coverings be worn in areas with high amounts of potential community transmission.⁷⁸⁰ The guidance stated to wear "cloth face coverings fashioned from household items or made at home from common materials ... as an additional, voluntary public health measure."⁷⁸¹ The CDC went as far as posting a video teaching the public how to make masks with a T-shirt and rubber bands.

These initial changes in statements and reversals of policy are understandable, as they were said in the panicked early days of the novel coronavirus, and public health officials were

⁷⁷³ Holly Yan, *Want to prevent another shutdown, save 33,000 lives and protect yourself? Wear a face mask, doctors say*, CNN (June 29, 2020).

⁷⁷⁴ *Id.*

⁷⁷⁵ Chris Megerian, *et al.*, *CDC recommends wearing face masks during coronavirus pandemic*, LOS ANGELES TIMES (Apr. 3, 2020); Colin Dwyer & Allison Aubrey, *CDC Now Recommends Americans Consider Wearing Cloth Face Coverings In Public*, NPR (Apr. 3, 2020).

⁷⁷⁶ Holly Yan, *supra* note 773.

⁷⁷⁷ This tweet has since been deleted.

⁷⁷⁸ *60 Minutes*, CBS (Mar. 8, 2020).

⁷⁷⁹ *See generally*, Holly Yan, *supra* note 773.

⁷⁸⁰ Chris Megerian, *supra* note 775; Colin Dwyer & Allison Aubrey, *supra* note 775.

⁷⁸¹ RECOMMENDATIONS REGARDING THE USE OF CLOTH FACE COVERINGS, ESPECIALLY IN AREAS OF SIGNIFICANT COMMUNITY-BASED TRANSMISSION, CTRS. FOR DISEASE CONTROL AND PREVENTION (Apr. 3, 2020) *available at* <https://stacks.cdc.gov/view/cdc/86440>.

working with the limited information they had on hand. However, public health officials eventually acquired more information about COVID-19 at a rapid pace.

Ultimately, a systematic review carried out by Cochrane Collaboration—one of the most highly regarded methodologies in evidence-based healthcare—found that the pooled randomized control trials they analyzed “did not show a clear reduction in respiratory viral infection with the use of medical/surgical masks” and that “[t]here were no clear differences between the use of medical/surgical masks compared with N95/P2 respirators in healthcare workers when used in routine care to reduce respiratory viral infection.”⁷⁸² These results appear to directly contradict public health agencies’ and local governments’ support for broadly requiring masking throughout much of the pandemic.

FINDING: Public Health Officials Flip Flopping on the Efficacy and Use of Face Masks Without Full Scientific Transparency Caused Mistrust in Public Health Establishments.

On January 20, 2021, in one of his very first actions as President, President Biden signed Executive Order (EO) 13991. Part of the order reads:

The heads of executive departments and agencies (agencies) shall immediately take action, as appropriate and consistent with applicable law, to require compliance with CDC guidelines with respect to wearing masks, maintaining physical distance, and other public health measures by: on-duty or on-site Federal employees; on-site Federal contractors; and all persons in Federal buildings or on Federal lands.⁷⁸³

President Biden signed another EO, the very next day, compelling the Transportation Security Administration and other federal agencies to also require face masks on all forms of domestic and international travel.⁷⁸⁴ This language in essence made CDC guidances actionable. Before, these guidances were non-binding recommendations provided by the public health officials as a best practice; however, President Biden’s EO called for these actions to be mandatory.

Approximately four months later, the Biden Administration and the CDC amended the guidelines on mask wearing. On April 27, 2021, it was announced that fully vaccinated individuals did not need masks during small outdoor gatherings, but that they should still be worn at large outdoor gatherings as well as indoor events.⁷⁸⁵

⁷⁸² Tom Jefferson, *et al.*, *Physical interventions to interrupt or reduce the spread of respiratory viruses*, COCHRANE (Jan. 30, 2023).

⁷⁸³ Exec. Order No. 13991, 86 FR 7045 (Jan. 25, 2021).

⁷⁸⁴ Exec. Order No. 13998, 86 FR 7205 (Jan. 21, 2021).

⁷⁸⁵ Jason Hoffman & Maegan Vazquez, *Biden pushes new CDC mask guidance as a reason why all Americans should get vaccinated*, CNN (Apr. 27, 2021); Elizabeth Cohen, *et al.*, *CDC issues new outdoor mask guidance for fully vaccinated people*, CNN (Apr. 27, 2021).

On May 13, 2021, the CDC announced that the mask mandate was effectively lifted and individuals who were fully vaccinated did not need to wear masks at all (except as otherwise required, such as the mandate on public transportation which was still in effect).⁷⁸⁶

This abrupt announcement and change caused mass confusion amongst state and local officials, as well as the public at large. People did not know which way to turn or which policy to follow.⁷⁸⁷ Senior government officials did not alleviate the confusion. Merely two days before the CDC dropped the guidance on required face masks, Dr. Walensky appeared before Senate HELP and adamantly defended the guidance at the time. At the hearing, Dr. Walensky stated the measures that are known to prevent the spread of the virus must remain the policy, despite calls from lawmakers that suggested the CDC was too harsh in requesting masks for outdoors.⁷⁸⁸ This abrupt announcement stunned medical and public health experts. At the Senate hearing, Dr. Walensky doggedly argued the CDC policy was the most appropriate at the time, yet two days later it was changed without providing people proper notice to prepare for the lifting of the restriction.

Due to the change in Biden Administration policy and CDC guidance, many states began lifting their mask mandates.⁷⁸⁹ On May 20, 2021, in yet another whiplash moment, Dr. Fauci stated he believed Americans were “misinterpreting” the guidance.⁷⁹⁰ In an interview, he stated “[the CDC] said: If you are vaccinated, you can feel safe — that you will not get infected either outdoors or indoors. It did not explicitly say that unvaccinated people should abandon their masks.”⁷⁹¹ This is one of many statements that were provided by public health leadership without the backing of a scientific study. It was a declaration by Dr. Fauci, verified by Dr. Fauci.

On July 26, 2021, the CDC issued yet another change in guidance, stating even vaccinated individuals should wear masks when indoors, if in a region with a substantial and high transmission.⁷⁹² The guidance also recommended masks be mandated at schools for all students and faculty, regardless of vaccination status. The Biden Administration and CDC provided no scientific justification or information for the change, apart from citing the new Delta variant.⁷⁹³

Citing the availability of vaccines, treatments, and improved testing options, on February 25, 2022, the CDC stated residents in areas of substantial and high transmissions (of which 70 percent of the country was no longer considered due to a change in the CDC metrics when determining COVID-19 risk by county) did not need to wear a mask, regardless of vaccination

⁷⁸⁶ Paul LeBlanc & Kaitlan Collins, *Biden touts new CDC mask guidance as ‘a great day for America’*, CNN (May 13, 2021).

⁷⁸⁷ *Id.*

⁷⁸⁸ An Update from Federal Officials on Efforts to Combat COVID-19: Hearing before Senate Committee on Health, Education, Labor & Pensions, 117th Cong., (May 11, 2021).

⁷⁸⁹ Chas Banner, *Mask Mandates Are Back – Even for the Vaccinated. Here’s What to Know*, N.Y. MAGAZINE (Aug. 7, 2021).

⁷⁹⁰ See, Wilson Wong, *Fauci says public is ‘misinterpreting’ latest CDC mask guidance*, NBC (May 20, 2021).

⁷⁹¹ *Id.*

⁷⁹² HOW TO PROTECT YOURSELF & OTHERS, CTRS. FOR DISEASE CONTROL AND PREVENTION (updated July 26, 2021) (archived copy with Select Subcomm. Staff).

⁷⁹³ *Id.*

status.⁷⁹⁴ By April 2022, mask mandates were lifted in all U.S. states, except Hawaii that still had a mask mandate in schools.⁷⁹⁵

In times of national public health crisis, Americans should be able to turn to the CDC to guide us through turmoil. Yet, during the COVID-19 pandemic, the worst public health crisis in our modern era, the CDC constantly redirected their opinions and provided conflicting answers. These actions undermined the American people's belief in the CDC, public health leadership, and science as a whole. At the start of the pandemic, 69 percent of Americans believed what the CDC said, yet by March 2022 that number was only 44 percent.⁷⁹⁶ This must change before a future pandemic.

FINDING: The Biden Administration Exceeded its Authority by Mandating Masks.

In February 2021, the CDC, under President Biden's EO, required the use of masks on public transportation under section 264(a) of the Public Health Service Act of 1944 [hereinafter "PHSA"].⁷⁹⁷ On April 13, 2022, the CDC announced it extended the requirement for face masks on public transportation by an additional 15 days.⁷⁹⁸ However, a lawsuit filed by 21 state Attorneys General called to block the federal mandate extension, particularly after the CDC suggested in a guidance in late February 2022 that almost 70 percent of Americans could stop wearing masks.⁷⁹⁹

On April 18, 2022, a federal judge of the District Court for the Middle District of Florida, found the mandate unlawful, stating the CDC exceeded its legal authority.⁸⁰⁰ The PHSA allows the CDC to prevent the interstate spread of communicable disease.⁸⁰¹ The CDC argued the mask requirement, which was issued as an emergency action, was "reasonable and necessary measure to prevent the introduction, transmission, and spread of COVID-19" and that it was acting within the scope of power granted by Congress under the PHSA.⁸⁰²

The court held the CDC's interpretation of its powers granted by Congress was overly broad and struck down the mandate as unlawful.⁸⁰³ The court noted that while section 264(a) of the PHSA does allow for regulations to curb the spread of communicable diseases, the power to do so must be done within the enumerated actions of the regulation, namely "inspection,

⁷⁹⁴ Berkeley Lovelace Jr. & Erika Edwards, *Indoor mask use no longer necessary across most of the U.S., CDC says*, NBC (Feb. 25, 2022).

⁷⁹⁵ Katie Teague, *et al.*, *Where Are COVID-19 Mask Mandates Still in Effect?*, CNET (Apr. 13, 2022).

⁷⁹⁶ James Hamblin, *Can Public Health Be Saved?*, THE N.Y. TIMES (Mar. 12, 2022).

⁷⁹⁷ *Health Freedom Defense Fund, Inc. v. Joseph R. Biden*, No. 8:21-cv-01693-KKM-AEP, 2022 (M.D. Fla. Apr. 18, 2022).

⁷⁹⁸ Will Stone & Pien Huang, *CDC extends transportation mask mandate until May 3*, NPR (Apr. 13, 2022).

⁷⁹⁹ Apoorva Mandavilli, *New C.D.C. Guidelines Suggest 70 Percent of Americans Can Stop Wearing Masks*, THE N.Y. TIMES (Feb. 25, 2022).

⁸⁰⁰ *Health Freedom Defense Fund, Inc. v. Joseph R. Biden*.

⁸⁰¹ *Id.*

⁸⁰² *Id.*

⁸⁰³ *Id.*

fumigation, disinfection, sanitation, pest extermination, and destruction of contaminated animals and articles, and other measures.”⁸⁰⁴

The CDC argued that “sanitation” measures were intended for the general promotion of hygiene and prevention of disease, and as such the mask mandate was appropriate under that definition, even though “sanitation” is not defined by the PHSA.⁸⁰⁵ The court ruled masking was distinct from sanitation.⁸⁰⁶ The court also discussed that since its enactment, the PHSA has rarely been invoked, and “generally limited to quarantining infected individuals and prohibiting the import or sale of animals known to transmit disease.”⁸⁰⁷ The decision further noted the CDC’s use of section 264(a) (notably, shutting down the cruise ship industry and stopping landlords from evicting tenants who had not paid their rent) were ruled as acts that also exceeded the CDC’s statutory authority.⁸⁰⁸

The court’s decision also discussed how the CDC did not adequately follow the rules of the Administrative Procedure Act [hereinafter “APA”] by not providing the public with an adequate review and comment period and further not properly explaining its reasoning.⁸⁰⁹ Ultimately, the court ruled the mask mandate exceeded the CDC’s, and by extension the Biden Administration’s, statutory authority and violated the procedures for agency rulemaking under the APA.⁸¹⁰ This decision almost immediately ended the mask mandate for public transportation. The judge presiding over the case wrote in her opinion, “[i]f Congress intended this definition, the power bestowed on the C.D.C. would be breathtaking...And it certainly would not be limited to modest measures of ‘sanitation’ like masks.”⁸¹¹

FINDING: The U.S. Centers for Disease Control and Prevention Relied on Flawed Studies to Support the Issuance of Mask Mandates.

In issuing guidances that mandated the use of masks across the country, the CDC publicly relied on several different studies to justify the actions. The CDC provided a list of approximately 15 studies that demonstrated wearing masks reduced new infections.⁸¹² Yet, all 15 of the provided studies are observational studies that were conducted after COVID-19 began and, importantly, none of them were RCTs.⁸¹³ RCTs are considered the gold standard in medical research.

⁸⁰⁴ *Id.*

⁸⁰⁵ *Id.*

⁸⁰⁶ *Id.*

⁸⁰⁷ *Id.*

⁸⁰⁸ *Id.*

⁸⁰⁹ *Id.*

⁸¹⁰ *Id.*

⁸¹¹ *Id.*

⁸¹² Science Brief: *Community Use of Masks to Control the Spread of SARS-CoV-2*, CTRS. FOR DISEASE CONTROL AND PREVENTION (last updated Dec. 6, 2021).

⁸¹³ *Id.*

One study the CDC utilized to mandate masks was the “Missouri hairstylists.”⁸¹⁴ There, an observational cohort study, focused on two hairstylists that were positive for COVID-19 while providing hair styling services to customers.⁸¹⁵ The stylists worked on 139 customers over several days, and both stylists were masked the whole time. Many of the customers were as well, but not all of them were. Out of 139 people, 67 customers chose to test for COVID-19 after receiving their service, and all of whom tested negative. The other 72 either did not test for COVID-19 or did not report any symptoms of the virus.

While these numbers appear significant, this study is far from perfect. For example, the 72 clients who reported no COVID-19 symptoms could absolutely have been positive but asymptomatic, or purposely chose not to report to the Green County Health Department.⁸¹⁶ There was also no control group for this study. There was no way to know, how many people, if anyone at all, could have been infected had neither stylist worn a mask during the appointments. Further, the study does not discuss any alternatives as to why no one became sick. There was no discussion of the ventilation of the salon, the hand hygiene of the stylists, or the fact that a client and stylists generally do not come face to face.

The CDC also utilized a study of 1,000 public school children in Arizona that concluded students without mask mandates were 3.5 times as likely to experience COVID-19 outbreaks as the ones that did have mask mandates.⁸¹⁷ The study published in September 2021 reviewed school-associated COVID-19 outbreaks and compared rates across schools with and without mandates.⁸¹⁸

However, this study also posed serious flaws. The very first lines of the paper note the authors studied school mask policies and COVID-19 outbreaks between “July 15-August 31, 2021.”⁸¹⁹ This time frame is important, because the schools that were reviewed for the study were not all open at the same time. For example, some of these schools were not open during the month of July at all; some of the other schools did not have a start date until August 10; and some of the schools only had a few weeks of student activity during the summer.⁸²⁰ There was also not a control for the vaccination status of staff and students and the definition of an outbreak of COVID-19 was two or more cases among staff or students within a 14-day period versus cases per week per student.⁸²¹ Further, the list of Maricopa County schools used for the study included: at least three schools from Pima County (two hours away), one preschool, at least one virtual academy, and more than 80 vocational programs.⁸²² When asked about these discrepancies, the

⁸¹⁴ M. Joshua Hendrix, *et al.*, *Absence of Apparent Transmission of SARS-CoV-2 from Two Stylists After Exposure at a Hair Salon with a Universal Face Covering Policy — Springfield, Missouri, May 2020*, MMWR (July 17, 2020).

⁸¹⁵ *Id.*

⁸¹⁶ Jeffrey H. Anderson, *Do Masks Work?*, CITY JOURNAL (Aug. 11, 2021).

⁸¹⁷ Megan Jegn, *et al.*, *Association Between K–12 School Mask Policies and School-Associated COVID-19 Outbreaks — Maricopa and Pima Counties, Arizona, July–August 2021*, MMWR (Sep. 24, 2021).

⁸¹⁸ *Id.*

⁸¹⁹ *Id.*

⁸²⁰ *Id.*

⁸²¹ *Id.*

⁸²² *Id.*

journal replied “*MMWR* is committed to quickly correcting errors when they are identified. We reviewed the specific items that you describe below and found no errors.”⁸²³

A similar study out of Georgia was published in May 2021.⁸²⁴ In this study, authors reviewed case rates of 90,000 students, comparing schools with and without mandates.⁸²⁵ It showed 37 percent lower instances of COVID-19 in schools where staff were required to wear a mask and 21 percent lower for children.⁸²⁶ However, the authors noted the difference was not statistically significant, and thus the data could not be used to infer causal relationships.⁸²⁷

Yet, in an interview with *Face the Nation*, at a White House briefing, and a public tweet, the CDC cited the Arizona study and claimed in a blanket statement that lack of school mask mandates more than tripled the risk of outbreaks.⁸²⁸

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⁸²³ David Zweig, *The CDC’s Flawed Case for Wearing Masks in School*, THE ATLANTIC (Dec. 6, 2021).

⁸²⁴ Jenna Gettings, *et al.*, *Mask Use and Ventilation Improvements to Reduce COVID-19 Incidence in Elementary Schools — Georgia, November 16–December 11, 2020*, MMWR (May 28, 2021).

⁸²⁵ *Id.*

⁸²⁶ *Id.*

⁸²⁷ *Id.*

⁸²⁸ David Zweig, *The CDC’s Flawed Case for Wearing Masks in School*, THE ATLANTIC (Dec. 6, 2021).



Additional, peer-reviewed, literature on masking began to emerge as the pandemic progressed. In May 2020, a study published in *Emerging Infectious Diseases* found “[i]n pooled analysis, we found no significant reduction in influenza transmission with the use of face masks.”⁸²⁹ There, researchers conducted a professional literature review of several RCTs surrounding different nonpharmaceutical interventions for pandemic influenza studies, including ten on face masks.⁸³⁰ Also in May 2020, the *New England Journal of Medicine* published an article on masking in hospitals.⁸³¹ Those researches observed, “[w]e know that wearing a mask outside health care facilities offers little, if any, protection from infection.”⁸³² A November 2022 *British Medical Journal* study found that masking of Spanish school-aged children with cloth masks did not lower SARS-CoV-2 transmission, “suggesting that this intervention was not effective.”⁸³³

During a deposition related to the lawsuit filed by the Attorneys General of Louisiana and Missouri, which alleges collusion by the Biden Administration to censor COVID-19 speech on social media, counsel for the plaintiff asked Dr. Fauci which studies the CDC relied upon to justify the mask mandates.⁸³⁴ They asked Dr. Fauci how many studies, and if any placebo-based

⁸²⁹ Jingyi Xiao, et al., *Nonpharmaceutical Measures for Pandemic Influenza in Nonhealthcare Settings—Personal Protective and Environmental Measures*, EMERGING INFECTIOUS DISEASES (May 26, 2020).

⁸³⁰ EMERGING INFECTIOUS DISEASES (May 26, 2020).

⁸³¹ Michael Klompas, M.D., M.P.H., et al., *Universal Masking in Hospitals in the Covid-19 Era*, THE NEW ENGLAND JOURNAL OF MEDICINE (Apr. 1, 2020).

⁸³² *Id.*

⁸³³ Ermengol Coma, et al., *Unravelling the role of the mandatory use of face covering masks for the control of SARS-CoV-2 in schools: a quasi-experimental study nested in a population-based cohort in Catalonia (Spain)*, BRITISH JOURNAL OF MEDICINE (Nov. 3, 2022).

⁸³⁴ *Missouri v. Biden*, 3:22-cv-01213, (W.D. La. Jan. 11, 2023) (Deposition of Dr. Anthony Fauci (Nov. 23, 2022)).

randomized, double-blind studies were conducted between February 2020 and April 2020.⁸³⁵ Dr. Fauci answered that he could not recall.⁸³⁶ It is absolutely essential that these decisions—decisions that had real life consequences—can be verified after the fact.⁸³⁷

Dr. Fauci admitted that at the population level, masks do not provide effective coverage, stating, “[f]rom a broad public-health standpoint, at the population level, masks work at the margins — maybe 10 percent.”⁸³⁸ He does go on to say that for an individual, who religiously wears a mask, the highest standard of a well-fitted KN95 or N95 is effective.⁸³⁹ However, the reality of that perfect storm of factors coming together for one person, let alone the entire country, is impossible.

In late January 2023, the most rigorous and comprehensive review of the scientific literature on masks during the COVID-19 pandemic was published by Cochrane.⁸⁴⁰ Cochrane is considered the worlds most respected organization for evaluating health interventions, is known for being the single best resource for methodologic research,⁸⁴¹ and is recognized as having the highest standard of evidence-based healthcare.⁸⁴²

The January 2023 publication found that wearing any kind of face covering “probably makes little or no difference” in reducing the spread of respiratory illness.⁸⁴³ The study reviewed 15 trials comparing outcomes of wearing surgical masks versus no mask and also versus N95 masks, in hospital and community settings during the pandemic. The conclusion was that the value of wearing masks was approximately zero.⁸⁴⁴ “There is just no evidence that they make any difference. Full stop.”⁸⁴⁵

The trajectories of the rate of COVID-19 infections for states with mask mandates and states without is virtually identical. Eleven states never mandated masks, while the rest had some form of enforcement.⁸⁴⁶ Mandates generally began in early 2020 and stayed until summer of 2021, some into 2022.⁸⁴⁷

⁸³⁵ *Id.*

⁸³⁶ *Id.*

⁸³⁷ *Id.*

⁸³⁸ David Wallace-Wells, *Dr. Fauci Looks Back: ‘Something Clearly Went Wrong’*, THE N.Y. TIMES (Apr. 24, 2023).

⁸³⁹ *Id.*

⁸⁴⁰ Tom Jefferson, *et al.*, *Physical interventions to interrupt or reduce the spread of respiratory viruses*, COCHRANE (Jan. 30, 2023).

⁸⁴¹ Jeremy Grimshaw, *So what has the Cochrane Collaboration ever done for us? A report card on the first 10 years*, CMAJ (Sep. 28, 2004).

⁸⁴² A. Cipriani, *et al.*, *What is a Cochrane review?*, EPIDEMIOLOGY AND PSYCHIATRIC SCIENCES (Sep. 20, 2011).

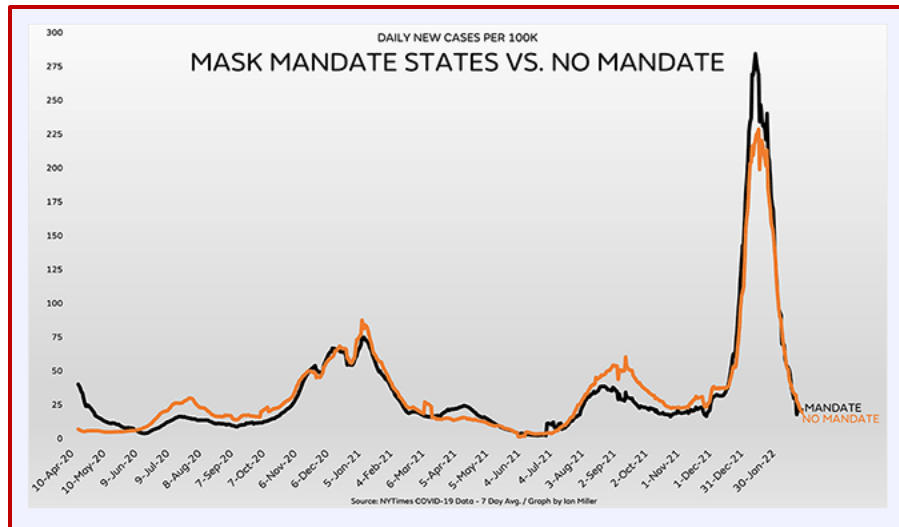
⁸⁴³ Tom Jefferson, *et al.*, *Physical interventions to interrupt or reduce the spread of respiratory viruses*, COCHRANE (Jan. 30, 2023).

⁸⁴⁴ Scott Atlas, Steve Hanke, Philip Kerpen, & Casey Mulligan, *COVID Lessons Learned: A Retrospective After Four Years*, COMM. TO UNLEASH PROSPERITY (Mar. 2024).

⁸⁴⁵ A. Cipriani, *et al.*, *What is a Cochrane review?*, EPIDEMIOLOGY AND PSYCHIATRIC SCIENCES (Sep. 20, 2011).

⁸⁴⁶ Ian Miller, *Unmasked* (Post Hill Press, 2022).

⁸⁴⁷ *Id.*



It is apparent that the CDC and the Biden Administration cherry-picked observational data to fit their narrative that masks are fully effective. Yet, that is not the role of the CDC. The CDC is an agency meant to protect the American people, and part of that responsibility includes conducting, sponsoring, or at the very least examining clinical trials to actually have the best available research before formulating its guidance.

FINDING: Forcibly Masking Young Children, Ages Two and Older, Caused More Harm than Good.

One area where the mask mandate may have caused quantifiable harm is the masking of children. The April 3, 2020-February 25, 2022 (with a brief lift between May 13, 2021-July 27, 2021) CDC guidance masking in schools was unbelievably far reaching. It called for “universal indoor masking by all students (age two and older), staff, teachers, and visitors to K-12 schools, regardless of vaccinations status.”⁸⁴⁸

Many countries, including the U.K., Sweden, Norway, and Denmark, followed the guidelines of the WHO. The WHO specifically did not recommend masking children aged five or younger because they are at low risk of illness, masks are “not in the overall interest of the child,” and also because most children that age are not capable of wearing a mask properly or efficiently.⁸⁴⁹ The WHO also recommended children aged six through 11 not routinely wear masks because of potential adverse impact to psychosocial and learning development.⁸⁵⁰ The WHO further explicitly advised against children wearing masks during physical activities, such as outside playground time, so as not to impede their breathing.⁸⁵¹

But beyond those logical reasons as to not require young children to wear a mask all day, every day, are additional, still unknown consequences. These can be issues such as delayed

⁸⁴⁸ OPERATIONAL GUIDANCE FOR K-12 SCHOOLS AND EARLY CARE AND EDUCATION PROGRAMS TO SUPPORT SAFE IN-PERSON LEARNING, CTRS. FOR DISEASE CONTROL AND PREVENTION (last updated Oct. 4, 2023).

⁸⁴⁹ *Coronavirus disease (COVID-19): Children and masks*, WORLD HEALTH ORG. (Mar. 7, 2022).

⁸⁵⁰ *Id.*

⁸⁵¹ *Id.*

speech and language disorders in young children. Many children that wore masks were more hesitant to talk with a mask on, further substantiating insecurities in communication.⁸⁵²

COVID-19 did not affect children at the same rate as adults. A May 2020 *Journal of the American Medical Association* [hereinafter “JAMA”] review of North American pediatric hospitals article published, “[o]ur data indicates that children are at far greater risk of critical illness from influenza than from COVID-19.”⁸⁵³ COVID-19 was far less serious for children than it was for adults.

Yet even with the early knowledge children were not as susceptible to the virus, many students were subjected to wearing masks for much longer than necessary. Further, in 2022, the CDC did not list speech or language impairments as an adequate reason for a mask exemption for schools.⁸⁵⁴ The consequences of this are already being felt. The American Speech-Language-Hearing Association published a 2023 survey that concluded two-thirds of participating speech-language pathologists reported an increase in client referrals since 2020.⁸⁵⁵

Ignoring the science and facts of COVID-19 and the harms of masking young children was profoundly immoral on behalf of the leadership of the country’s public health officials. The future consequences of these types of draconian policies are not yet known, but public health leaders in the future should remember that all policy must be decided in a balanced manner.

⁸⁵² Stephanie H. Murray, *Speech Therapy Shows the Difficult Trade-Offs of Wearing Masks*, THE ATLANTIC (Mar. 2, 2022).

⁸⁵³ Lara S. Shekardemian, et al., *Characteristics and Outcomes of Children With Coronavirus Disease 2019 (COVID-19) Infection Admitted to US and Canadian Pediatric Intensive Care Units*, JAMA (May 2020).

⁸⁵⁴ Stephanie H. Murray, *Speech Therapy Shows the Difficult Trade-Offs of Wearing Masks*, THE ATLANTIC (Mar. 2, 2022).

⁸⁵⁵ Liza Stahnke, *Elusive Words: Confronting the Post-Pandemic Skills Gap*, ASHAWIRE (May 17, 2024).

VIII. While Testing for COVID-19 Was Flawed, Utilizing Public-Private Partnerships Resulted in Readily Available and Accurate Tests

Historically, the CDC has taken the lead on developing tests for new diseases and distributing the tests through the public health laboratory network.¹⁰⁵² Once the genetic sequence of COVID-19 was identified, the CDC assumed initial responsibility for developing COVID-19 test kits.¹⁰⁵³ The agency produced a diagnostic test, which was distributed to public health laboratories in early February 2020.¹⁰⁵⁴ However, issues with the test's accuracy led to delays in widespread testing. The FDA began issuing EUAs¹⁰⁵⁵ in February 2020 to allow labs and manufacturers to develop, validate, and distribute COVID-19 tests.¹⁰⁵⁶ EUAs allowed for quicker deployment of tests without going through the typical, lengthier approval process.

As the CDC's tests were limited and, therefore, hindered testing capacity, the federal government encouraged private companies and laboratories to develop their own tests. In March 2020, commercial laboratories, such as Quest Diagnostics and LabCorp, were approved to develop and distribute tests, which significantly expanded testing capacity.¹⁰⁵⁷ Additionally, the federal government partnered with private industry to increase testing capabilities, including the launch of drive-through testing sites in collaboration with companies like Walmart, Walgreens, CVS, and others.¹⁰⁵⁸ The COVID-19 Testing Supply Chain Stabilization Task Force was also created to address shortages of testing materials.¹⁰⁵⁹

In addition to developing and distributing tests, the federal government also issued testing guidance. Early on in March 2020, the CDC's testing guidelines prioritized people who were symptomatic, had traveled to affected areas, or were exposed to confirmed cases.¹⁰⁶⁰ Healthcare workers and those in critical roles were also prioritized due to limited testing capacity.¹⁰⁶¹ As testing availability expanded by June of 2020, the CDC updated its guidelines to include asymptomatic individuals, especially those in high-risk settings like long-term care facilities or prisons.¹⁰⁶² The guidance gradually shifted to encourage broader testing.

¹⁰⁵² U.S. DEP'T OF HEALTH AND HUMAN SERVICES OFFICE OF INSPECTOR GENERAL, A-04-20-02027, *CDC's Internal Control Weaknesses led to its initial COVID-19 Test Kit Failure, but CDC Ultimately Created a Working Test Kit*, at 4-5 (Oct. 2023) [hereinafter "HHS IG report"].

¹⁰⁵³ *Id.* at 6.

¹⁰⁵⁴ *Id.*

¹⁰⁵⁵ See below, "The Review of Pfizer's Biologics License Application," pg. 311.

¹⁰⁵⁶ HHS IG report, *supra* note 1502, at 6, figure 2.

¹⁰⁵⁷ Noah Weiland & Katie Thomas, *Trump Administration Moves to Speed Coronavirus Testing*, THE N.Y. TIMES (Mar. 13, 2020).

¹⁰⁵⁸ Amy Goldstein, *et al.*, *Trump says he will partner with private sector to expand coronavirus testing but details are sketchy*, THE WASH. POST (Mar. 13, 2020).

¹⁰⁵⁹ *FEMA COVID-19 Supply Chain Task Force: Supply Chain Stabilization*, FEMA.

¹⁰⁶⁰ Roni Caryn Rabib & Katie Thomas, *Coronavirus Testing Offered With Just a Doctor's Approval, C.D.C. Says*, THE N.Y. TIMES (Mar. 4, 2020).

¹⁰⁶¹ Carolyn Y. Johnson, *et al.*, *In hard-hit areas, testing restricted to health care workers, hospital patients*, THE WASH. POST (Mar. 21, 2020).

¹⁰⁶² *COVID-19 timeline*, U.S. CTRS. FOR DISEASE AND CONTROL (June 13, 2020) (CDC releases consolidated guidelines for COVID-19 testing— including for nursing homes, long-term care facilities, and high-density critical infrastructure workplaces, like food production facilities.)

The federal government also provided funding to facilitate testing. There was significant financial support for testing through the CARES Act, which allocated billions of dollars for testing and disease surveillance.¹⁰⁶³ This funding helped expand testing sites, purchase testing supplies, and support state and local public health efforts. The Health Care Enhancement Act allocated an additional \$25 billion for testing, including funds for states, territories, and tribes to develop testing plans and increase capacity.¹⁰⁶⁴

FINDING: Career Scientists at the U.S. Centers for Disease Control and Prevention Undermined Trust in Public Health by Overpromising and Underdelivering Early Testing Kits, Including Knowingly Putting Tests with a High Failure Rate on the Market Without Appropriate Disclosures.

In the early stages of the pandemic, states sent samples to the CDC, which then tested those samples in its labs.¹⁰⁶⁵ Although the CDC has limited capacity to conduct these tests, Dr. Redfield testified that samples from states were not refused by the CDC, albeit it may have taken several days to test those samples.¹⁰⁶⁶ States wanted their own test kits so that they could test their own samples, and the CDC sought to facilitate that request.

Dr. Redfield was not involved in the CDC's development of these test kits.¹⁰⁶⁷ In fact, Dr. Redfield testified that the CDC is "not a manufacturing company [and] had [he] been involved in those decisions at the time, [he] would have recommended a contract manufacturing company manufacture [the test kits]."¹⁰⁶⁸

While the CDC's career scientists' desire to quickly develop these test kits was laudable, the execution was disastrous. These career scientists assured the public that reliable, widespread testing would soon be accessible, helping to contain outbreaks and protect public health.¹⁰⁶⁹ These officials opted to develop their own test kits rather than use tests already approved and distributed internationally¹⁰⁷⁰ or seek assistance from industry. This slowed the process significantly and prevented wider access to testing.

The CDC's first batch of COVID-19 test kits, distributed in February 2020, were found to be faulty due to contamination issues.¹⁰⁷¹ These tests that were rushed to market without

¹⁰⁶³ See, CARES Act, *supra* note 396.

¹⁰⁶⁴ *Id.*

¹⁰⁶⁵ Transcribed Interview of Robert Redfield, M.D., Former Dir., U.S. Ctrs. for Disease and Control, at 43 (Mar. 17, 2022).

¹⁰⁶⁶ *Id.*

¹⁰⁶⁷ *Id.* at 75 ("There were people at the states, obviously a number of them said it would be easier for them if they could do the tests themselves. And people at CDC had decided that they would try to facilitate that. And this is not something I was engaged in the decision making at the time. But the lab group and the incident command group decided that the lab team would develop reagents that then they would distribute to the public health labs.").

¹⁰⁶⁸ *Id.*

¹⁰⁶⁹ Arman Azad, *WHO and CDC never discussed providing international test kits to the US, global health agency says*, CNN (Mar. 18, 2020).

¹⁰⁷⁰ *Id.*

¹⁰⁷¹ Dina Temple-Raston, *CDC Report: Officials Knew Coronavirus Test Was Flawed But Released It Anyway*, NPR (Nov. 6, 2020).

sufficient validation had high failure rates. These included both false positives and false negatives, which undermined the reliability of testing as a public health tool.¹⁰⁷²

However, within weeks of distribution, it was discovered that the tests were producing inconclusive results due to a flaw in one of the components used in the chemical analysis (referred to as a “reagent”) of the collected sample.¹⁰⁷³ This flaw led to a significant delay in the ability of public health labs to conduct tests, leaving the U.S. blind to the early spread of the virus. These issues led to delays in testing at a critical time when the virus was beginning to spread widely in the U.S. Despite the promises of widespread testing, many areas of the country faced severe shortages of tests, leading to long wait times for results and a lack of timely data to inform public health decisions. There were instances where CDC scientists did not fully disclose the limitations and failure rates of certain tests.¹⁰⁷⁴ For example, some tests that were approved by the FDA under EUAs were later found to be less accurate than initially reported.¹⁰⁷⁵

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¹⁰⁷² *Summary of the Findings of the Immediate Office of the General Counsel's Investigation Regarding CDC's Production of COVID-19 Test Kits*, DEP'T OF HEALTH AND HUMAN SERVICES (June 19, 2020) available at <https://www.documentcloud.org/documents/6953861-6-19-20-Summary-of-the-Findings-of-the-Immediate.html>.

¹⁰⁷³ *Id.*

¹⁰⁷⁴ Dan Vergano, *The Government Asked Us Not To Release Records From The CDC's First Failed COVID Test. Here They Are*, BUZZFEED NEWS (Dec. 8, 2021).

¹⁰⁷⁵ *Id.*

Figure 2: Initial COVID-19 Test Kit Production Timeline

(Specific dates are in parentheses.)

DECEMBER 2019

(31) The World Health Organization (WHO) notes several cases of viral pneumonia in Wuhan, China.



JANUARY 2020

(9) CDC activates a Center-Led Response.

(10) Researchers in China post the genome sequence for COVID-19. CDC begins creation of the COVID-19 test kit.

(18) CDC CLIA lab is approved to use COVID-19 test on human samples.

(20) First U.S. case of the novel coronavirus is confirmed by CDC.

(21) CDC activates the EOC for an Agency-Wide Response.

(24) CDC publicly posts the procedure and sequences for its COVID-19 assay, allowing others the ability to develop similar test kits.

(30) WHO declares a public health emergency of international concern.

(31) HHS declares a national public health emergency for the United States.

FEBRUARY 2020

(1) Test kit is assembled.

(2) CDC ships test kits to intermediary for distribution to PHLs.

(4) FDA issues EUA for CDC's COVID-19 test kit.

(6) Initial COVID-19 test kits are distributed to PHLs.

(8) PHLs begin to report problems with COVID-19 test kits.

(10) CDC begins to troubleshoot COVID-19 test kit problems.

(28) Second COVID-19 test kits are distributed to PHLs.

MARCH 2020

(15) FDA reissues EUA.



The combination of overpromising, delays, and the release of unreliable tests created confusion and frustration among the public. This confusion was compounded by mixed messages from federal officials, which further eroded trust. The initial missteps in testing not only hampered efforts to control the pandemic but also had long-lasting effects on public trust in government and public health institutions. The perception that CDC scientists had misled the public about the availability and effectiveness of testing contributed to skepticism about other public health measures, including vaccination.

Although CDC scientists eventually acknowledged the problem with the test kits, the delay in addressing the issue and the lack of initial transparency contributed to a growing

mistrust in the public health officials' handling of the pandemic. The failure of the CDC's early test kits highlighted the need for more rigorous testing and validation procedures before rolling out diagnostic tools in a public health emergency.

CDC scientists overpromising widespread and reliable testing, followed by the underdelivering of faulty and unreliable tests, created a significant gap between public expectations and reality. This gap, exacerbated by a lack of transparency about the limitations of early tests, contributed to a broader erosion of confidence in public health measures. Moving forward, it is essential that lessons from these early failures are used to improve the development, validation, and communication of public health interventions.

FINDING: Public-Private Partnerships Were More Effective in Increasing Testing Production, Distribution, and Capacity than Career Government Bureaucrats.

In the early stages of the pandemic, the CDC was tasked with developing and distributing diagnostic tests for COVID-19. However, as previously discussed the initial rollout was slow, and the tests produced by the CDC were plagued by inaccuracies and supply shortages. These early setbacks hindered the U.S.'s ability to rapidly scale up testing capacity and track the virus's spread. Furthermore, the CDC's focus on centralized control over test production and distribution created bottlenecks that delayed broader testing efforts.

By mid-March 2020, it became clear that public health authorities could not handle the growing demand for testing alone. The involvement of private industry, particularly diagnostic companies, biotechnology firms, and commercial laboratories, became essential to improve testing at scale. The shift to include the private sector catalyzed a rapid expansion in testing availability and quality.

Private industry played a pivotal role in increasing the production capacity for COVID-19 testing kits. Commercial diagnostic laboratories such as LabCorp, Quest Diagnostics, Abbott Laboratories, and Roche Diagnostics leveraged their established infrastructure and experience to ramp up production of both molecular [hereinafter "PCR"] and antigen-based testing kits.¹⁰⁷⁶ By mid-2020, these companies had significantly increased the daily output of tests, contributing to an exponential rise in the U.S.'s overall testing capacity.

For example, Abbott Laboratories [hereinafter "Abbot"] developed the BinaxNOW rapid antigen test, which became widely used due to its affordability, ease of use, and fast turnaround time.¹⁰⁷⁷ With FDA approval pursuant to an EUA, Abbott was able to produce millions of tests, vastly increasing access to COVID-19 diagnostics.¹⁰⁷⁸ Similarly, Roche Diagnostics produced

¹⁰⁷⁶ REPORT TO CONGRESS, COVID-19 STRATEGIC TESTING PLAN, at 11 U.S. DEP'T OF HEALTH AND HUMAN SERVICES (May 24, 2020).

¹⁰⁷⁷ Jessica Prince-Guerra, et al., *Evaluation of Abbott BinaxNOW Rapid Antigen Test for SARS-CoV-2 Infection at Two Community-Based Testing Sites — Pima County, Arizona, November 3–17, 2020*, MMWR (Jan. 22, 2021).

¹⁰⁷⁸ BINAXNOW COVID-19 Antigen Self Tests, Abbott, available at https://www.globalpointofcare.abbott/us/en/product-details/binaxnow-covid-19-antigen-self-test-us.html?utp=UTID&utid=SEM_G_BR_BinaxNOW_Tests&utm_term=abbott%20binaxnow%20home%20test&utm_source=google&utm_medium=cpc&gclid=Cj0KCQiAgJa6BhCOARIsAMiL7V90XdpTKJfKIXSfigZEpW9T_s8wGfO8emzrBSLWVwWWCRKDivXcP9UaAsCREALw_wcB&gclsrc=aw.ds#find-test.

Suspended in Times of Crisis,” Mr. Bailey characterized his efforts to stop the government’s suppression of content on social media as “a pitched battle for the very character of our nation.”

This is why *Missouri v. Biden* is so important. The question of our time is whether Americans will enjoy the legacy of free speech handed down to us by the founding generation and protected by subsequent generations, or whether federal officials will control what we say, what we hear, and how we debate the veracity of claims and arguments. We are locked in a pitched battle for the very character of our nation. If we do not prevail over government officials who seek to control the speech of millions of Americans, we will be left with, in the words of Justice Gorsuch, ‘a shell of a democracy and civil liberties just as hollow.’¹¹²⁸

FINDING: The Biden Administration and Many Public Health Officials Exaggerated the Power of COVID-19 Vaccines.

COVID-19 vaccines were tremendously important in reducing the severity of COVID-19 symptoms and were extremely effective in doing so.¹¹²⁹ However, the Biden Administration oversold the power of these vaccines. On more than one occasion, President Biden himself overstated the vaccine’s ability to prevent infection and transmission. These false statements likely contributed to Americans’ confusion about COVID-19 vaccines and reduced overall vaccine confidence.

President Biden earned a fact check from the *Associated Press* [hereinafter “AP] for a statement which the AP described as “an absolute guarantee...that people who get COVID-19 vaccines are completely protected from infection, sickness, and death from the coronavirus.”¹¹³⁰ Specifically, President Biden said during a July 21, 2021 CNN Town Hall that “[i]f you’re vaccinated, you’re not going to be hospitalized, you’re not going to be in the IC unit, and you’re not going to die.”¹¹³¹ According to the AP’s fact checkers, by the time of this statement nearly 5,500 vaccinated people had been hospitalized or died with COVID-19.¹¹³² As a result, the AP asserted that Biden “[went] too far in assurances on vaccines.”¹¹³³

Similarly, during his announcement of the COVID-19 vaccine mandate for federal workers and contractors on September 9, 2021, President Biden implied that COVID-19 vaccines

¹¹²⁸ Churches vs. Casinos: The Constitution is not Suspended in Times of Crisis: Hearing Before Select Subcomm. on the Coronavirus Pandemic, 118th Cong. 1, (June 21, 2023).

¹¹²⁹ Oliver J. Watson, *Global impact of the first year of COVID-19 vaccination: a mathematical modelling study*, THE LANCET (Sept. 2022).

¹¹³⁰ Calvin Woodward & Hope Yen, *AP FACT CHECK: Biden goes too far in assurances on vaccines*, ASSOCIATED PRESS (July 22, 2021).

¹¹³¹ Alexandra Jaffe & Aamer Madhani, *Biden says getting vaccinated ‘gigantically important’*, ASSOCIATED PRESS (July 21, 2021).

¹¹³² Calvin Woodward & Hope Yen, *supra* note 1130.

¹¹³³ *Id.*

were effective at preventing the spread of the virus when he said, “The bottom line – we’re going to protect vaccinated workers from unvaccinated coworkers.”¹¹³⁴

Other officials also made false or misleading statements about COVID-19 vaccines. On March 29, 2021, during an appearance on the *Rachel Maddow Show*, Dr. Walensky claimed that CDC data indicated that “vaccinated people do not carry the virus, don’t get sick, and that it’s not just in the clinical trials but it’s also in the real-world data.”¹¹³⁵ On April 1, 2021, a CDC spokesperson walked back Dr. Walensky’s assertions in a comment to *The New York Times*.

Dr. Walensky spoke broadly during this interview...It’s possible that some people who are fully vaccinated could get Covid-19. The evidence isn’t clear whether they can spread the virus to others. We are continuing to evaluate the evidence.¹¹³⁶

On May 16, 2021, during an appearance on CBS’ *Face the Nation*, Dr. Fauci claimed that vaccinated individuals can go without masks even if they have an asymptomatic case of COVID-19 because “it is very unlikely that a vaccinated person, even if there’s a breakthrough infection, would transmit it to someone else.”¹¹³⁷ Dr. Fauci also took it a step further and indicated that vaccinated people become “dead ends” for the virus.

[T]hat’s the reason why we say when you get vaccinated, you not only protect your own health, that of the family, but also you contribute to the community health by preventing the spread of the virus throughout the community. And in other words, you become a dead end to the virus. And when there are a lot of dead ends around, the virus is not going to go anywhere.¹¹³⁸

During each of their appearances before the Select Subcommittee, Dr. Walensky and Dr. Fauci half-heartedly defended their earlier statements. Dr. Walensky testified that she was “speaking in generalities.”

Dr. Rochelle Walensky (June 13, 2023)

Q. Dr. Walensky, simply yes or no. Does a spokesperson from the CDC going on record and correcting the statements that you made undermine and fracture the confidence in CDC leadership?

A. Dr. Joyce, I know you know that I was speaking in generalities, that we saw data and evidence that was over 90 percent that the vaccines

¹¹³⁴ Kathryn Watson, *et al.*, *Biden announces COVID-19 vaccine mandates that will affect 100 million Americans*, CBS NEWS (Sept. 10, 2021).

¹¹³⁵ The Rachel Maddow Show, MSNBC (Mar. 29, 2021).

¹¹³⁶ Apoorva Mandavilli, *Can Vaccinated People Spread the Virus? We Don’t Know, Scientists Say*, THE N.Y. TIMES (Apr. 1, 2021).

¹¹³⁷ Transcript, Anthony Fauci, *Face the Nation* (May 16, 2021).

¹¹³⁸ *Id.*

were effective in preventing severe disease and death and in fact, in preventing symptomatic disease. And that once people had been vaccinated, even if they were to get infected, they were not getting sick and they were not able to transmit to others, so that was the information.¹¹³⁹

Dr. Fauci argued that it “is a complicated issue” and that the vaccines did prevent infection and transmission “in the beginning.”

Dr. Anthony Fauci (June 3, 2024)

Q. Did the COVID vaccine stop transmission of the virus?

A. That is a complicated issue, because, in the beginning, the first iteration of the vaccines did have an effect—not 100 percent, not a high effect—they did prevent infection and, subsequently, obviously, transmission. However, it’s important to point out, something that we did not know early on that became evident as the months went by is that the durability of protection against infection, and hence, transmission was relatively limited, whereas the duration of protection against severe disease, hospitalization, and deaths was more prolonged. We did not know that in the beginning. In the beginning, it was felt that, in fact, it did prevent infection and, thus, transmission, but that was proven, as time went by, to not be a durable effect.¹¹⁴⁰

Dr. Fauci said that it was “not a durable effect,” therefore acknowledging that the vaccines did not effectively prevent the spread of the virus’ later variants.¹¹⁴¹ However, even with the most charitable read of the contemporary data supporting these statements, it appears these were gross overstatements of the COVID-19 vaccines’ protective abilities, even against the earlier variants.¹¹⁴² Perhaps conveniently, the CDC stopped tracking all breakthrough infections beginning May 1, 2021, and instead only tracked breakthrough cases that led to hospitalization or death.¹¹⁴³ The CDC argued that this decision would help “maximize the quality of the data collected on cases of greatest clinical and public health importance.”¹¹⁴⁴ The CDC’s final report showing breakthrough infections indicated that 10,262 infections had occurred across 46 U.S. states.¹¹⁴⁵

¹¹³⁹ Oversight of CDC Policies and Decisions During the COVID-19 Pandemic: Hearing Before the Select Subcomm. on the Coronavirus Pandemic, 118th Cong. 1, at 23 (June 13, 2023).

¹¹⁴⁰ Fauci Hearing, *supra* note 231, at 15

¹¹⁴¹ *Id.*

¹¹⁴² Apoorva Mandavilli, *Can Vaccinated People Spread the Virus? We Don’t Know, Scientists Say.*, THE N.Y. TIMES (Apr. 1, 2021).

¹¹⁴³ CDC COVID-19 Vaccine Breakthrough Case Investigations Team, *COVID-19 Vaccine Breakthrough Infections Reported to CDC — United States, January 1–April 30, 2021*, MMWR (May 28, 2021).

¹¹⁴⁴ Rachel Roubein & David Lim, *CDC under fire for decision to limit tracking of Covid-19 cases in vaccinated people*, POLITICO (July 30, 2021).

¹¹⁴⁵ CDC COVID-19 Vaccine Breakthrough Case Investigations Team, *supra* note 1143.

With the outbreak of the Delta variant beginning around July 2021, it became obvious that any mild protection the vaccine may have provided against infection and transmission was significantly diminished. This first emerged with data reported from Barnstable County, Massachusetts after a COVID-19 outbreak following Independence Day celebrations.¹¹⁴⁶ A CDC study found that three-quarters of the 469 cases were in fully vaccinated individuals.¹¹⁴⁷ The CDC then decided to reverse course and returned to recommending masking regardless of an individual's vaccination status in many areas.¹¹⁴⁸ According to CNN, a source involved with the decision process indicated that:

New unpublished data showing that vaccinated people infected with the Delta coronavirus variant can have as much virus as those who are unvaccinated is the primary driver for the CDC's latest mask guidance change.¹¹⁴⁹

As the Delta variant continued to spread throughout the U.S., this became clear to most Americans as breakthrough infections became commonplace. Ironically, President Biden, Dr. Walensky, and Dr. Fauci, along with numerous other fully vaccinated public figures, reported testing positive for COVID-19 during variant outbreaks, despite being fully vaccinated and boosted.¹¹⁵⁰

The Biden Administration's exaggeration of the COVID-19 vaccine's ability to prevent infection and transmission of COVID-19 may have contributed to Americans' waning trust in vaccines overall.¹¹⁵¹ It is likely that many Americans, especially those who were young and healthy, elected to be vaccinated under the pretense that it would ensure they would not get their loved ones sick. When these pretenses turned out to be false, Americans were understandably upset. This may have also contributed to the lackluster numbers of individuals who elected to receive subsequent booster doses. An October 2023 study, published in the journal *Vaccine*, studied the reasons for why less than 20 percent of eligible Americans had obtained a bivalent booster dose.¹¹⁵² According to the study's findings, 23.1 percent of respondents indicated their reason for not getting the booster included that they believed it wouldn't protect them from being infected.¹¹⁵³

¹¹⁴⁶ Katie Brace, *et al.*, *New Provincetown COVID Cases 'Overwhelmingly' in Vaccinated: Town Manager*, NBC BOSTON (July 12, 2021).

¹¹⁴⁷ Catherine M. Brown, *et al.*, *Outbreak of SARS-CoV-2 Infections, Including COVID-19 Vaccine Breakthrough Infections, Associated with Large Public Gatherings — Barnstable County, Massachusetts, July 2021*, MMWR (Aug. 6, 2021).

¹¹⁴⁸ Jacqueline Howard, CDC updates guidance, recommends vaccinated people wear masks indoors in certain areas, CNN (July 27, 2021).

¹¹⁴⁹ *Id.*

¹¹⁵⁰ Jamie Gumbrecht & Jen Christensen, *Fauci tests positive for Covid-19*, CNN (June 15, 2022); Media Statement, CDC, *CDC Director Tests Positive For COVID-19* (Oct. 22, 2022); Maegan Vazquez, *et al.*, *Biden tests positive for Covid-19 and is experiencing mild symptoms*, CNN (July 21, 2022).

¹¹⁵¹ *Public trust in vaccines shows a dip*, NATURE INDIA (May 8, 2024).

¹¹⁵² Elizabeth T. Jacobs, *et al.*, *Understanding low COVID-19 booster uptake among US adults*, VACCINE (Oct. 6, 2023).

¹¹⁵³ *Id.*

millions of young, healthy workers for whom the marginal benefit of a booster was, according to the arguments made by Dr. Gruber and Dr. Krause, likely very low.¹¹⁹⁵

FINDING: U.S. Centers for Disease Control and Prevention Director Rochelle Walensky Overruled Expert Advisors in an Apparent Attempt to Satisfy President Joe Biden's Arbitrary Vaccine Approval Goals.

On September 17, 2021, VRBPAC convened a public meeting to evaluate the use of Pfizer's primary series COVID-19 vaccine as booster doses for those who had already received the primary series. The panel voted against making a recommendation for the broad population, but recommended FDA authorize boosters for Americans aged 65 and up, those aged 18 to 64 who were at high risk for severe illness, as well as those in certain high-risk occupations.¹¹⁹⁶ On September 22, 2021, FDA announced an amended EUA for booster doses in the same populations recommended by VRBPAC.¹¹⁹⁷

On September 23, 2021, ACIP convened its public meeting on the same topic.¹¹⁹⁸ The ACIP concurred with FDA's recommendation for a COVID-19 vaccine booster for Americans 65 and older and aged 18 to 64 who were at high risk for serious illness.¹¹⁹⁹ However, departing from the FDA in a 6-9 vote, ACIP specifically ruled against a broad recommendation for workers in higher risk professions including health care workers and teachers.¹²⁰⁰

On September 24, 2021, in a decision deemed "highly unusual," Dr. Walensky overruled ACIP's recommendation and issued updated CDC guidance that fully concurred with FDA.¹²⁰¹ Dr. Walensky characterized her announcement of new CDC guidance simply as an endorsement of ACIP's recommendations. However, she completely skipped over the fact that she had overruled her own agency's advisory committee on the key aspect of boosting workers in high-risk professions.

Today, CDC Director Rochelle P. Walensky, M.D., M.P.H., endorsed the CDC Advisory Committee on Immunization Practices' (ACIP) recommendation for a booster shot of the Pfizer-BioNTech COVID-19 vaccine in certain populations and also recommended a booster dose for those in high risk occupational and institutional settings.¹²⁰²

¹¹⁹⁵ *Id.*

¹¹⁹⁶ *Center for Biologics Evaluation and Research, Summary Minutes*, FOOD & DRUG ADMIN. (Sept. 17, 2021) available at <https://www.fda.gov/media/152597/download>.

¹¹⁹⁷ News Release, FDA Authorizes Booster Dose of Pfizer-BioNTech COVID-19 Vaccine for Certain Populations, U.S. Food & Drug Administration (Sept. 22, 2021).

¹¹⁹⁸ Melissa Mahtani & Meg Wagner, *The latest on Covid-19 and vaccine boosters*, CNN (Sept. 23, 2021).

¹¹⁹⁹ Meeting of the Advisory Committee on Immunization Practices (ACIP), Summary Minutes (Sept. 22-23, 2021, Publish date: Nov. 4, 2021) available at <https://stacks.cdc.gov/view/cdc/114944>.

¹²⁰⁰ *Id.*

¹²⁰¹ Apoorva Mandavilli & Benjamin Mueller, *C.D.C. Chief Overrules Agency Panel and Recommends Pfizer-BioNTech Boosters for Workers at Risk*, THE N.Y. TIMES (Sept. 24, 2021).

¹²⁰² Press Release, CDC Statement on ACIP Booster Recommendations, Centers for Disease Control and Prevention (Sept. 24, 2021).

During ACIP’s meeting, experts who voted against the measure argued that there was not sufficient data to support the recommendation, that it was too narrow and premature, and that there was little marginal benefit in acting then rather than waiting for better data.¹²⁰³ They also said that they felt they were being pulled into an “emotional decision” and that this decision could undermine confidence in the primary vaccine series.

Ms. Bahta said she felt like they were being pulled into an emotional decision. The science shows that this is a very effective vaccine. This decision was made for the vaccinated, not the unvaccinated. She did not believe they had the data in the younger age groups to make a decision for a booster dose. To her, it would suggest that the vaccine does not work. While they know this is not true, that is likely how that message will be conveyed to the broader public. That was why she voted “not” for the third and fourth interim recommendations.¹²⁰⁴

Dr. Bell agreed with everything Dr. Bahta said in explaining why she voted “no” and also to emphasize that this represented the beginning of a lot of activity around booster doses and other vaccination efforts that are forthcoming. In this situation, at this moment, and given the lack of evidence about the marginal benefits of booster doses for people in certain groups who received a Pfizer primary series, it was too narrow and too soon. In terms of the potential risks for adverse outcomes of waiting until more is known, there was little marginal benefit to making this booster dose available at this time in her opinion given all of the unknowns.¹²⁰⁵

Reportedly, Dr. Walensky’s decision to break with the recommendations of the ACIP came as a surprise to some of her own staff.¹²⁰⁶ According to reports, it was understood that “[h]ours before [Walensky’s] statement, agency insiders predicted she would stick with the usual protocol because doing otherwise would undermine the process and upset the advisers as well as her own staff.”¹²⁰⁷

During ACIP’s meeting, prior to the votes, Dr. Walensky spoke directly to ACIP members, saying, “[k]now that I am grateful for your efforts and that I so appreciate your expertise, your counsel, and your partnership.”¹²⁰⁸ Unfortunately, it appears that Dr. Walensky may not have meant what she said given that she flagrantly ignored ACIP’s expertise and counsel on this key decision.

¹²⁰³ *Id.*

¹²⁰⁴ ACIP Summary Minutes, *supra* note 1199, at 92. Meeting of the Advisory Committee on Immunization Practices (ACIP), Summary Minutes (Sept. 22-23, 2021, Publish date: Nov. 4, 2021) available at <https://stacks.cdc.gov/view/cdc/114944>.

¹²⁰⁵ *Id.*

¹²⁰⁶ Apoorva Mandavilli & Benjamin Mueller, *supra* note 1201.

¹²⁰⁷ *Id.*

¹²⁰⁸ *Id.*

II. The Review of Pfizer's Biologics License Application

COVID-19 vaccines were novel in many ways, including that they were widely distributed under EUA before they received full approval from the FDA. Specifically, the first COVID-19 vaccine was administered under EUA on December 11, 2020, and the first vaccine received full approval on August 23, 2021.¹²⁰⁹ During the course of this 255-day period, more than 360 million doses of COVID-19 vaccines were distributed throughout the U.S.¹²¹⁰ Since the FDA first established its EUA program in 2004, the anthrax vaccine was the only other vaccine to receive an EUA and this vaccine was only administered to an extremely limited cohort of people.¹²¹¹

Throughout the early rollout of COVID-19 vaccinations in the winter and spring of 2021, there was an aggressive and widespread campaign—often with the support of government public health institutions—to convince the American people to get vaccinated. However, the nuances of the vaccines' regulatory status were unclear to most regular people. Instead, these novel mRNA vaccines were dubbed simply as “safe and effective,” with very little opportunity for patients to discuss these vaccines with their doctor and assess their individual risks and benefits.

In actuality, the FDA's standards for EUA differ from their standards for approving a BLA. Most importantly, for a BLA to be approved a product must demonstrate that it is safe and effective, whereas for an EUA to be authorized the product must only demonstrate that it may be safe and effective.¹²¹² The decision to widely distribute COVID-19 vaccines under an EUA likely had significant negative effects on the public's perceptions of the vaccines. It appears that some FDA officials shared this concern as it was later used as a justification for why the review of the BLA was aggressively accelerated.

On May 18, 2021, Pfizer became the first company to submit a BLA to the FDA for review.¹²¹³ Under normal circumstances, this review would have taken around 12 months and have been completed around May 2022. However, this BLA was given priority review status, which stipulates that the process should be completed in around 8 months, therefore setting an ADD of mid-January 2022. Details provided by Dr. Gruber, and her deputy, Dr. Krause, indicated that high-level FDA officials imparted significant pressure on them to continually accelerate the timeline despite their concerns that safety and efficacy of the vaccines would suffer.

The FDA ultimately accelerated the process substantially, with Pfizer receiving its official approval letter for its COVID-19 vaccine under the brand name COMIRNATY on August 23, 2021 – nearly 5 months faster than the typical priority review timeline.¹²¹⁴ In September 2021, it

¹²⁰⁹ New Release, U.S. Food & Drug Admin., FDA Approves First COVID-19 Vaccine (Aug. 23, 2021).

¹²¹⁰ Kanishka Singh, *U.S. administers 361.7 million doses of COVID-19 vaccines -CDC*, REUTERS (Aug. 22, 2021).

¹²¹¹ 70 Fed. Reg. 5452 (Feb. 2, 2005).

¹²¹² Guidance Document, Emergency Use Authorization of Medical Products and Related Authorities, U.S. FOOD & DRUG ADMIN. (Jan. 2017).

¹²¹³ Letter from Mary A. Malarkey, Dir., Office of Compliance and Biologics Quality & Marion F. Gruber, Dir., Offices to Vaccines Research and Review, to Amit Pate, Pfizer Inc. (Aug. 23, 2021) *available at* <https://www.fda.gov/media/151710/download>.

¹²¹⁴ *Id.*

became public that Dr. Gruber and Dr. Krause were retiring from the FDA.¹²¹⁵ Shortly afterwards it was reported that Dr. Gruber's and Dr. Krause's departure was related to their internal disagreement about whether available data supported booster shots for the general public.¹²¹⁶

Since then, evidence further indicates that the FDA may have allowed politics to interfere with what should have been dispassionate and scientifically driven regulatory process. Dr. Gruber and Dr. Krause expressed legitimate concerns about the hyper-acceleration of the review of COVID-19 vaccines and booster shots and were ignored. When they detailed these concerns to Dr. Marks he pressured them further.¹²¹⁷ When the issue was brought to Dr. Woodcock, she placed Dr. Marks in charge and sidelined Dr. Gruber and Dr. Krause.¹²¹⁸ Unfortunately, the FDA's acceleration of the process may have contributed to the public's waning confidence in vaccines, public health, and government institutions, and possibly to facilitate harmful COVID-19 vaccine mandates.¹²¹⁹

FINDING: The Biden Administration Sidelined Senior Scientists After They Expressed Concern Regarding the Rapid Pace of Review of Pfizer's Biologics Approval Application.

As Director of OVR, Dr. Gruber was the most senior expert in charge of reviewing BLA's for vaccines, and the ultimate decisionmaker. According to their own testimony, Dr. Gruber, Dr. Krause, and Dr. Marks initially agreed to set the ADD for mid-October 2021.¹²²⁰ According to Dr. Krause, Dr. Marks came back soon afterwards and requested it be accelerated to September 15, 2021.¹²²¹ Dr. Gruber and Dr. Krause discussed with their team and told Dr. Marks that it would be possible to compress the review and meet the September 15 deadline.¹²²² Not long after that, Dr. Marks approached them yet again and demanded that the timeline be pushed up even earlier than September 15—a target date which was already eight months earlier than the standard review timeline, and four months earlier than the priority review timeline.¹²²³

¹²¹⁵ Dan Diamond, *Two FDA officials who oversee coronavirus vaccine reviews to depart*, THE WASH. POST (Sept. 1, 2021).

¹²¹⁶ Noah Weiland & Apoorva Mandavilli, *In Review, Top F.D.A. Scientists Question Imminent Need for Booster Shots*, THE N.Y. TIMES (Sept. 13, 2021, updated Oct. 1, 2021).

¹²¹⁷ E-Mail from Marion Gruber, Ph.D., Dir., Office of Vaccines Research & Review, U.S. Food & Drug Admin, to Peter Marks, FDA Center for Biologics Evaluation and Research (CBER), *et al.*, (July 19, 2021, 11:59 AM).

¹²¹⁸ Woodcock TI, *supra* note 1171, at 79-91

¹²¹⁹ Elizabeth Williams & Robin Rudowitz, *Headed Back to School in 2024: An Update on Children's Routine Vaccination Trends*, KFF (July 18, 2024).

¹²²⁰ SELECT SUBCOMM. ON THE ADMINISTRATIVE STATE, REGULATORY REFORM, AND ANTITRUST OF THE COMM. ON THE JUDICIARY, INTERIM STAFF REPORT, POLITICS, PRIVATE INTERESTS, AND THE BIDEN ADMIN.'S DEVIATION FROM AGENCY REGS. IN THE COVID-19 PANDEMIC, at 85-86 (June 24, 2024) [hereinafter "Judiciary Report"].

¹²²¹ *Id.*

¹²²² *Id.*

¹²²³ *Id.* at 248.

In a July 15, 2021 email from Dr. Gruber to Dr. Marks, she explained that after discussing with her team, she and Dr. Krause believed it would be impossible to accelerate the review any further without “cutting corners.”¹²²⁴

From: Gruber, Marion [REDACTED]
Sent: Thursday, July 15, 2021 8:00 AM
To: Marks, Peter [REDACTED]; Witten, Celia (CBER) [REDACTED]
Cc: Krause, Philip [REDACTED] >
Subject: Pfizer COVID-19 vaccine BLA review timeline

Dear Peter,
Phil and I have further discussed with DVRPA and DVP management the review timeline for the above BLA. As you know we are targeting September 15 as the ADD. It will not be possible to move the ADD up further without cutting corners and lowering our review standards and that I would not be able to defend. We have described our rationale and logic in the attached memo. Feel free to share with JW.

Marion
Marion F. Gruber, Ph.D.
Director
Office of Vaccines Research & Review
Center for Biologics Evaluation & Research
Food & Drug Administration, DHHS
10903 New Hampshire Ave.
Building 71, Rm. 3230
Silver Spring, Maryland 20993
Tel.: [REDACTED]
Email: [REDACTED]

Attached to this email was a memo explaining their logic in more detail.¹²²⁵

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¹²²⁴ E-Mail from Marion Gruber, Ph.D., Dir., Office of Vaccines Research & Review, U.S. Food & Drug Admin, to Peter Marks, FDA Center for Biologics Evaluation and Research (CBER) (July 15, 2021, 8:00 AM).

¹²²⁵ Internal memo, U.S. FOOD & DRUG ADMIN. (July 15, 2021).

Pfizer COVID-19 STN 125742.0 BLA target AD: 09/15/2021

OVRR's decision to expedite the planned completion of the Pfizer BLA review to September 15, 2021, was based on a careful consideration of the steps that need to take place. OVRR's logic is outlined below.

The Pfizer BLA is a complex BLA

Of note, the pivotal study supporting the BLA was conducted in over 40,000 subjects. To provide additional assurance of the safety and effectiveness of this product that is currently administered to millions of subjects in the US and globally, we requested 6 months safety follow-up to support the BLA as opposed to the 2 months safety follow-up that supported the EUA. The applicant has also submitted additional efficacy data on substantial numbers of cases in vaccine and control groups that were not available with the EUA request submission and data on post-authorization safety experience. These additional data are substantial and enable additional important analyses.

The BLA merits a complete and thorough review

OVRR's reviews of vaccine BLAs, unlike those of regulators in other countries, do not rely on summary tables that are generated by the developer. OVRR views it as essential that review of the safety and efficacy data not only includes an evaluation of the data analyses conducted by the applicant, but also includes CBER's own analysis of the datasets submitted by Pfizer. This has been OVRR's standard for all other BLAs, and while time-consuming, OVRR believes that confidence in COVID vaccines would not be served by starting to cut corners on this review.

While the efficacy data may appear simple to evaluate, longer term follow-up of placebo-controlled data provides essential information that may be of high relevance to discussions about boosting. Moreover, the safety data represent the only placebo-controlled data we have on the safety of this vaccine. These placebo-controlled data are likely to be free of biases that might occur in post-licensure observational studies, so it is imperative to carefully review the reported adverse events, including evaluation of the sponsor's attribution of these events (or lack thereof) to vaccination.

As compared with other BLAs, the proposed completion date of Sept 15 would be unprecedented

The Pfizer COVID-19 BLA received priority designation, allowing 8 months for CBER review and is a "rolling" BLA. Note that the final piece of the roll was received on May 18, 2021 at which point the review clock started. We are targeting September 15, 2021 as the date we will be taking regulatory action, which is less than 4 months from the date the last section of the BLA was submitted. Thus, we will be reviewing this very large and complex BLA in a 1/3 rd of the time typically allowed for a BLA standard application and in less than half the time allocated for a priority review application.

This is possible only with deprioritization of other reviews, including some related to COVID, and reassignment of work to other experienced medical officers.

At this time, while we have hired additional medical officers, we have a limited number of clinical reviewers with the specialized experience needed to assess complex preventive vaccine files requiring comprehensive review, such as those for COVID vaccines that have progressed to pursuing an EUA or BLA. Addressing the high volume of COVID-related work has necessitated deprioritizing some vaccine files.

In addition, we have de-prioritized certain COVID-vaccine related submissions (including some from Pfizer), e.g., amendments pertaining to protocols and studies in pregnant women and immunocompromised subjects, until such time that the BLA review is completed.

However, Pfizer requested advice on 4 booster protocols and advice on the safety data base to support use of the COVID-19 vaccine in pediatric populations 6 months – 12 years of age. These cannot be deprioritized and will need to be reviewed by staff and overseen by supervisors familiar with the Pfizer COVID vaccine IND ad EUA, concurrent with review activities for the Pfizer COVID-19 BLA.

While it was not possible to completely reassign other COVID-19 vaccine- related and non-COVID vaccine-related review work for the MOs assigned to the Pfizer BLA, workload adjustments have been made to allow them to focus nearly exclusively on review of this BLA.

In addition, if the trajectory of the pandemic/emergence of variant of concerns (i.e., delta variant) necessitates the review of EUA amendments for booster doses for the currently U.S. EUA authorized COVID-19 vaccines, from a public health perspective, these reviews will need to take priority over completing the BLA review by September 15, 2021.

Additional support from outside OVRP will not speed up the review

Review efforts for the Pfizer COVID-19 vaccine BLA in the various disciplines, including CMC, nonclinical, PV and facility is ongoing. Information requests have been sent to Pfizer as part of these reviews, and responses are pending. However, the rate-limiting step in regard to potentially accelerating the review timeline to earlier than September 15 is the clinical review, considering the complexity of the clinical safety and effectiveness data. The safety review encompasses a critical evaluation and interpretation of solicited and unsolicited safety data and SAES, and clinical AEs of interest including, but not limited to, the myocarditis signal that has been observed following the administration of the Pfizer COVID-19 vaccine under EUA. We are also performing subgroup analyses of safety and effectiveness data for race, ethnicity and subjects with underlying conditions. Completion of these reviews may require additional correspondence with the sponsor. We hope that reviewers will be able to complete their detailed review memos for the various review activities by the beginning of September as planned. After this has been finished, there are important additional review activities to be completed, including label

negotiations, supervisory review, SBRA preparation, etc. such that it would not be possible to issue the license until September 15.

The experienced MOs assigned to this file are working closely with the data analytics team in CDER-OCS and staff in CBER/OBE who are supporting their review efforts. The need for coordination of evaluation and consistency within the review would lead to diminishing returns if additional staff would be added to this effort. In addition, the reviews have already been initiated and sections of the review are being written as they are completed. Other sections depend on the reviews of the earlier sections, so those parts of the review cannot be completed until the earlier parts of the review have been done, and because they need to take the subtleties of the earlier parts into account, cannot as reliably be performed by medical officers who are new to the file. Thus, assigning additional MOs (even if experienced) to assist in review of the Pfizer COVID vaccine BLA, it is likely that the review effort would be will delayed rather than expedited the review effort as these reassigned individuals would need to familiarize themselves with the file.

Furthermore, reassignment of experienced medical officers to the Pfizer BLA would lead to a cascade of further reassignments and their own assignments will be delayed ultimately leading to an increase in back-log including critical ongoing review activities to support:

- Many anticipated several BLA submissions in in 3/4Q of 2021 including the BLAs for the [REDACTED] and BLAs for [REDACTED] all of which are likely to qualify for priority review designation
- The [REDACTED] BLA,
- Several BLA supplements including an efficacy supplement for [REDACTED] for the pediatric population,
- Efficacy supplements for [REDACTED] and [REDACTED]
- Booster protocols for the Pfizer, [REDACTED] COVID EUAs.

In summary, it is not possible to further abbreviate the BLA review timeline for the Pfizer COVID-19 vaccine BLA, our target review date for this file remains September 15, 2021.

The paragraph headers for this memo provide a succinct summary of their arguments:

- The Pfizer BLA is a complex BLA
- The BLA merits a complete and thorough review
- As compared with other BLAs, the proposed completion date of Sept 15 would be unprecedented
- This is possible only with reprioritization of other reviews, including some related to COVID, and reassignment of work to other experienced medical officers
- Additional support from outside OVRP will not speed up review

- Additional support from outside of OVR, if effectively used, might reduce the need to deprioritize certain submissions¹²²⁶

Dr. Marks forwarded this email and the attached memo to Dr. Woodcock and suggested that they set up a phone call to discuss the situation.

From: [Marks, Peter](#)
To: [Woodcock, Janet](#)
Cc: [Tierney, Julia](#)
Subject: FW: Pfizer COVID-19 vaccine BLA review timeline
Date: Thursday, July 15, 2021 10:11:27 AM
Attachments: [image001.png](#)
[Pfizer COVID-19 vaccine BLA review timeline.docx](#)

Dear Janet,

Perhaps we can have a brief call tomorrow? I can fill you in on the conversation that I had with Marion and Phil subsequent to their sending me this document. I have asked them to provide me with a timeline of milestones, and they are meeting with the review team today to be able to do so tomorrow morning. That said, they are intransigent at this time on the Sept 15 date.

Thanks very much.

Best Regards,

Peter

From: Gruber, Marion [REDACTED] >

Sent: Thursday, July 15, 2021 8:00 AM

To: Marks, Peter <[REDACTED]>; Witten, Celia (CBER) <[REDACTED]>

Cc: Krause, Philip <[REDACTED]>

Subject: Pfizer COVID-19 vaccine BLA review timeline

Dear Peter,

Phil and I have further discussed with DVRPA and DVP management the review timeline for the above BLA. As you know we are targeting September 15 as the ADD. It will not be possible to move the ADD up further without cutting corners and lowering our review standards and that I would not be able to defend. We have described our rationale and logic in the attached memo. Feel free to share with OW.

Marion

Marion F. Gruber, Ph.D

Director

Office of Vaccines Research & Review

Center for Biologics Evaluation & Research

Food & Drug Administration, DHHS

10903 New Hampshire Ave.

Building 71, Rm. 3230

Silver Spring, Maryland 20993

Tel.: [REDACTED]

¹²²⁶ *Id.*

¹²²⁷ E-Mail from Janet Woodcock, Acting FDA Dir. to Peter Marks, CBER Dir. (July 16, 2021, 11:10 AM).

Dr. Janet Woodcock (May 13, 2024)

Q. And you said, "Peter, you can find out more when you take over." Are you referring to Dr. Marks taking over the review from Dr. Gruber?

A. Yes, I believe I was.

Q. So is it fair to say that ...[b]y July 16th, you had decided that Dr. Marks would be taking over?

A. That's fair.

Q. Did you make that decision on your own?

A. Yes.

Q. And did Dr. Marks request that, or was it your idea?

A. It was my idea.¹²²⁸

On July 19, 2021, Dr. Gruber, Dr. Krause, Dr. Marks, Dr. Woodcock, and her Chief of Staff, Ms. Julia Tierney, convened a meeting to discuss this further. On July 21, 2021, Dr. Gruber sent an email to the group memorializing her perspective of what had been discussed during the meeting.¹²²⁹

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¹²²⁸ Woodcock TI, *supra* note 1171, at 80.

¹²²⁹ E-Mail from Marion Gruber, Ph.D., Dir., Office of Vaccines Research & Review, U.S. Food & Drug Admin, to Peter Marks, FDA Center for Biologics Evaluation and Research (CBER) (July 19, 2021, 11:59 AM).

From: Gruber, Marion [REDACTED] [REDACTED].gov
Sent: Wednesday, July 21, 2021 11:59 AM
To: Marks, Peter [REDACTED] [REDACTED].gov; Woodcock, Janet <[REDACTED]>
Cc: Tierney, Julia <[REDACTED]>; Krause, Philip [REDACTED]
Subject: Review of Pfizer/BioNTech's BLA for Comirnaty, COVID-19 mRNA vaccine - Summary of meeting dated July 19 2021 - 8:30 am

Dear Janet and Peter,

The following summarizes my understanding of the July 19, 2021, 8:30 am meeting held between you, Phil Krause, Julie Tierney and myself to discuss the review of Pfizer/BioNTech's BLA for Comirnaty, COVID-19 mRNA vaccine. During this meeting, I made reference to the memo that Dr. Krause and I composed and sent to Dr. Marks on July 15, 2021, delineating OVR's rationale for why the review timeline and target action due date, September 15, 2021, for this BLA cannot be compressed further. To recap, that memo stated that the review requires a thorough evaluation and FDA's own analysis of the safety, effectiveness and manufacturing information submitted to support licensure of this vaccine. This has been OVR's standard for all other BLAs, and while time-consuming, OVR believes that public confidence in COVID-19 vaccines would not be served by rushing our review and evaluation of the submitted data. In addition, Dr. Krause and I pointed out the very important regulatory issues that still need to be settled by the time we take action on this BLA—including the pediatric plan — which is becoming increasingly complex in light of increasing evidence of association of this vaccine and development of myocarditis (especially in young males, but also ages included in the BLA indication). This also impacts the finalization of post-marketing requirements and post-marketing commitments. In addition, there are pending information requests to the sponsor, and there will likely be additional information requests based on ongoing review of the data, and the timing of the sponsor response is beyond CBER control.

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I reiterated during our meeting that OVRP is targeting September 15, 2021, as the date we will be taking regulatory action, which is less than 4 months from the date the last section of the BLA was submitted. Thus, we will be reviewing this complex BLA with a large amount of data, in a third of the time typically allowed for a BLA standard application and in less than half the time allocated for a priority review application. In response to your questions, I described OVRP's BLA review assignment processes. I emphasized that for this particular BLA, we assigned two experienced medical officers to this file who are working closely with the data analytics team in CDER-OCS and three statisticians from CBER/OBE who are supporting these review efforts. I did not emphasize this during our meeting, but you should also know that our typical review process includes frequent formal and informal communications with managers at all levels and other OVRP experts not directly assigned to the review team. I reiterated that adding staff to this review at this advanced stage would likely slow down the review due to the need to bring new people up to speed. You inquired whether we need additional help and also asked about the expertise of MOs assigned to this file noting that there would be staff in FDA, e.g., pediatric cardiologist that could assist in the review. You expressed concern about the rising COVID-cases in the US and globally, largely caused by the Delta variant and stated your opinion that, absent a license, states cannot require mandatory vaccination and that people hesitant to get an EUA authorized vaccine would be more inclined to get immunized when the product is licensed. You emphasized your interest in licensing this vaccine as soon as possible—a goal that we agree with. We too are concerned about the rising COVID-19 cases in the US, however, our concern is that a review that is hyper-accelerated beyond the already very rapid September 15 target date and as a consequence, may be less thorough than our typical review seems more likely to undermine confidence in the vaccine (and, indeed, in FDA's credibility) than to increase it.

You informed us of your decision that OVRP management and oversight of the BLA review will be delegated to Dr. Marks who will provide you with weekly updates on the review process and ensure that due diligence is exercised while I am away [REDACTED]. You also informed me that Dr. Krause will not be involved in the BLA oversight as he will be overseeing other regulatory and programmatic programs in OVRP. I expressed my disagreement with these decisions because standard procedures are for the deputy Office Director to assume an Acting Role when the Office Director is out of the Office. I note that Dr. Krause is a recognized expert in vaccine regulation, development and very familiar with the scientific and clinical issues presented by this specific vaccine product and that the review team relies on his expertise and guidance.

I would also like to emphasize OVRP staff's dedication and experience in promoting public health by making safe and effective vaccines available for use in the United States. Since I believe we all agree in the importance both of a rapid decision and a thorough scientific and credible review, Dr. Krause and the OVRP staff will stand ready to assist in any way possible to achieve both of these goals. Please confirm that this summary reflects your recollection of this meeting. If it does not, I would appreciate your letting me know any specific areas where your recollection is different.

Thank you,
Marion

Marion F. Gruber, Ph.D
Director

Office of Vaccines Research & Review
Center for Biologics Evaluation & Research
Food & Drug Administration, DHHS
10903 New Hampshire Ave.
Building 71, Rm. 3230
Silver Spring, Maryland 20993

Dr. Gruber wrote that she and Dr. Krause remained concerned that:

[A] review that is hyper-accelerated beyond the already very rapid September 15 target date and as a consequence, may be less thorough than

our typical review seems more likely to undermine confidence in the vaccine (and, indeed, in FDA's credibility) than to increase it.¹²³⁰

Dr. Gruber also memorialized Dr. Woodcock's decision to have Dr. Marks take over the review from her and Dr. Krause.

You informed us of your decision that OVRP management and oversight of the BLA review will be delegated to Dr. Marks who will provide you with weekly updates on the review process and ensure that due diligence is exercised while I am away [REDACTED]. You also informed me that Dr. Krause will not be involved in the BLA oversight as he will be overseeing other regulatory and programmatic programs in OVRP. I note that Dr. Krause is a recognized expert in vaccine regulation, development and very familiar with the scientific and clinical issues presented by this specific vaccine product and that the review team relies on his expertise and guidance.¹²³¹

Dr. Woodcock testified that the rationale she gave during the meeting for sidelining Dr. Gruber and Dr. Krause with Dr. Marks centered around the fact that Dr. Gruber was going to be on vacation out of the country and that Dr. Krause was an expert but not a clinician.

Dr. Janet Woodcock (May 13, 2024)

- Q. Do you generally agree with [Dr. Gruber's] summary of the meeting?
- A. She's focusing on the timelines. I did not focus on that in my part of the meeting. I focused on the fact that she would be on vacation, which is perfectly reasonable, some family time, out of the country, not in a position to oversee this very complicated, as she said, review. Dr. Krause is an expert, he's not a clinician, and I asked that Peter take over the review.¹²³²

During a Select Subcommittee hearing on February 15, 2024, Dr. Marks dismissively testified that, in his view, Dr. Gruber and Dr. Krause's concerns were simply related to "the workload."

Dr. Peter Marks (February 15, 2024)

- Q. Do you recall why Dr. Gruber and Dr. Krause expressed concern about accelerating the approval of the vaccine?

¹²³⁰ E-Mail from Marion Gruber, Ph.D., Dir., Office of Vaccines Research & Review, U.S. Food & Drug Admin, to Janet Woodcock, *et al.* (July 21, 2021, 11:59 AM).

¹²³¹ *Id.*

¹²³² Woodcock TI, *supra* note 1171, at 92.

A. They were concerned about the workload.¹²³³

Yet, these explanations seem to leave out the fact that Dr. Gruber and Dr. Krause had become “intransigent” on moving up the ADD any further than September 15, which they explained their expert rationale for in detail, and that neither Dr. Gruber nor Dr. Krause stated they were unable to fulfill their duties.

FINDING: The Biden Administration Accelerated the Approval of Pfizer’s Biologics Approval Application to Impose Vaccine Mandates

Dr. Gruber’s summary of the July 19 meeting also explicitly highlighted the fact that Dr. Marks and Dr. Woodcock indicated that they wanted to move up the target date in part because it was a prerequisite for vaccine mandates to take effect.

You expressed concern about the rising COVID-cases in the US and globally, largely caused by the Delta variant and stated [their] opinion that, absent a license, states cannot require mandatory vaccination and that people hesitant to get an EUA authorized vaccine would be more inclined to do so when the product is licensed.¹²³⁴

When confronted with Dr. Gruber’s meeting summary, Dr. Woodcock testified that she was unable to recall whether vaccine mandates came up at all during their July 19 meeting.

Dr. Janet Woodcock (May 13, 2024)

Q. Did the topic of vaccine mandates or mandatory vaccination policies come up at all during this meeting?

A. Not by me.

Q. But do you recall that someone else may have brought them up?

A. I do not recall.¹²³⁵

However, she was able to recall that at some undetermined time, Dr. Marks had indeed discussed COVID-19 vaccine mandates, as well as the fact that the regulatory status of COVID-19 vaccines could have an impact on the ability for mandates to be levied.

Dr. Janet Woodcock (May 13, 2024)

¹²³³ Accessing America’s Vaccine Safety Systems, Part 1: Hearing Before the Select Subcomm. on the Coronavirus Pandemic, 118th Cong. 2, (Feb. 15, 2024) [hereinafter “Accessing America’s Vaccine Safety Systems, Part 1”].

¹²³⁴ E-Mail from Marion Gruber, Ph.D., Dir., Office of Vaccines Research & Review, U.S. Food & Drug Admin, to Janet Woodcock, *et al.* (July 21, 2021, 11:59 AM).

¹²³⁵ Woodcock TI, *supra* note 1171, at 90.

Q. Did any conversations [with Dr. Marks] ever touch on vaccine mandates or mandatory vaccination policies, broadly?

A. I believe Dr. Marks brought it up a couple times.

Q. A couple of times?

A. Yes.

Q. Do you remember specific times?

A. No.

Q. Did Dr. Marks ever speak to you about the need for full biologics approval in order for groups to institute vaccine mandates?

A. Dr. Marks commented on the fact that mandates for some populations would be tied to their FDA status.¹²³⁶

During his transcribed interview with the House Committee on the Judiciary, Dr. Marks said that he recalled having some discussions about the link between full BLA approval and vaccine mandates, but that he did not recall bringing up vaccine mandates during the July 19 meeting, or who else may have done so.

Dr. Peter Marks (April 15, 2024)

Q. Was it Janet Woodcock or was it you who told Dr. Gruber that [absent a license, states cannot require mandatory vaccination]?

A. I can't speak to – I don't know. And I don't recall that I would have ever said this, but I can't recall, and I can't speak to who said it.

Q. Were you present when this was said?

A. Again, I can't recall it being said. I was something that had – look, the potential for vaccine mandates is something that may have been discussed over the course of time even from early on in – during the pandemic. But I can't recall who would have introduced this to the conversation.¹²³⁷

¹²³⁶ Woodcock TI, *supra* note 1171, at 29.

¹²³⁷ Judiciary Report, *supra* note 1220, at 89.

During the Select Subcommittee’s February 15, 2024 hearing, Dr. Marks testified indecisively that he didn’t recall the specific conversation about vaccine mandates, but that it was probably just a “statement of fact.”

Dr. Peter Marks (February 15, 2024)

- Q. Do you recall any conversations regarding the need to approve the vaccines in order for it to then be mandated?
- A. There was an acknowledgment that an approval could allow vaccine mandates to occur, but they were not conversations over that, that it were –
- Q. So Dr. Gruber wrote that you and Dr. Woodcock expressed your opinion that absent a license States cannot require mandatory vaccination. Do you recall this conversation?
- A. I don't know what you're referring to, but I -- there is probably -- it's just a statement of fact that that once you have a licensed vaccine, a mandate could be placed.¹²³⁸

Contradictorily to Dr. Marks and Dr. Woodcock, during transcribed interviews with the House Committee on the Judiciary, Dr. Gruber and Dr. Krause both testified unambiguously that it was both Dr. Marks and Dr. Woodcock that expressed their opinion that a license was a prerequisite to vaccine mandates.

Dr. Gruber testified:

Dr. Marion Gruber (July 18, 2023)

- Q. Your email is addressed to both Marks and Woodcock, and here you say “you.” Was it Woodcock or was it Marks or was it both of them that expressed their opinion that a license would be needed for vaccine mandates?
- A. It was both of them. Yeah.¹²³⁹

Dr. Krause testified:

Dr. Philip Krause (September 7, 2023)

- Q. Did Dr. Woodcock or Dr. Marks state that the goal was to require mandatory vaccination?

¹²³⁸ Accessing America’s Vaccine Safety Systems, Part 1, *supra* note 1233.

¹²³⁹ Judiciary Report, *supra* note 1220, at 60

A. Well, the, as stated in the email, the discussion was about State mandates, and of course there already had been some mandates announced by the Biden administration. There was a so-called vaccinate-or-test rule that they had put in place at some point earlier in August, as I recall. And so there was no doubt, in my recollection, that Dr. Marks and Dr. Woodcock saw the licensure of the vaccine as a prerequisite to mandates.¹²⁴⁰

Dr. Gruber and Dr. Krause each also testified that making way for vaccine mandates appeared to have played a key role in Dr. Marks' and Dr. Woodcock's persistence in hyper-accelerating the review of the Pfizer BLA:

Dr. Gruber testified:

Dr. Marion Gruber (July 18, 2023)

Q. Were there other reasons they gave, or could you determine why they had such a sense of urgency to get this?

A. The reason given to me was the vaccine mandate. And the second reason that I also mentioned in my email was that people hesitant to take a vaccine that is authorized but not approved may be inclined to take the vaccine if it's licensed. These were the two reasons provided to me.¹²⁴¹

Dr. Krause testified:

Dr. Philip Krause (September 7, 2023)

Q. Now you just said the need to support mandates. In your view, was that need part of why you all were being pressured to meet this deadline, as this assumed date?

A. Given that they brought it up, it's hard to imagine that that was not a component of this pressure.¹²⁴²

It is highly concerning that paving the way for mandatory vaccination policies played any role at all in the FDA's process for approving COVID-19 vaccines, and even worse that it appears to have played a primary role in their rationale for casting aside top vaccine experts and hyper-accelerating the target date. Vaccine mandates are fundamentally highly political matters

¹²⁴⁰ Judiciary Report, *supra* note 1220, TI of Philip Krause, at 288.

¹²⁴¹ Judiciary Report, *supra* note 1220, TI of Marion Gruber, at 131.

¹²⁴² Judiciary Report, *supra* note 1220, TI of Philip Krause, at 295.

that the FDA should be completely insulated from. Dr. Woodcock readily admitted this fact during her transcribed interview.

Dr. Janet Woodcock (May 13, 2024)

Q. Does the FDA have any role in shaping policies like vaccine mandates?

A. No.¹²⁴³

FINDING: U.S. Food and Drug Administration Officials Refused to Rebut Allegations the Biden White House Was Involved in the Pfizer Biologics Approval Application.

On August 9, 2021, after Dr. Marks had taken over the review of the Pfizer BLA but prior to it being completed, Secretary Austin issued a memo which announced his intention to seek President Biden's approval to mandate COVID-19 vaccines for U.S. servicemembers "no later than mid-September, or immediately upon the U.S. Food and Drug Administration (FDA) licensure, whichever comes first."¹²⁴⁴ As indicated by the memo, Secretary Austin was unable to issue a mandate for COVID-19 vaccines without a waiver from President Biden so long as the vaccines remained under EUA status.¹²⁴⁵ The memo implies that Secretary Austin was aware that the FDA had been working with an ADD of September 15, 2021. Additionally, the DOD's press release announcing this memo denotes that "the Food and Drug Administration is expected to give full approval to the vaccines, possibly by the end of the month."¹²⁴⁶

Ultimately, the FDA issued its approval for Pfizer's BLA on August 23, 2021, and the very next day, on August 24, 2021, Secretary Austin issued a memo announcing the DOD's vaccine mandate.¹²⁴⁷ The dubious timing raises concerns about whether Secretary Austin, or any other executive branch official, may have been involved in the hyper-acceleration of the ADD. Dr. Krause testified that he thought it appeared that some decisions that OVRP would normally make were being made elsewhere:

Dr. Philip Krause (September 7, 2023)

I admit that the events in July and August surrounding the BLA and the booster vaccines made me concerned, without direct knowledge of any

¹²⁴³ Woodcock TI, *supra* note 1171, at 96-97

¹²⁴⁴ Memorandum from Lloyd Austin, Sec'y of Defense, to All Dept's of Defense Employees, *Message to the Force* (Aug. 9, 2021).

¹²⁴⁵ Wen. W. Shen, CONG. RESEARCH SERVS., R46745, *State and Federal Authority to Mandate COVID-19 Vaccination* (May 17, 2022) (in the military context, for instance, additional waiver requirements under 10 U.S.C. § 1107a may apply to the administration of medical products subject to EUAs to servicemembers.)

¹²⁴⁶ Jim Garamone, *Biden to Approve Austin's Request to Make COVID-19 Vaccine Mandatory for Service Members*, DOD NEWS (Aug. 9, 2021).

¹²⁴⁷ Memorandum from Lloyd Austin, Sec'y of Defense, to Senior Pentagon Leadership, Commanders of the Combatant Commanders, Defense Agency and DOD Field Activity Directors, *Rescission of August 24, 2021 and November 30, 2021 Coronavirus Disease 2019 Vaccination Requirements for Members of the Armed Forces* (Jan. 10, 2023).

specific outside interference, because I didn't know of any communications from the outside, but it appeared as though major decisions that normally would have been within the purview of the office were now being made outside of the office, whether at the center director's level or even elsewhere.¹²⁴⁸

During the Select Subcommittee's February 15, 2024 hearing, Dr. Marks testified that he had made the decision to pressure Dr. Gruber and Dr. Krause to move up the ADD earlier than September 15th all on his own.

Dr. Peter Marks (February 15, 2024)

Q. Did anyone instruct you, or is this just a decision you made on your own?

A. This was a decision I had made on my own.¹²⁴⁹

Dr. Woodcock denied that she had any foreknowledge of Secretary Austin's plans to issue a vaccine mandate.

Dr. Janet Woodcock (May 13, 2024)

Q. Were you aware that Secretary Austin was planning to mandate COVID 19 vaccination before August 24th, 2021?

A. No.¹²⁵⁰

Dr. Woodcock also denied communicating with anyone at DOD about the Pfizer BLA prior to it being approved.

Dr. Janet Woodcock (May 13, 2024)

Q. Did you communicate with anyone at the Department of Defense prior to the BLA being issued regarding the BLA?

A. No.¹²⁵¹

However, Dr. Woodcock admitted that she did communicate with the White House about the Pfizer BLA.

Dr. Janet Woodcock (May 13, 2024)

¹²⁴⁸ Judiciary Report, *supra* note 1220, Krause TI, at 303.

¹²⁴⁹ Accessing America's Vaccine Safety Systems, Part 1, *supra* note 1233.

¹²⁵⁰ Woodcock TI, *supra* note 1171, at 99.

¹²⁵¹ Woodcock TI, *supra* note 1171, at 68.

Q. Did you communicate with anybody at the White House regarding the Pfizer BLA prior to being issued?

A. Yes.¹²⁵²

When pressed for details, Counsel for the Biden Administration instructed Dr. Woodcock not answer questions about her conversations with the White House regarding the ADD for the Pfizer BLA review as it evolved over time.

Dr. Janet Woodcock (May 13, 2024)

Majority Staff. Did you discuss with the White House the expected ADD as it evolved over time for the Pfizer BLA?

HHS Counsel. Now we're getting more into the substance, and at that point, we're not going to be able to answer.

Majority Staff. You're instructing the witness not to answer?

HHS Counsel. Yes.¹²⁵³

Dr. Woodcock also testified that updates about the timing of an ADD for a vaccine approval are typically shared with the CDC, because they have related actions to take, but not other agencies. She explained that this information is considered to be private.

Dr. Janet Woodcock (May 13, 2024)

Q. So as the ADD gets moved around, are you providing updates to CDC about that?

A. Yes.

Q. Do you typically provide -- or does FDA typically provide updates about the ADD to other agencies besides CDC?

A. FDA typically does not do that, because that's market moving information, right? And we ask anybody that we generally wouldn't tell people that, right? But this relationship with CDC is very special, because they have an action to take as well.

Q. So outside of CDC, it's generally private information?

¹²⁵² Woodcock TI, *supra* note 1171, at 68.

¹²⁵³ *Id.*

A. Absolutely.¹²⁵⁴

Dr. Woodcock acknowledged that despite this information being considered private, it would sometimes be reported publicly in the papers.

Dr. Janet Woodcock (May 13, 2024)

Q. [D]uring the COVID vaccine approval process, was the ADD shared with any other agencies?

A. Not to my -- not by me.

Q. To your knowledge, did somebody else?

A. Well, it often would appear in the papers. But that might have been from the companies, because we tell the company, so they can prepare. They have to -- the company has to do a lot of work. It's a very frenetic activity toward the end of that, as you approach that final date. And so the companies would be told. So it's hard to know who would -- but the dates might start floating around out there. You know, companies are talking about that.¹²⁵⁵

It seems unlikely that neither Dr. Marks nor Dr. Woodcock recall who brought up vaccine mandates during the July 19 meeting, despite recalling many other details about the call and the time around it. It also seems unlikely that Dr. Gruber was mistaken in her very detailed summary—which was sent only two days after the meeting had taken place—and that she and Dr. Krause were both mistaken during their transcribed interviews.

Instead, it appears that Dr. Marks and Dr. Woodcock may have chosen to feign amnesia, or intentionally not answer questions, because they would otherwise be implicated in a pressure campaign to hyper-accelerate the approval of COVID-19 vaccines to facilitate the state and federal vaccine mandates which were predicated on the vaccines' regulatory status. Nonetheless, their testimony directly contradicts that of Dr. Gruber and Dr. Krause and has left unanswered questions about what information the White House or other executive branch agencies were given ahead of the BLA approval.

Ultimately, Dr. Gruber did sign off on the approval of Pfizer's BLA and testified that she believed the vaccine to be safe and effective.¹²⁵⁶ However, this does not excuse the fact that the process that was undertaken was reckless, nor does it undo the public's perception that these vaccines were approved in a hurry to satisfy a political agenda—one that culminated with harmful vaccine mandates which trampled on individual liberty, informed consent, and the doctor-patient relationship. Federal health agencies should have done more to ensure the integrity of the process and preserve the American people's trust in public health and vaccines. In failing

¹²⁵⁴ Woodcock TI, *supra* note 1171, at 25-26.

¹²⁵⁵ See generally, Woodcock TI, *supra* note 1171.

¹²⁵⁶ Judiciary Report, *supra* note 1220, at 120.

to do so, they have likely done irreparable damage to public trust and our pandemic preparedness.

IV. Public Health Officials Disregarded Natural Immunity, Despite Its Proven Effectiveness and Durability

Natural immunity, and infection acquired immunity, is the body's ability to create internal protection against an invading infectious agent.¹²⁵⁷ In the simplest of terms, a protein called an antibody enters the blood stream and helps the body detect and fight the infectious agent if it returns. It is science known and studied since the 1800s.¹²⁵⁸ A common example of this, is varicella, commonly referred to as chickenpox. At least 90 percent of U.S. adults are immune to chickenpox because of having the disease as a child.¹²⁵⁹ In addition to the high rate of infection acquired immunity to the disease, upon license of the vaccine in 1995, the number of cases of chickenpox fell by more than 95 percent.¹²⁶⁰

FINDING: Those Who Recovered From COVID-19 Were Conferred Infection Acquired Immunity.

While the novelty and lack of initial knowledge of COVID-19 cannot be understated, there was established science that infection from coronaviruses, including SARs-CoV-1, produced infection acquired immunity.¹²⁶¹ However, it was not long before global research was presented establishing that COVID-19 did produce a natural immunity after infection.

A study, published in October 2021, examined the rate of COVID-19 reinfection amongst people in Lombardy, Italy during the first wave of infections.¹²⁶² The study suggested reinfection of COVID-19 were rare and those who'd recovered from the virus were at a lower risk of reinfection.¹²⁶³ The results further suggested natural immunity provided a protective effect for approximately a year, similar to the reported vaccine studies.¹²⁶⁴ An additional Italian study from March 2020, followed 36 patients with documented cases of the virus.¹²⁶⁵ These patients were followed until September 2021 and the 17 who remained unvaccinated continued to present IgG antibodies through at least 18 months.¹²⁶⁶ Another February 2020 study out of Qatar, and published in the *New England Journal of Medicine*, concluded previous infection of COVID-19 prevented reinfection of the virus.¹²⁶⁷ A January 2021 study, out of La Jolla Institute for Immunology, found 95 percent of the 200 participants presented durable immune responses for

¹²⁵⁷ Natural Immunity, CLEVELAND CLINIC (last reviewed Aug. 15, 2024).

¹²⁵⁸ Jonathan D. Kaunitz, *The Development of Monoclonal Antibodies*, PUBMED CENTRAL (Apr. 2017).

¹²⁵⁹ *Varicella (Chickenpox): Questions and Answers*, IMMUNIZE.ORG, available at <https://www.immunize.org/wp-content/uploads/catg.d/p4202.pdf>.

¹²⁶⁰ *Id.*

¹²⁶¹ Jonathan D. Kaunitz, *The Development of Monoclonal Antibodies*, PUBMED CENTRAL (Apr. 2017).

¹²⁶² Jose Vitale, et al. *Assessment of SARS-CoV-2 Reinfection 1 Year After Primary Infection in a Population in Lombardy, Italy*, PUBMED CENTRAL (May 2021).

¹²⁶³ *Id.*

¹²⁶⁴ *Id.*

¹²⁶⁵ Sheena Meredith, *COVID-19: Why Are We Ignoring Infection-Acquired Immunity?*, MEDSCAPE (Feb. 28, 2022).

¹²⁶⁶ Puya Dehgani-Mobaraki, et al., *Long-term persistence of IgG antibodies in recovered COVID-19 individuals at 18 months post-infection and the impact of two-dose BNT162b2 (Pfizer-BioNTech) mRNA vaccination on the antibody response: Analysis using fixed-effects linear regression model*, PUBMED (Dec. 6, 2022).

¹²⁶⁷ Heba N. Altarawneh, et al., *Protection against the Omicron Variant from Previous SARS-CoV-2 Infection*, THE NEW ENGLAND JOURNAL OF MEDICINE (Feb. 9, 2022).

up to eight months after infection of COVID-19.¹²⁶⁸ Additional studies out of England and Israel in 2021 found infection rates at equally low levels amongst those who were fully vaccinated and those who previously were infected COVID-19.¹²⁶⁹

On February 16, 2023, more than three years into the pandemic, *The Lancet* published a report confirming the benefits associated with naturally acquired immunity.¹²⁷⁰ The study found that a previous COVID-19 infection offers at least the same level of protection as two doses of a high-quality mRNA vaccine, such as Moderna or Pfizer-BioNTech.¹²⁷¹ Protection from reinfection of the virus may be as high as 40 weeks or longer.¹²⁷² The likelihood of not getting infected again is the strongest with the original (ancestral) strain of the virus, and the alpha, beta, and delta variants, all of which remain at more than 78 percent after 40 weeks.¹²⁷³ The omicron variant is lower, at 36.1 percent.¹²⁷⁴ Protection was high across all variants for severe disease (hospitalization or death).¹²⁷⁵

This study was the first to “comprehensively assess natural immunity protection against COVID-19 reinfection by variant (primary infection and reinfection) and to evaluate waning immunity with time since primary infection.”¹²⁷⁶ The researchers considered the severity of symptoms, the variant of the virus, and how long since the subjects last positive COVID-19 test. *The Lancet* article included 65 studies from 19 countries.

Even with all the global scientific data that those who contracted COVID-19 had some extended time of immunity, the Biden Administration and the CDC began pushing a vaccine-only strategy to population immunity. When the vaccines rolled out, approximately 91 million Americans were previously infected with COVID-19.¹²⁷⁷ Yet any status of recently recovered from the virus, and thus provided with a short time of immunity, was completely ignored.

FINDING: Herd Immunity is a Real Concept and Occurrence Supported by Public Health Leaders, Such as Dr. Fauci, and There Was a Coordinated Effort from Public Health Officials to Ignore Natural Immunity and Suppress Dissenting Opinions.

The very initial phases of the COVID-19 pandemic were understandably riddled with unknowns. Yet, while research and information were gathered, the science supported infection from coronaviruses, including SARS-CoV-1, provided individuals with infection acquired

¹²⁶⁸ Sharon Reynolds, *Lasting Immunity Found After Recovery From COVID-19*, NATIONAL INSTITUTES OF HEALTH (Jan. 26, 2021).

¹²⁶⁹ See, Victoria Hall, et al., *SARS-CoV-2 Infection Rates of Antibody-Positive Compared With Antibody-Negative Health-care Workers in England: a Large, Multicenter, Prospective Cohort Study*, THE LANCET (2021); Yair Goldberg, et al. *Protection of Previous SARS-CoV-2 Infection is Similar to that of BNT162b2 Vaccine Protection: A Three Month Nationwide Experience from Israel*, AMERICAN JOURNAL OF EPIDEMIOLOGY (Apr. 24, 2021).

¹²⁷⁰ Caroline Stein, *Past SARS-CoV-2 infection protection against re-infection: a systematic review and meta-analysis*, THE LANCET (Feb. 16, 2023; updated: Mar. 11, 2023).

¹²⁷¹ *Id.*

¹²⁷² *Id.*

¹²⁷³ *Id.*

¹²⁷⁴ *Id.*

¹²⁷⁵ *Id.*

¹²⁷⁶ *Id.*

¹²⁷⁷ *Id.*

immunity.¹²⁷⁸ Many researchers recognized the existing science and called for policy makers and government officials to use infection acquired immunity from COVID-19 to help control the spread of virus. Many countries around the world allowed a previous COVID-19 infection to satisfy the individuals “vaccination status.”¹²⁷⁹ Yet U.S. public health officials resisted including infection acquired immunity when developing guidance and policy during the pandemic. Despite this resistance, in a transcribed interview, Dr. Fauci testified that natural immunity is a “real thing.”

Dr. Anthony Fauci (January 9, 2024)

Q. I want to talk about natural immunity for a minute. In general, is natural immunity a real thing?

A. It’s a real thing.¹²⁸⁰

The lack of policy conversations was evident when other ideas were proposed, but never discussed. For example, the Great Barrington Declaration was an open letter published in October 2020 in response to mass lockdowns.¹²⁸¹ Originally signed by scientists from the University of Oxford, Stanford University, and Harvard University, the document presented the idea that lockdowns had adverse effects on both the mental and physical health of populations.¹²⁸² It called for a “focused protection” pandemic strategy.¹²⁸³ The authors proposed reducing the harms of lockdown by ending the mandatory restrictions for most people, yet still protecting those most vulnerable.¹²⁸⁴

Critics of this idea immediately disqualified the proposal, to an unsettling degree never seen before in the scientific community. Anyone associated with the paper was immediately categorized as a “fringe” theorist.¹²⁸⁵ Rather than being allowed to engage in scientific and political debate, the alternative proposal was dubbed dangerous and referred to as a “let it rip,” meaning the vast spread of the virus, approach. The Great Barrington Declaration categorically did not advocate for individuals to intentionally get infected with COVID-19 or a “let it rip” approach. It was evident from the early days of the COVID-19 pandemic that public health leadership had little interest in engaging in any form of alternative debate.

¹²⁷⁸ Li-Ping Wu, *et al.*, *Duration of Antibody Responses After Severe Acute Respiratory Syndrome*, PUBMED CENTRAL (Oct. 2007).

¹²⁷⁹ *Id.*

¹²⁸⁰ Fauci TI 2, *supra* note 81, at 209.

¹²⁸¹ Dr. Martin Kulldorff, *et al.*, THE GREAT BARRINGTON DECLARATION (Oct. 4, 2020) [hereinafter “Great Barrington Declaration”].

¹²⁸² *Id.*

¹²⁸³ *Id.*

¹²⁸⁴ *Id.*

¹²⁸⁵ The Editorial Board, *How Fauci and Collins Shut Down Covid Debate*, WALL STREET JOURNAL (Dec. 21, 2021).

In an October 8, 2020, email from Dr. Collins to Dr. Fauci, Dr. Lane, and Dr. Tabak, Dr. Collins wrote that the proposal of a policy other than national lockdowns needed “a quick and devastating published take down of its premise.”¹²⁸⁶

One week after this e-mail, Dr. Collins called The Great Barrington Declaration “a fringe component of epidemiology...that is not mainstream science.”¹²⁸⁷ He also called it outright dangerous.¹²⁸⁸ Following the sharp rebuke from NIH officials, reputable and international media outlets, such as *Wired*, *The Wall Street Journal*, *The Guardian*, *Independent*, and *Telegraph*, amongst almost all others, began dismissing the idea that the Great Barrington Declaration was something to be taken seriously or even discussed.¹²⁸⁹

The Biden Administration was also complicit in attempting to manage the flow of information on COVID-19 in public spaces. In an August 2024 letter to the House Committee on the Judiciary, Mr. Zuckerberg wrote that the White House pressured the company to censor certain posts about COVID-19.¹²⁹⁰

This kind of rhetoric and behavior created a scientific environment that fostered hostility and outright contempt for differing opinions. Scientists and doctors were demonized by colleagues and peers within their own community.¹²⁹¹ Even though it was evident this virus was novel, there was absolutely no room for any kind of high level, scientific debate.

The authors of the Great Barrington Declaration, and any other scientists with a different idea than what was dictated by public health officials, were not demanding their policy ideas become law. They were simply asking that they be allowed to participate in the conversation. Yet infection acquired immunity was never part of the national public health policy during the coronavirus pandemic.

The job of public health officials is to offer the best scientific advice to protect the nation as a whole. Yet during the COVID-19 pandemic, many public health leaders narrowly focused on one mission, to the detriment of others, including the trust of the public. In July 2023 at a panel for Braver Angels, Dr. Collins reflected on his time as a leader during the pandemic.

If you're a public-health person and you're trying to make a decision, you have this very narrow view of what the right decision is, and that is something that will save a life...so you attach infinite value to stopping the disease and saving a life. You attach a zero value to whether this actually totally disrupts people's lives, ruins the economy, and has many kids kept

¹²⁸⁶ E-Mail from Francis Collins, Dir., National Institutes of Health, to Anthony Fauci, *et al.*, Dir., National Institute for Allergy and Infectious Diseases (Oct. 8, 2020, 2:31 PM).

¹²⁸⁷ Joel Achenbach, *Proposal to hasten herd immunity to the coronavirus grabs White House attention but appalls top scientists*, THE WASH. POST (Oct. 14, 2020).

¹²⁸⁸ *Id.*

¹²⁸⁹ See generally, Matt Reynolds, *There is no 'scientific divide' over herd immunity*, WIRED (Oct. 7, 2020).

¹²⁹⁰ Zuckerberg Letter, *supra* note 1123.

¹²⁹¹ Vinay Prasad & Jeffrey S. Flier, *Scientists who express different views on Covid-19 should be heard, not demonized*, STAT (Apr. 27, 2020).

out of school in a way that they never quite recovered is a public-health mindset...another mistake we made.¹²⁹²

At a March 8, 2023 Select Subcommittee hearing, Congressman Ronny Jackson (R-T.X.) asked Dr. Redfield what can be done to gain back to trust of public health officials.

Dr. Robert Redfield (March 8, 2023)

...The second thing, you have to have the courage when you're a public health official to say, I don't know, when you don't know. And I think the really fundamental—that's how we begin to lose it when people say that we're going to be OK once we get 30 percent immunity, and then later they say 50 percent, and then later they say 70 percent. And then the press says, well, why did you say 50 percent before, and now you're saying 70 percent? And the [public health officials] says, well, I didn't think the public was ready to hear that.¹²⁹³

Public health leaders' aggressive exclusion of natural immunity even being considered to be a part of fighting the COVID-19 virus created an environment of hostility in the scientific community that will produce lasting adverse effects. Public health leadership appeared to only push a single agenda and not foster an atmosphere of mutual respect and robust discussion.

¹²⁹² Braver Angels, *supra* note 858; *See also*, Scott Atlas, Steve Hanke, Philip Kerpen, & Casey Mulligan, *COVID Lessons Learned: A Retrospective After Four Years*, COMM. TO UNLEASH PROSPERITY (Mar. 2024).

¹²⁹³ Investigating the Origins of COVID-19, *supra* note 12.

V. Vaccine Mandates Were Not Supported by Science and Caused More Harm than Good

The FDA issued EUAs for the Pfizer-BioNTech and Moderna COVID-19 vaccines in December 2020.¹²⁹⁴ The EUA for the Janssen (Johnson and Johnson) COVID-19 vaccine was issued in February 2021.¹²⁹⁵ By early February, the U.S. had administered approximately 26.5 million vaccinations, one of the fastest rollouts in the world.¹²⁹⁶ By the late February, the doses administered had nearly doubled to 50 million doses administered.¹²⁹⁷

On March 19, 2021, the U.S. administered its 100-millionth vaccine.¹²⁹⁸ By April 18, half of all U.S. adults had received at least one COVID-19 dose,¹²⁹⁹ and on April 19 all U.S. adults became eligible for a vaccine.¹³⁰⁰ In early August 2021, 70 percent of the U.S. population had been vaccinated, including 90 percent of Americans aged 65 and older.¹³⁰¹

In December 2020, President-Elect Biden stated he would not make vaccines mandatory. President Biden and other officials within the administration repeated this promise throughout much of early and mid-2021 as the vaccines were rolled out.¹³⁰² However, the promise was soon broken when, on August 24, 2021, Secretary Austin announced the first federal COVID-19 vaccine mandate—one day after the FDA’s full approval of the Pfizer vaccine. This first COVID-19 vaccine mandate required the secretaries of each Military Department to “immediately begin full vaccination of all members of the Armed Forces under DoD authority.” Secretary Austin also noted that this decision was made “with the support of the President.”¹³⁰³ DOD did not, however, mention or reference specific scientific studies supporting its mandate. Further, Secretary Austin has repeatedly ignored letters from members of Congress regarding making data-driven decisions concerning the health of the military.¹³⁰⁴

After DOD issued its mandate, the floodgates of federal mandates opened. On September 9, 2021, President Biden issued Executive Order 14043, which required federal employees to be vaccinated against COVID-19 by November 8, 2021, or risk removal or termination from their federal employment.¹³⁰⁵ On November 4, 2021, OSHA issued a rule which required all

¹²⁹⁴ *COVID-19 Vaccines*, U.S. DEP’T OF HEALTH & HUMAN SERVICES, available at <https://www.hhs.gov/coronavirus/covid-19-vaccines/index.html>.

¹²⁹⁵ *Id.*

¹²⁹⁶ AJMC Staff, *A Timeline of COVID-19 Vaccine Developments in 2021*, AJMC (Jun. 3, 2021).

¹²⁹⁷ *Biden marks 50M vaccine doses in first 5 weeks in office*, ASSOCIATED PRESS (Feb. 25, 2021).

¹²⁹⁸ Zeke Miller, *Biden eyes new goal after US clears 100M shots since Jan. 20*, ASSOCIATED PRESS (Mar. 19, 2021).

¹²⁹⁹ Jeannette Muhammad, *Global COVID-19 Deaths Top 3 Million*, NATIONAL PUBLIC RADIO (Apr. 17, 2021).

¹³⁰⁰ *Biden marks 50M vaccine doses in first 5 weeks in office*, ASSOCIATED PRESS (Feb. 25, 2021).

¹³⁰¹ AJMC Staff, *What We’re Reading: 70% of US Vaccinated; CMS Payment Rules Released; Alcohol Consumption and Cancer*, AJMC (Aug. 3, 2021).

¹³⁰² Tommy Pigott, *Biden Promised No Mandate. He Lied*, RAPID RESPONSE (Sept. 10, 2021).

¹³⁰³ Memorandum for Senior Pentagon Leadership Commanders of the Combatant Commands Defense Agency and DOD Field Activity Directors, *Mandatory Coronavirus Disease 2019 Vaccination of Department of Defense Service Member* (Aug. 24, 2021).

¹³⁰⁴ Press Release, Congressman Brad Wenstrup, *Wenstrup Demands Substantive Response from DOD After Receiving Four-Sentence Reply* (Feb. 27, 2023).

¹³⁰⁵ *Guidance on Enforcement of Coronavirus Disease 2019 Vaccination Requirements for Federal Employees – Executive Order 14043*, U.S. OFFICE OF PERSONNEL MANAGEMENT; See, Exec. Order No. 14043, 86 Fed. Reg. 50985 (Sept. 14, 2021).

employers with 100 or more employees to impose COVID-19 vaccine mandates, and CMS announced a COVID-19 vaccine mandate for all healthcare workers who worked at facilities participating in Medicare and Medicaid.¹³⁰⁶ Finally, on November 30, 2021, HHS announced a rule which required COVID-19 vaccination for all Head Start staff, contractors and volunteers.

Although all five of the major federal COVID-19 vaccine mandates are now rescinded, overturned, or otherwise ended, their consequences will live on for years to come. The COVID-19 vaccine mandates caused people to lose their livelihoods, hollowed out our healthcare and education workforces, reduced our military readiness and recruitment, caused vaccine hesitancy, reduced trust in public health, trampled individual freedoms, deepened political divisions, and interfered in the patient-physician relationship.

Aside from these severe consequences, the scientific basis of the COVID-19 vaccine mandates was highly questionable. COVID-19 mandates ignored natural immunity, stratification of risk from the virus, risk of adverse events from the vaccine, as well as the fact that the vaccines don't prevent the spread of COVID-19. Meanwhile, as the Select Subcommittee has established in previous hearings, federal health officials consistently overstated the power of the vaccines and deepening political divides with statements like "the pandemic of the unvaccinated."¹³⁰⁷ This is yet another example of the Biden Administration's guidance and policies straying far outside boundaries of the available evidence while proudly proclaiming they were "following the science."¹³⁰⁸ For them, "because I told you so" was good enough.

DOD Mandate

Secretary Austin issued the DOD COVID-19 vaccine mandate on August 24, 2021¹³⁰⁹, and it remained in effect until he rescinded it on January 10, 2023¹³¹⁰, as required by the National Defense Authorization Act (NDAA) for FY2023.¹³¹¹ Over the course of the 16 months that the COVID-19 vaccine mandate was in effect, approximately 8,000 service members were separated due to their COVID-19 vaccination status.¹³¹² However, more than 17,500 troops' religious exemptions were still being adjudicated just prior to the rescission of the COVID-19 vaccine mandate.¹³¹³ Some have also argued that the NDAA language did not go far enough to ameliorate the harms done by the COVID-19 vaccine mandate since separated servicemembers

¹³⁰⁶ Release, White House, *Fact Sheet: Biden Administration Announces Details of Two Major Vaccination Policies* (Nov. 4, 2021).

¹³⁰⁷ President Joe Biden, *Remarks by President Biden on Fighting the COVID-19 Pandemic* (Sept. 9, 2021).

¹³⁰⁸ President Joe Biden, *Statement by President Joe Biden on CDC Guidance* (July 27, 2021).

¹³⁰⁹ Release, White House, *Fact Sheet: Biden Administration Announces Details of Two Major Vaccination Policies* (Nov. 4, 2021).

¹³¹⁰ *DOD Rescinds COVID-19 Vaccination Mandate*, U.S. DEP'T OF DEFENSE (Jan. 10, 2023).

¹³¹¹ James M. Inhofe National Defense Authorization Act for Fiscal Year 2023, Pub. L. No. 117-263, Stat. 2395 (2022).

¹³¹² Oren Liebermann, *Only 43 of more than 8,000 discharged from US military for refusing Covid vaccine have rejoined*, CNN (Oct. 2, 2023).

¹³¹³ Steve Beynon, *Thousands of Troops with COVID Vaccine Exemption Requests No Longer Facing Separation With Mandate Gone*, MILITARY TIMES (Jan. 4, 2023).

would not be automatically reinstated or paid back-pay.¹³¹⁴ The Select Subcommittee agrees that members should be reinstated with back-pay.

Federal Workforce Mandate

On September 9, 2021, President Biden issued Executive Order 14043 which mandated all 3.5 million Federal employees get a COVID-19 vaccine by November 22, 2021.¹³¹⁵ In the press release issued by the White House, President Biden stated that “[t]he health and safety of the Federal workforce, and the health and safety of members of the public with whom they interact, are foundational to the efficiency of the civil service” and that “[the COVID-19 vaccines] protect people from getting infected and severely ill, and they significantly reduce the likelihood of hospitalization and death.” The Biden Administration formally ended this mandate (along with the CMS mandate) at the end of the public health emergency on May 11, 2023.¹³¹⁶

OSHA Mandate

On November 4, 2021, the Biden Administration announced that it would be using OSHA to require all businesses with more than 100 employees to impose COVID-19 vaccine mandates.¹³¹⁷ Under this Emergency Temporary Standard [hereinafter “ETS”], employees would be required to be vaccinated or face weekly testing. Companies would also have to give workers paid time off to receive vaccines and to recover from any adverse effects. Companies that did not comply would face up to \$14,000 in fines per violation.¹³¹⁸

This was met with immediate questions about whether OSHA had the authority to impose such a requirement. On November 9, 2021, the National Federation of Independent Businesses [hereinafter “NFIB”] filed a lawsuit against OSHA regarding this ETS.¹³¹⁹ Shortly thereafter, on November 12, 2021, the Fifth Circuit Court of Appeals issued a stay.¹³²⁰ On December 17, 2021, the Sixth Circuit Court of Appeals vacated the Fifth Circuit Court’s stay, which allowed the ETS to be implemented. On January 13, 2022, the Supreme Court sided with NFIB and blocked OSHA from enforcing the ETS only three days after it took effect. However, many employers had already implemented COVID-19 vaccine mandates by this point.

¹³¹⁴ Press Release, Ted Cruz, U.S. Senator for Texas, Sen. Cruz Introduces Updated Legislation Building on Vaccine Mandate Ban to Reinstatement Service Members Fired Over COVID Vaccine (Jan. 24, 2023).

¹³¹⁵ President Joe Biden, *Remarks by President Biden on Fighting the COVID-19 Pandemic* (Sept. 9, 2021).

¹³¹⁶ Release, White House, *The Biden-Harris Administration Will End COVID-19 Vaccination Requirements for Federal Employees, Contractors, International Travelers, Head Start Educators, and CMS-Certified Facilities* (May 1, 2023).

¹³¹⁷ Release, U.S. Dep’t of Defense, *DOD Rescinds COVID-19 Vaccination Mandate* (Jan. 10, 2023).

¹³¹⁸ Press Briefing by Mr. Munoz, *Background Press Call on OSHA and CMS Rules for Vaccination in the Workplace*, White House Briefing Room (Nov. 4, 2021).

¹³¹⁹ *NFIB Files Lawsuit Against OSHA’s Vaccine Mandate on America’s Businesses* (Nov. 9, 2021).

¹³²⁰ *BST Holdings LLC, et al. v. OSHA, et al.* No. 21-60845 (5th Cir. 2021).

CMS Mandate

In the same November 4, 2021 announcement of the OSHA COVID-19 vaccine mandate, the Biden Administration simultaneously announced that it would be proceeding with a COVID-19 vaccine mandate for all health care workers who worked at facilities that participate in Medicare and Medicaid. Due to the incredibly large scale of these two federal programs, this mandate covered approximately 10 million people and around 76,000 healthcare providers.¹³²¹ This was among the most consequential COVID-19 vaccine mandates due to the significant number of individuals within its scope, the fact that it remained in effect until May 2023, and because of the damage it did to the health care workforce in the middle of a pandemic. Many healthcare workers voluntarily quit or were fired for not complying with the mandate.¹³²²

Head Start Mandate

On November 30, 2021, the Office of Head Start—the early education federal program within HHS—announced an interim final rule [hereinafter “IFR”] imposing both vaccination and masking requirements for grant recipients of the program.¹³²³ Specifically, the IFR mandated the COVID-19 vaccine for all staff, volunteers, and contractors, and universal masking for all individuals two years of age or older.¹³²⁴ Prior to May 11, 2023—the formal end of the COVID-19 public health emergency—the Biden Administration had been prohibited from enforcing the Head Start mandate by a federal district court judge on September 21, 2022.¹³²⁵

Other Jurisdictions:

The Biden Administration’s federal COVID-19 vaccine mandates directly imposed vaccination requirements on more than 12 million Americans, but even individuals not within the scope of these mandates were likely to come across another entity imposing one.¹³²⁶ Many of these entities may have even been encouraged to do so because of the federal COVID-19 vaccine mandates, even if not compelled to do so. States, counties, municipalities, schools, employers, restaurants, airlines, gyms, entertainment venues and many others across the country, imposed COVID-19 vaccine mandates.

In response to the OSHA mandate, many private companies pushed to implement their own vaccine policies. After the FDA hurriedly granted full approval for the Pfizer vaccine on August 23, 2021, these companies, amongst others, announced some form of vaccination plan: American Express, Amtrak, Cisco, Citigroup, CVS Health, Deloitte, Delta Airlines, DoorDash, Equinox, Facebook, Ford, General Electric, Goldman Sachs, Google, Lyft, McDonalds, MGM Resorts International, Microsoft, Morgan Stanley, NBCUniversal, Netflix, The New York Times, Saks, Southwest Airlines, Twitter, Uber, United Airlines, Walgreens, The Walt Disney

¹³²¹ Tom Hals, *Judge blocks U.S. COVID-19 vaccine rule for health workers in 10 states*, REUTERS (Nov. 29, 2021).

¹³²² Dave Muoio, *How many employees have hospitals lost to vaccine mandates? Here are the numbers so far*, FIERCE HEALTHCARE (Feb. 22, 2022).

¹³²³ 86 Fed. Reg. 68,052 (Nov. 30, 2021).

¹³²⁴ *Id.*

¹³²⁵ *Louisiana v. Becerra*, 3:21-CV-04370 (W.D. La. Sep. 21, 2022).

¹³²⁶ *See Where 12 Million U.S. Employees Are Affected by Government Vaccine Mandates*, THE N.Y. TIMES (Dec. 18, 2021).

Company, Walmart, and The Washington Post.¹³²⁷ Individuals who refused to comply with these COVID-19 vaccine mandates were fired from their jobs and many more people opted to quit instead.¹³²⁸

College campuses across the country were another area significantly impacted by COVID-19 vaccine mandates. Prior to the COVID-19 vaccine rollout, college students had already been robbed of the traditional college experience and doors were opened in piecemeal and inconsistent ways. However, the implementation of COVID-19 vaccine mandates across campuses brought new disruptions. There were still many campuses in 2023 that required students to receive a COVID-19 vaccine and boosters, even though booster shots have been shown to be unnecessary for younger people¹³²⁹ and that the risk for myocarditis is higher in young men.¹³³⁰

The negative effects brought by COVID-19 lockdowns has been widely reported, and for some, the so-called “vaccine passport” requirements became a new de-facto lockdown. For example, in Washington DC, bars, gyms, concert venues and other businesses were forced to require proof of COVID-19 vaccination for their patrons.¹³³¹ Even after the COVID-19 vaccine mandate was dropped,¹³³² many businesses elected to keep their vaccination requirement in effect.¹³³³ A similar story played out in other jurisdictions across the country.

FINDING: COVID-19 Vaccine Mandates Caused Massive Collateral Damage and Were Very Likely Counterproductive.

Vaccines alone, and therefore COVID-19 vaccine mandates, could not and did not bring us to “herd immunity.”¹³³⁴ Yet, they caused collateral damage that has been felt by millions of Americans.

A May 2022 *British Medical Journal* [hereinafter “BMJ”] *Global Health* paper written by Dr. Bardosh and several other public health and bioethics experts from around the world found that COVID-19 vaccine mandates caused significant collateral damage. The paper’s summary stated:

¹³²⁷ Haley Messenger, *From McDonald’s to Goldman Sachs, here are the companies mandating vaccines for all or some employees*, NBCNEWS (Aug. 3, 2021).

¹³²⁸ Andrea Hsu, *Thousands of Workers are Opting to Get Fired, Rather than take the Vaccine*, NATIONAL PUBLIC RADIO (Oct. 24, 2021).

¹³²⁹ Berkeley Lovelace Jr., *Younger, health people don’t need another COVID booster, vaccine expert says*, NBCNEWS (Jan. 11, 2023).

¹³³⁰ Berkeley Lovelace, Jr., *Small study points to possible cause of myocarditis following mRNA vaccination in young men*, NBCNEWS (May 5, 2023).

¹³³¹ Karina Elwood & Fritz Hahn, *Ready your vaccination cards: DC businesses prepare to enforce new mandate*, THE WASH. POST (Jan. 14, 2022).

¹³³² Tori Bergel, *DC Is Ending Its Vaccination and Mask Mandates*, WASHINGTONIAN, (Feb. 14, 2022).

¹³³³ Tierney Plumb, *Some D.C. Restaurants and Bars Refuse to Stop Asking for Customers’ Vax Status*, EATER (Feb. 16, 2022).

¹³³⁴ Christie Aschwanden, *Five reasons why COVID herd immunity is probably impossible*, NATURE (Mar. 18, 2021).

Our analysis strongly suggests that mandatory COVID-19 vaccine policies have had damaging effects on public trust, vaccine confidence, political polarization, human rights, inequities and social wellbeing. We question the effectiveness and consequences of coercive vaccination policy in pandemic response and urge the public health community and policymakers to return to non-discriminatory, trust-based public health approaches.¹³³⁵

The BMJ paper also found that COVID-19 vaccine mandates primarily served to encourage vaccination in younger people who were the least at-risk for serious COVID-19 illness, and that this further entrenched distrust and provoked reactance:

Although studies suggest that current policies are likely to increase population-level vaccination rates to some degree, gains were largest in those under 30 years old (a very low-risk group) and in countries with below average uptake. Moreover, insights from behavioral psychology suggest that these policies are likely to entrench distrust and provoke *reactance*—a motivation to counter an unreasonable threat to one’s freedom.¹³³⁶

Dr. Bardosh testified before the Select Subcommittee during a July 27, 2023 hearing titled “Because I Said So: Examining the Science and Impact of COVID-19 Vaccine Mandates.” Dr. Bardosh was able to further expand on these findings on COVID-19 vaccine mandates.

Dr. Kevin Bardosh (July 27, 2023)

Q. Dr. Bardosh, let’s amplify some of the points from your publications. There was a collaborative effort that you published with researchers and physicians from Johns Hopkins, Harvard, and Oxford, yes or no?

A. Yes.

Q. Did the COVID vaccine mandates from your research, from your publication with others, erode civil liberties?

A. Yes, it did.

Q. Did the COVID vaccine mandates fracture trust in public health officials?

A. Yes, it did.

¹³³⁵ Bardosh, The unintended consequences of COVID-19 vaccine policy, *supra* note 891.

¹³³⁶ *Id.*

Q. Did the COVID vaccine mandates create financial stress to individuals and families who lost their jobs to the COVID mandates?

A. Absolutely.

Q. And Dr. Bardosh, do you feel that the decrease in individuals receiving routine pediatric immunizations for their children, do you feel that is due to the mandates of the COVID vaccine?

A. Yes, I do.

Q. And, finally, and I thank you for your brevity. Dr. Bardosh, do you feel that the COVID-19 vaccine mandates have harmed America?

A. Yes, I do.¹³³⁷

This collateral damage may have hampered our preparedness for a future pandemic. Dr. Bardosh testified that COVID-19 vaccine mandates decreased public confidence in vaccines and would likely be responsible for resistance to vaccines during a future pandemic.

Dr. Kevin Bardosh (July 27, 2023)

Q. Dr. Bardosh, your paper in the British Medical Journal of Global Health discusses the unintended consequences of the COVID-19 vaccine mandates. The Biden administration imposed several, including the DOD mandate for military service members, the executive order mandate for federal employees and contractors, the OSHA mandate for employers with 100 or more employees, the CMS mandate for health care workers at facilities that participate in Medicare and Medicaid, and the HHS Head Start Program COVID-19 vaccine mandate for which we know young children are at the least risk. Can you highlight the ramifications of these mandates, such as no jab, no job policies, vaccine passports and social lockdowns for the unvaccinated?

A. There is no doubt in my mind that these mandate policies are going to be responsible for the increase in distrust the next time there's a pandemic and the mobilization of resistance to a future vaccine in a future pandemic. And I think it's really shocking, and kind of a little bit sad, that my colleagues in the public health community, who are pro-mandate, don't understand this.¹³³⁸

¹³³⁷ Because I Said So: Examining the Science and Impact of COVID-19 Vaccine Mandates: Hearing Before the Select Subcomm. on the Coronavirus Pandemic, 118th Cong. 1, (July 27, 2023) [hereinafter "Because I Said So"].

¹³³⁸ *Id.*

As Dr. Bardosh highlighted in the BMJ paper, vaccine mandates appear to have also deepened political divisions. Rhetoric from some politicians and public health officials painted the vaccine mandates as a necessary policy to combat the selfish, ignorant, or malevolent “unvaccinated” who were perpetuating the pandemic. Americans wanted to be honestly educated about both the virus and the vaccines, not indoctrinated. During the Select Subcommittee’s July 27, 2021 hearing, Dr. Bardosh testified that this rhetoric was a “scapegoating response,” which evidence indicates may be counterproductive as a public health strategy.

Dr. Kevin Bardosh (July 27, 2023)

- Q. Dr. Bardosh, in your paper, you mentioned that political leaders singled out the unvaccinated and blamed them for the continuation of the pandemic. In fact, in July 2021, during the onset of the Delta variant wave, CDC Director Rochelle Walensky said that it was “becoming a pandemic of the unvaccinated.” Is this the sort of blame, Dr. Bardosh, that you were referring to in your paper?
- A. Yes, it is a scapegoating response.
- A. Dr. Bardosh, I am going to make this simpler. Do you believe that this sort of language is harmful when we look for public officials in charge to scapegoat?
- A. I think, you know, the HIV/AIDS community has done a lot of research on stigma and scapegoating, right? And, you know, decades of research has shown that stigma as a public health strategy is counterproductive. I will leave it at that.¹³³⁹

In a December 2022 interview with Fox 5 New York, Dr. Anthony Fauci appeared to admit this fact, saying “[I] would like people to use good judgment to protect themselves and their family in that community without necessarily having to mandate anything, because, you know, there is a fatigue about being mandated. People don't like to be told what to do.”¹³⁴⁰ Yet, this was too-little too-late. Dr. Fauci himself had already made extremely inflammatory remarks about the unvaccinated, including during an interview for an audiobook by journalist Michael Specter, where he patronizingly deemed concerns about not getting vaccinated as simply “ideological bullshit,” and implied that institutions should make life difficult for the unvaccinated using vaccine mandates:

Once people feel empowered and protected legally, you were going to have schools, universities, and colleges are going to say; “you want to come to this college, buddy? You're going to get vaccinated. Lady, you're going to get vaccinated.” Big corporations like Amazon and Facebook and all of those others are going to say; “you want to work for us, you get vaccinated.”

¹³³⁹ Vaccine mandates hearing transcript page 30

¹³⁴⁰ Julia Musto, *Fauci acknowledges Americans have mandate 'fatigue': 'People don't like to be told what to do'*, FOX NEWS (Dec. 10, 2022).

And it's been proven that when you make it difficult for people in their lives, they lose their ideological bullshit, and they get vaccinated.¹³⁴¹

During the Select Subcommittee's June 3, 2024 hearing, Dr. Fauci did acknowledge in hindsight that not all objections to COVID-19 vaccines were ideological bullshit.

Dr. Anthony Fauci (June 3, 2024)

Q. Are all objections to COVID vaccinations ideological bullshit, Dr. Fauci?

A. No, they're not.¹³⁴²

Some also argued that COVID-19 vaccine mandates would be self-defeating to their stated goal of ending the pandemic. In June 2021, Psychologist Katrin Schmelz and Economist Samuel Bowles co-wrote a *Washington Post* article titled; "Imposing vaccine mandates may be counterproductive, our research suggests."¹³⁴³ Schmelz and Bowles wrote that their research suggested it could "hurt voluntary compliance, prolonging the pandemic and raising its social costs."¹³⁴⁴ Specifically, their representative panel survey in Germany showed that:

[M]aking vaccination a legal requirement might have retarded the rate of vaccinations, as it would have substantially reduced willingness to be vaccinated, consistent with self-determination and reactance theory in psychology and what economists' term "control aversion."¹³⁴⁵

Vaccine mandates appear to have also contributed to a surge in vaccine hesitancy and overall distrust of public health authorities. For example, a recent CDC study showed that immunization rates for kindergarteners fell each year since the start of the pandemic.¹³⁴⁶ This trend could prove to be problematic for vaccination efforts during a future pandemic.

COVID-19 vaccine mandates also forced millions of people to choose between their livelihoods and being vaccinated—even if they had closely held personal or religious beliefs or a medical reason. This is not only unjust, but it also caused thousands of people to lose their jobs in the middle of a pandemic and unstable economic environment.¹³⁴⁷

During the Select Subcommittee's July 27, 2023 hearing, Ms. Allison Williams testified about losing her job as an ESPN sports reporter after she sought an exemption to Disney's

¹³⁴¹ Michael Specter, Fauci (Puskin 2020) (audio).

¹³⁴² Fauci Hearing, *supra* note 231, at 97

¹³⁴³ Katrin Schmelz & Samuel Bowles, *Imposing vaccine mandates may be counterproductive, our research suggests*, THE WASH. POST (June 7, 2021).

¹³⁴⁴ *Id.*

¹³⁴⁵ *Id.*

¹³⁴⁶ Raneer Seither, *et al.*, *Coverage with Selected Vaccines and Exemption Rates Among Children in Kindergarten — United States, 2023–24 School Year*, MMWR (Oct. 17, 2024).

¹³⁴⁷ *Id.*

COVID-19 vaccine mandates. Ms. Williams told the Select Subcommittee that at the time of the mandate she was actively working with fertility specialists while she and her husband were trying for their second child.¹³⁴⁸ Her doctor supported her decision to forgo the vaccine due to her young age, good health, and ongoing efforts to conceive a child.¹³⁴⁹ Nonetheless, Ms. Williams employment was terminated.¹³⁵⁰

Ms. Allison Williams (July 27, 2023)

Just like that, newly pregnant, I was stripped of my job, my health insurance and having my personal and medical decisions the topic of national news. It is hard to explain what it is like to have so much taken from you for doing what you know in your heart and mind to be the right thing for you and your family. The financial toll it took on my family and so many others like us was significant, and still enduring. The lost wages and sacrifices made by families like mine who stood up to the overreaching, unjustified mandates to preserve their autonomy and health can never be fully recovered.¹³⁵¹

Worse still, one of the most impacted sectors, due to the CMS COVID-19 vaccine mandate, was our healthcare work force. The very same people heralded as “heroes” in 2020 were soon being fired for noncompliance in 2021.¹³⁵² This notion is absurd on its face, but it is taken to another level given the fact that the healthcare workforce, particularly in the nursing field, was at crisis-level shortages across the country during this time.

In a December 2022 article, the AP highlighted this absurd hypocrisy and noted that “foundations are pouring millions of dollars into efforts to ensure that more stay in the [nursing] profession.”¹³⁵³ The AP also cited an April 2022 study published by *Health Affairs* which showed that more than 100,000 nurses, or 1.8 percent of the nationwide work force, left in 2021.¹³⁵⁴ Many of these potential consequences were known prior to the federal COVID-19 vaccine mandates and were being widely discussed throughout the media, yet the Biden Administration continued anyway.¹³⁵⁵ Sadly, many of the individuals who were fired or left their jobs likely acquired natural immunity from previous infection and this immunity may have been greater than immunity acquired through vaccination.

¹³⁴⁸ Because I Said So: Examining the Science and Impact of COVID-19 Vaccine Mandates: Hearing Before the Select Subcomm. on the Coronavirus Pandemic, 118th Cong. 1, (July 27, 2023) [hereinafter “Because I Said So”].

¹³⁴⁹ *Id.*

¹³⁵⁰ *Id.* at 30.

¹³⁵¹ *Id.*

¹³⁵² Dave Muoio, *How many employees have hospitals lost to vaccine mandates? Here are the numbers so far*, FIERCE HEALTHCARE (Feb. 22, 2022); Honoring Our Public Heroes Who Protect Us All, CDC Foundation, available at <https://www.cdcfoundation.org/hero#:~:text=A%20hero%20is%20every%20public,essential%20health%20and%20wellbeing%20services>.

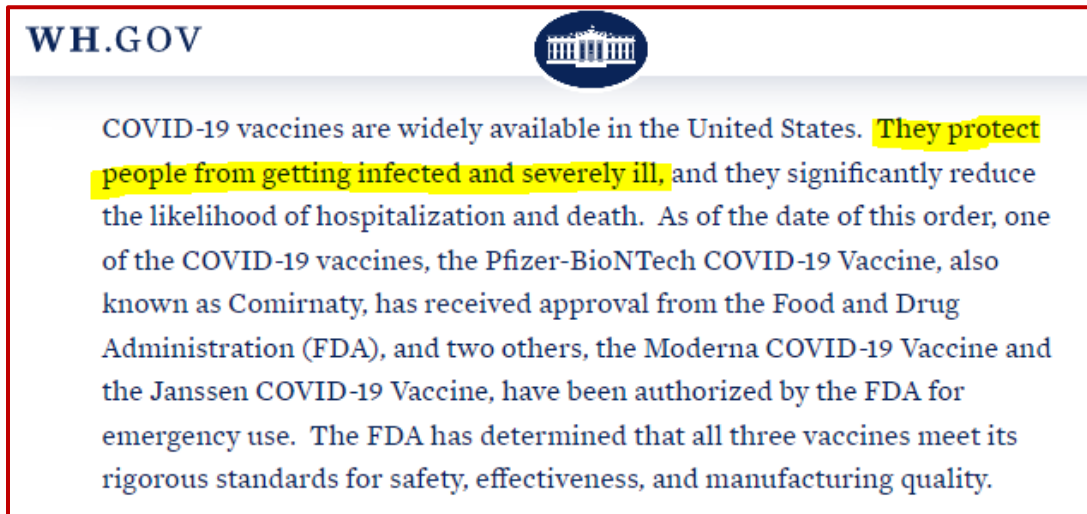
¹³⁵³ Alex Daniels, *Foundations, major donors tackle nation’s nursing shortage*, ASSOCIATE PRESS (Dec. 5, 2022).

¹³⁵⁴ David I. Auerbach, *et al.*, *A Worrisome Drop In The Numbers Of Young Nurses*, HEALTH AFFAIRS FOREFRONT (Apr. 13, 2022).

¹³⁵⁵ Taylor Dotson & Nicholas Tampio, *Vaccine mandates will backfire. People will resist even more*, THE WASH. POST (July 31, 2021).

FINDING: COVID-19 Vaccine Mandates Were Not Supported by Science.

Not only did COVID-19 vaccine mandates cause many unintended consequences, but they were also not based in science. In President Biden's September 9, 2021, announcement of the federal workforce mandate, he cited the notion that the vaccines protect against infection as the first example for why the mandate was necessary.¹³⁵⁶



However, it was already evident then and is now commonly known that the vaccines do not prevent you from getting infected or transmitting the virus.¹³⁵⁷ This seems to invalidate the most basic logic of a vaccine mandate. As noted by Dr. Vinay Prasad, Professor in the Department of Epidemiology and Biostatistics at the University of California San Francisco, “medical mandates are impermissible if they do not provide benefit to third parties.”¹³⁵⁸

The COVID-19 vaccine mandates also largely ignored the notion of naturally acquired immunity. By the standard of most COVID-19 vaccine mandates, there was no exception made for those who had previously contracted the virus. Rather, the more common exception was to allow weekly testing in place of being fully vaccinated. Chairman Wenstrup has direct experience with the disregard for natural immunity acquired from previous infection. The Chairman was vaccinated with the two-shot Pfizer vaccine. Six months later, he got COVID with mild symptoms.¹³⁵⁹ A House healthcare provider told the Chairman he needed a booster shot prior to going on official travel.¹³⁶⁰ Concerned about a hyperimmune response because of his recent recovery from infection, the Chairman asked for his antibody numbers—a strong number is 40, but the Chairman's number was 821.¹³⁶¹

¹³⁵⁶ President Joe Biden, *Remarks by President Biden on Fighting the COVID-19 Pandemic* (Sept. 9, 2021).

¹³⁵⁷ Umair Irfan, *Some Vaccinated People Have Gotten COVID-19, That's No Reason to Panic*, VOX, (July 15, 2021).

¹³⁵⁸ Vinay Prasad (@VPrasadMDMPH), Twitter, (May 13, 2023, 1:46 PM) available at <https://twitter.com/VPrasadMDMPH/status/1657442159904038913>.

¹³⁵⁹ Because I Said So, *supra* note 1348, at 42.

¹³⁶⁰ *Id.*

¹³⁶¹ *Id.*

This appears to fly in the face of decades of scientific research. While COVID-19 was a novel virus, there was clear science that infections from coronaviruses, including SARS-CoV-1, produced natural immunity.¹³⁶² In 2021, once research began to emerge specifically about SARS-CoV-2, this notion was further supported. A study published in October 2021, using data from Italy, showed that re-infections were rare, and that protection lasted around one year.¹³⁶³ Further studies concurred with this, and some even directly questioned the logic of COVID-19 vaccine mandates.¹³⁶⁴

In February 2023, a study was published in *The Lancet* that showed that natural immunity provides the same protection as two doses of an mRNA vaccine.¹³⁶⁵ Specifically, it showed that infection-acquired immunity cut the risk of hospitalization and death by 88 percent for at least 10 months.¹³⁶⁶ This study was the first to “comprehensively assess natural immunity protection against COVID-19 reinfection by variant (primary infection and reinfection) and to evaluate waning immunity with time since primary infection.”¹³⁶⁷

Relatedly, the COVID-19 vaccine mandates applied a one-size-fits-all approach to medicine which seriously undermined the patient-physician relationship. The mandates decreased the doctor’s decision-space to make individualized risk-based assessments to determine the proper course of action. This meant that regardless of previous infection from COVID-19, previous adverse reactions to vaccines, likelihood to suffer an adverse effect from the COVID-19 vaccine, or risk of serious illness from COVID-19 – all individuals were seen the same by COVID-19 vaccine mandates.

FINDING: COVID-19 Vaccine Mandates Hampered U.S. Military Readiness.

According to reports, only 43 of the more than 8,000 separated servicemembers rejoined the military.¹³⁶⁸ The DOD COVID-19 vaccine mandate directly led to the separation of thousands of US servicemembers, but it has also hindered the military’s ability to recruit. In fiscal year 2022, the military missed its recruiting goal by 15,000 personnel—25 percent of its target.¹³⁶⁹

¹³⁶² Li-Ping Wu, *et al. Duration of Antibody Responses After Severe Acute Respiratory Syndrome* PUBMED CENTRAL (Oct. 13, 2007).

¹³⁶³ Jose Vitale, *et al. Assessment of SARS-CoV-2 Reinfection 1 Year After Primary Infection in a Population in Lombardy, Italy*, PUBMED CENTRAL (May 28, 2021).

¹³⁶⁴ Sheena Meredith, *COVID-19: Why Are We Ignoring Infection-Acquired Immunity?*, MEDSCAPE (Feb. 28, 2022).

¹³⁶⁵ Akshay Syal, *Immunity acquired from a COVID infection is as protective as vaccination against severe illness and death, study finds*, NBCNEWS (Feb. 16, 2023).

¹³⁶⁶ Caroline Stein, *et al., Past SARS-CoV-2 infection protection against re-infection: a systematic review and meta-analysis*, THE LANCET (Mar. 11, 2023).

¹³⁶⁷ Steven Lim, *et al. Past SARS-CoV-2 Infection Protection Against Re-Infection: A Systematic Review and Meta-Analysis*, THE LANCET (Feb. 16, 2023).

¹³⁶⁸ SEE OREN IBERMAN CNN ARTICLE

¹³⁶⁹ David Barno & Nora Bensahel, *Addressing the U.S. Military Recruiting Crisis*, WAR ON THE ROCKS (Mar. 10, 2023).

While it is true that the recruiting shortfalls are likely also driven by other factors such as the increasing ineligibility of young people, low unemployment, and decreasing confidence in the military, there is little question that the COVID-19 vaccine mandate added to this problem. Plus, these other factors have also been exacerbated by our response to COVID-19 inside and outside of the military. For example, lockdowns and school closures certainly did little to help with the crisis of military eligibility in young people.¹³⁷⁰

Some military leaders have agreed that the COVID-19 vaccine mandate negatively impacted recruitment and military readiness. In August 2022, Major General James O. Eifert of the Florida National Guard penned an op-ed in the Wall Street Journal titled *The Vaccine Mandate Puts National Security at Risk*. Maj. Gen. Eifert wrote:

I've never been more worried about the future of the U.S. armed forces than I am right now. I say that as a concerned citizen who has served for more than 40 years, the last three of which have been as the adjutant general of the Florida National Guard. One of the military's most foundational duties is to recruit and retain men and women willing to defend their country. Unfortunately, current federal policy is rendering that goal unattainable.¹³⁷¹

Eifert goes on to argue that COVID-19 itself had not hurt his units' readiness, but rather the military's policy responses had. He also said he agreed with mandating the vaccine in the military at first but that "the circumstances have changed" and that the vaccines efficacy "appears to be shorter-lived than once thought."¹³⁷²

Similarly, in December 2022 during the Reagan Foundation Defense Forum, the now former Marine Corps Commandant Gen. David Berger conceded that the COVID-19 vaccine mandate was "for sure" hurting military recruitment efforts.¹³⁷³

Overall, it is highly concerning that the DOD elected to issue this controversial and sweeping mandate at a time when it was falling well short of recruiting goals. Also concerning is the questionable necessity of mandating the vaccine for such a young and healthy cohort of individuals who simultaneously faced higher risk of adverse events from the vaccine.¹³⁷⁴ Our nation's adversaries stood back and watched as we weakened our own military readiness due to this misguided policy.

¹³⁷⁰ Dave Philipps, *With Few Able and Fewer Willing, U.S. Military Can't Find Recruits*, THE N.Y. TIMES, (July 14, 2022).

¹³⁷¹ James O. Eifert, *The Vaccine Mandate Puts National Security at Risk*, THE WALL ST. JOURNAL (Aug. 4, 2022).

¹³⁷² *Id.*

¹³⁷³ Michelle Lee, *Top Marine general says COVID vaccine mandate is hurting military recruiting efforts*, FOX NEWS, (Dec. 5, 2022).

¹³⁷⁴ *Id.*

VI. The COVID-19 Vaccine, While Largely Safe and Effective, Had Adverse Events That Must be Thoroughly Investigated

According to the WHO, pharmacovigilance consists of “the science and activities relating to the detection, assessment, understanding and prevention of adverse effects or any other medicine/vaccine related problem.”¹³⁷⁵ Federal health agencies participate in a variety of interwoven pharmacovigilance efforts, including both passive and active surveillance.

Passive surveillance is the collection of unsolicited reports of adverse events that are sent to a centralized database.¹³⁷⁶ The Federal Government’s preeminent passive surveillance system is the VAERS.¹³⁷⁷

Active surveillance involves proactively gathering and analyzing data to verify signals found via passive surveillance, or to detect new ones.¹³⁷⁸ The Federal Government’s primary avenues for active surveillance are FDA CBER Sentinel BEST System, CMS data, and CDC’s VSD and V-Safe systems.¹³⁷⁹

The COVID-19 pandemic exposed some potential shortcomings and issues with these systems. For example, in mid-2021, concerns arose that CDC and FDA were not able to successfully utilize their surveillance systems to quickly identify increased risk of myocarditis in young males receiving the Pfizer COVID-19 vaccine, that other countries, including Israel and France, were able to recognize.¹³⁸⁰ Specifically, on May 17, 2021, CDC reported that “rates of myocarditis reports in the window following COVID-19 vaccination have not differed from expected baseline rates.”¹³⁸¹ One week later, and in the wake of reports from Israel’s Ministry of Health, CDC reversed their stance by saying the rates were “higher than expected.”¹³⁸²

FINDING: The Vaccine Adverse Event Reporting System is Insufficient and Not Transparent.

HHS describes VAERS as “a national early warning system to detect possible safety problems in U.S.-licensed vaccines. VAERS is co-managed by the CDC and the FDA.”¹³⁸³ VAERS is operated as a publicly available database which contains millions of reports which have been submitted by individuals. Importantly, healthcare professionals and vaccine manufacturers are specifically *required* to report adverse events that occur after a vaccination to VAERS, but anyone can report an adverse event.

¹³⁷⁵ *Regulation and Prequalification: What is Pharmacovigilance?*, WORLD HEALTH ORG.

¹³⁷⁶ *COVID-19 Vaccine Safety Surveillance*, U.S. FOOD & DRUG ADMIN. (Dec. 7, 2021).

¹³⁷⁷ U.S. Dept. of Health and Human Services, *Vaccine Adverse Event Reporting System (VAERS)*, U.S. DEP’T OF HEALTH & HUMAN SERVICES (last visited Nov. 14, 2024).

¹³⁷⁸ CDC: *Advisory Committee on Immunization Practices, COVID-19 VaST Work Group Report* (May 17, 2021).

¹³⁷⁹ *Id.*

¹³⁸⁰ *Id.*

¹³⁸¹ *Id.*

¹³⁸² Elizabeth Cohen, *A link between COVID-19 vaccination and a cardiac illness may be getting clearer*, CNN (Jun. 10, 2021).

¹³⁸³ *About VAERS*, U.S. DEP’T OF HEALTH & HUMAN SERVICES available at <https://vaers.hhs.gov/about.html>.

Although the system has been operational since 1990, it first became a source of significant controversy with the rollout of COVID-19 vaccines beginning in late 2020 and early 2021. During this time, many posts circulated on social media calling attention to alarming numbers of deaths and adverse events associated with COVID-19 vaccines.¹³⁸⁴ These posts were soon met with a barrage of fact-check articles refuting them.¹³⁸⁵ The chart below presents data from VAERS as of November 2024.¹³⁸⁶

Worldwide VAERS Reports for COVID-19 Vaccines as of 11/30/2024¹³⁸⁷	
Total Adverse Events	1,844,839
Hospitalizations	216,646
Permanent Disabilities	72,161
Deaths	38,068
Deaths Within Two Days of Vaccination	9,167

Possibly the most alarming figures are the comparisons between COVID-19 vaccines, which have only been widely available since early 2021, and all other vaccines combined since 1990. The charts below illustrate this comparison.¹³⁸⁸

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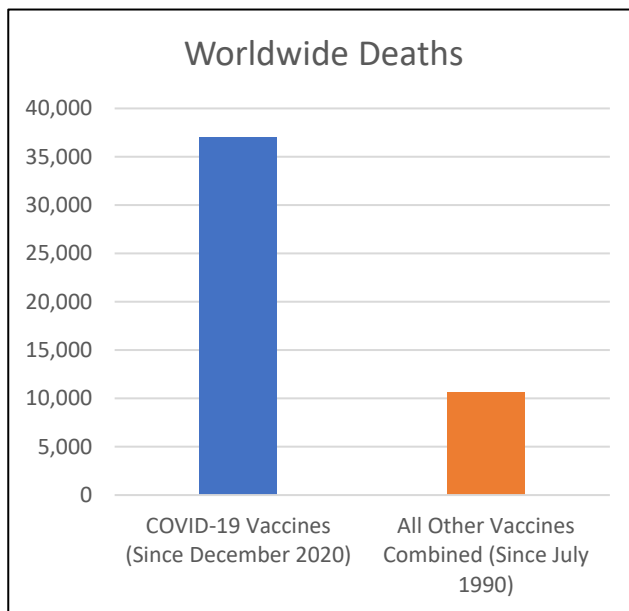
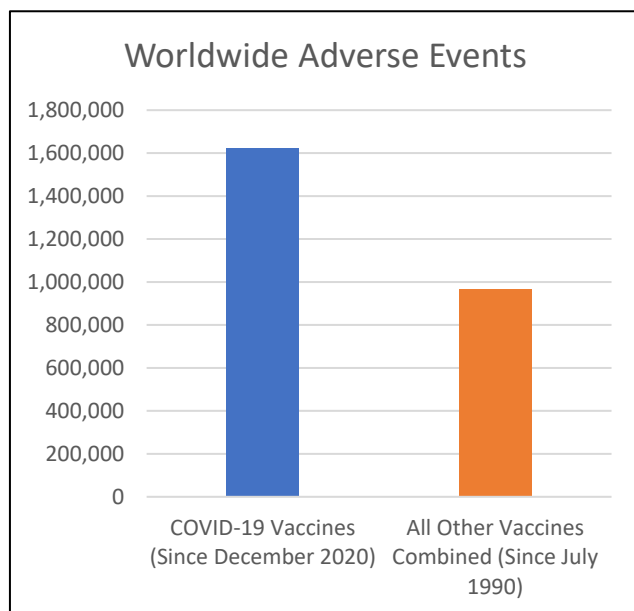
¹³⁸⁴ See generally, *Posts continue to misrepresent VAERS COVID-19 vaccine data*, ASSOCIATED PRESS (Sept. 23, 2022); Reuters Fact Check, *VAERS data does not suggest COVID-19 vaccines killed 150,000 people, as analysis claims*, REUTERS (Oct. 4, 2021); Catalina Jaramillo, *Viral Posts Misuse VAERS Data to Make False Claims About COVID-19 Vaccines*, FACTCHECK.ORG (Mar. 31, 2021).

¹³⁸⁵ See generally, *Posts continue to misrepresent VAERS COVID-19 vaccine data*, ASSOCIATED PRESS (Sept. 23, 2022); Reuters Fact Check, *VAERS data does not suggest COVID-19 vaccines killed 150,000 people, as analysis claims*, REUTERS (Oct. 4, 2021); Catalina Jaramillo, *Viral Posts Misuse VAERS Data to Make False Claims About COVID-19 Vaccines*, FACTCHECK.ORG (Mar. 31, 2021).

¹³⁸⁶ CDC WONDER On-line Database available at <http://wonder.cdc.gov/vaers.html> (last visited Nov 30, 2024).

¹³⁸⁷ *Id.*

¹³⁸⁸ *Id.*



As previously noted, anyone can submit a report to VAERS, and these reports are automatically published and available publicly. Therefore, a report on VAERS has likely not been proven to be caused by a vaccine. However, the vast discrepancy when comparing COVID-19 vaccines over three years, with all other vaccines over more than 30 years raises serious concerns. Pursuant to the EUA for COVID-19 vaccines, manufacturers and providers were required to report serious adverse events to VAERS irrespective of proof of attribution.¹³⁸⁹

An investigation conducted by the BMJ raised concerns that VAERS “isn’t operating as intended and that signals are being missed.”¹³⁹⁰ The BMJ found several troubling issues, including that the system may be severely understaffed in the face of unprecedented number of new reports being filed in the wake of the massive campaign to administer COVID-19 vaccines.¹³⁹¹

Specifically, the BMJ reports that “VAERS’s staffing was likely not commensurate with the demands of reviewing the serious reports submitted, including reports of death,” and that “Pfizer has around 1,000 more full time employees working on vaccine surveillance than the CDC.”¹³⁹² The Journal highlighted that other countries have acknowledged deaths that were “likely” or “probably” caused by COVID-19 vaccines, but CDC has only acknowledged deaths “causally” caused by COVID-19 vaccines—which may be sign that the system is severely overwhelmed.¹³⁹³

¹³⁸⁹ 86 Fed. Reg. 54111 (Jan. 19, 2021),

¹³⁹⁰ Jennifer Block, *Is the US’s Vaccine Adverse Event Reporting System broken?* BMJ (Nov. 10, 2023).

¹³⁹¹ *Id.*

¹³⁹² *Id.*

¹³⁹³ *Id.*

The BMJ also found that VAERS representatives were inconsistent at following up on reports made to the system.¹³⁹⁴ The BMJ spoke to more than 12 people who filed serious reports to VAERS and were “never contacted or were contacted months later.”¹³⁹⁵ This is made even worse since, as BMJ reported, the public database only contains preliminary reports, and all updates and corrections are housed on a “separate, back-end system” which is unavailable to patients, doctors, and other public users of the database.¹³⁹⁶

The BMJ spoke with was Dr. Whelan. Dr. Whelan testified at a Select Subcommittee Hearing on March 21, 2024, where he explained that VAERS fails to cultivate trust.

Dr. Patrick Whelan (March 21, 2024)

Q. Dr. Whelan, do you believe that it is appropriate to house all of the potential updates and corrections to VAERS on a non-public data base?

A. I think that there is a lot of trust involved in the medical community, but also for the general public, that something is happening behind the scenes when you can't see it right out in front. And I think, two, that people were very aware that you did not have the kind of follow up on vaccination generally that we expect, for instance, from our pediatricians, where you got a nurse who is going to call the following week and make sure that your child is doing OK. And, I mean, my own strong feeling is that we really needed a much more proactive surveillance mechanism, and I think that many of us understand that the FDA was under enormous pressure and also that it was an overwhelming task. And just judging by the number of VAERS reports that there have been and knowing how challenging it can be to actually file a VAERS report, the task could have been even vastly larger than the large task that it already is. I mean, you have to have some level, I think, of internal dialog that takes place. But I think ultimately, as you alluded earlier, you have to be able to create some level of trust among people, and I think the system currently does not cultivate that.¹³⁹⁷

During an interview about VAERS, vaccine expert and member of the CDC's ACIP, Dr. Paul Offit, told a story of a colleague who, to prove a point, successfully submitted a VAERS report saying that “he got a vaccine and he turned into the Incredible Hulk.”¹³⁹⁸ Surprisingly, Director Walensky testified before the Select Subcommittee in June 2023 and said, “we at CDC have a responsibility to comb through every single one of them [VAERS reports] to review the

¹³⁹⁴ *Id.*

¹³⁹⁵ *Id.*

¹³⁹⁶ *Id.*

¹³⁹⁷ Assessing America's Vaccine Safety Systems Part II: Hearing Before Select Subcomm. on the Coronavirus Pandemic, 118th Cong. 2 (Mar. 21, 2024).

¹³⁹⁸ MicrobeTV, *Beyond the Noise #26: VAERS*, YouTube (Jan. 16, 2024).

medical charts and to see if they are related.”¹³⁹⁹ Per the BMJ investigation, it appears that these updates and corrections may not be reflected in the public version of VAERS.

All this controversy raises questions about whether VAERS is a sufficient or effective surveillance system as currently operated.

FINDING: Existing Vaccine Safety Systems May Be Missing Important Safety Signals, Especially Related to Neurological Conditions.

In May 2024, *The New York Times* published an article which discussed the issue of COVID-19 vaccine injuries and the fact that many of those who have experienced one feel they have been ignored.¹⁴⁰⁰ The article focused on neurological issues that people believed were caused by the COVID-19 vaccine but have limited scientific evidence. For example, the article contained stories of individuals who had tinnitus after receiving a COVID-19 vaccine, including the Editor-in-Chief of the journal *Vaccine*, and a doctor who led several COVID-19 vaccine trials at Vanderbilt University.¹⁴⁰¹

Further, in the article Dr. Woodcock, expressed regret about the way that the FDA handled vaccine injuries during the pandemic.

I believe their suffering should be acknowledged, that they have real problems, and they should be taken seriously.

I’m disappointed in myself. I did a lot of things I feel very good about, but this is one of the few things I feel I just didn’t bring it home.¹⁴⁰²

Dr. Woodcock explained that she believes these neurological symptoms are difficult to establish causality for because they don’t have well-defined research definitions.

I mean, you’re not going to find ‘brain fog’ in the medical record or claims data.¹⁴⁰³

During her transcribed interview with the Select Subcommittee, Dr. Woodcock expanded on her statements to *The New York Times*. Dr. Woodcock testified that this difficulty with finding causality for neurological conditions existed before the COVID-19 pandemic because it is difficult to neatly put neurological symptoms into any diagnostic category.

¹³⁹⁹ Oversight of CDC Policies and Decisions During the COVID-19 Pandemic: *supra* note 1139 (statement of Dr. Rochelle Walensky, Dir., U.S. Ctrs. for Disease Control and Prevention).

¹⁴⁰⁰ Apoorva Mandavilli, *Thousands Believe Covid Vaccines Harmed Them. Is Anyone Listening?*, THE N.Y. TIMES (May 3, 2024, updated May 4, 2024).

¹⁴⁰¹ *Id.*

¹⁴⁰² *Id.*

¹⁴⁰³ *Id.*

Dr. Janet Woodcock (May 13, 2024)

- Q. The article indicates that you believe that some recipients of COVID vaccines "experienced uncommon but 'serious' and 'life changing' reactions beyond those described by federal agencies." What kinds of reactions are you referring to there?
- A. I'm referring to reactions that medical science has trouble dealing with. A common problem that occurred before this, before COVID used to be called chronic fatigue syndrome or myalgic encephalitis. And the medical establishment has struggled for 20 years trying to figure out what it is and still have no idea. All right. That's typically a post viral or post infectious illness. However, I think it could occur post any immune stimulus. So to answer your question, folks had brain fog, fatigue, prostration, some of them had neurologic symptoms. None of them fit neatly into any diagnostic category.
- Q. So because they didn't fit neatly inside a diagnostic category, you're saying they're harder to be described or analyzed?
- A. Well, they're harder to be identified because many of these folk struggled for months and months to even be acknowledged that there was anything wrong with them. Many of them were told, you know, they were just – It's all in your head. And that's very similar to chronic fatigue syndrome, myalgic encephalitis.¹⁴⁰⁴

Dr. Woodcock also testified that while Acting FDA Commissioner she had pushed for the NIH to establish a research arm to study these neuropathies' possible association with COVID-19 vaccines, so that the people experiencing them could get acknowledged and also to start exploring possible treatments.

Dr. Janet Woodcock (May 13, 2024)

- A. I even talked to NIH, I talked to even see if they would add an arm, because this is very similar to some of the things that people get with long COVID. You get-- people get long COVID much more frequently after getting COVID than they do getting this after vaccination. But it does seem to happen. So I wanted to get it studied because I think what the first thing we need is study. And the NIH study on long COVID is looking for syndromic definitions. That's mainly what they're doing, they're trying to find syndromic clusters so they can name these conditions in some the same way that people get POTS, postural orthostatic tachycardic syndrome, which is another probably autonomic neuropathy, okay? So my goal

¹⁴⁰⁴ Woodcock TI, *supra* note 1171, at 110.

was to try and, number one, get the people studied so they can be acknowledged, that they were having a problem, and also start working up ideas of treatment. Now, I wasn't really that hopeful because CFS ME has gone so long without effective interventions.¹⁴⁰⁵

Dr. Woodcock explained that these efforts ultimately stalled, apparently due to a lack of funding.¹⁴⁰⁶ However, she also indicated that she believed others at the FDA did not appear to have taken this issue as seriously as her.

Dr. Janet Woodcock (May 13, 2024)

Q. Why do you think that that stalled?

A. Well, I had too many things to do. And I think the main reason is without a signal, you know, like we get a lot of signals in our real, like I was telling you earlier. You have to work them up and they aren't causally related. That requires some strong causality – potentially causally related signals hardly get the companies to pay for it. They would have had to pay for a study like that at NIH.

Q. So do you feel that others within FDA took this as seriously as you did?

A. No.¹⁴⁰⁷

It therefore appears that the government must do more to prioritize research into these conditions for the sake of those experiencing them, but also to preserve public trust in vaccination going forward.

FINDING: The U.S. Centers for Disease Control and Prevention Created a New Surveillance System Specifically for COVID-19 but has not been Fully Transparent in Sharing the Data Collected in it.

Launched in December 2020, V-Safe is an active surveillance system which specifically monitors the safety of COVID-19 vaccines.¹⁴⁰⁸ It prompts enrollees for health check-ins via text messages and web surveys whereby individuals report on post-vaccination experiences.¹⁴⁰⁹

¹⁴⁰⁵ Woodcock TI, *supra* note 1171, at 112.

¹⁴⁰⁶ Woodcock TI, *supra* note 1171, at 114-115

¹⁴⁰⁷ Woodcock TI, *supra* note 1171, at 115.

¹⁴⁰⁸ *V-safe Landing Page*, U.S. CTRS. FOR DISEASE AND CONTROL, available at

https://vsafe.cdc.gov/vsafeportal/s/login/?language=en_US&ec=302&startURL=%2Fvsafeportal%2Fs%2F

¹⁴⁰⁹ *What is V-safe?* U.S. CTRS. FOR DISEASE AND CONTROL; Tanya R. Myers, *et al.*, *The v-safe after vaccination health checker: Active vaccine safety monitoring during CDC's COVID-19 pandemic response*, VACCINE (Jan. 23, 2023).

According to CDC, 10.1 million V-Safe participants completed more than 151 million surveys about their health experiences after receiving COVID-19 and mpox vaccines.¹⁴¹⁰

V-Safe is subject to a substantial amount of litigation. Specifically, CDC is defending against FOIA lawsuits filed by groups seeking data and millions of free-text responses gathered through the program.¹⁴¹¹ ICAN obtained and released the “checkbox” data in October 2022.¹⁴¹² ICAN also created a public dashboard which highlights the data, which they say contains “numerous alarming results.”¹⁴¹³ Specifically, ICAN reports that the data show 782,913 individuals, or more than 7.7 percent of users, reported a health event requiring medical attention, emergency room intervention, and/or hospitalization.¹⁴¹⁴

CDC is resistant to providing the free-text entries and have cited concerns that their release would be too burdensome, but a January 2024 court decision required the CDC release them over the course of the next 12 months.¹⁴¹⁵ The plaintiffs claim the CDC “design[ed] V-Safe to assure harms are hidden in free-text fields,” and that analysis of this data will allow “the rate of and adverse reaction [to] be calculated” which is “not possible with VAERS.”¹⁴¹⁶ If a system like V-Safe would be better equipped to actually calculate the rate of a particular adverse event, it is concerning that public health agencies have not leveraged this approach fully.

¹⁴¹⁰ *About V-safe,*? U.S. CTRS. FOR DISEASE AND CONTROL

¹⁴¹¹ Jenna Greene, *New data is out on COVID vaccine injury claims. What's to make of it?* REUTERS (Oct. 12, 2022).

¹⁴¹² Press Release, Informed Consent Action Network, *Breaking News: ICAN Obtains CDC V-safe Data* (Oct. 3, 2022).

¹⁴¹³ *Id.*

¹⁴¹⁴ *Id.*

¹⁴¹⁵ *Freedom Coalition of Doctors for Choice v. Centers for Disease Control and Prevention*, 2:23-cv-00102-Z, (N.D. Tex. Jan. 5, 2024); Greg Piper, *Judge orders CDC to quickly turn over millions of COVID vaccine-injury reports by early patients*, JUST THE NEWS (Jan. 13, 2024).

¹⁴¹⁶ Aaron Siri, *V-Safe Part 4: CDC Designs V-Safe to Assure Harms Are Hidden in Free-Text Fields So It Can Control What Becomes Public, Including Limiting the Harms Submitted to VAERS*, INJECTING FREEDOM (Jan. 10, 2023).

VII. The U.S. Government's Insufficient Systems for Compensating COVID-19 Vaccine Injuries

Generally, vaccines are safe and effective. However, it is inevitable that some individuals will experience adverse events, sometimes serious, life-altering, or life-threatening. This is true for practically any pharmaceutical product and COVID-19 vaccines are no exception. Since vaccines are an important tool to protect public health, it is imperative to preserve the public's trust in vaccination, and therefore compensating for these rare but inevitable harms is of vital importance. Conversely, limiting the liability of manufacturers also promotes the expeditious development of new and innovative vaccine technology which saves lives.

Furthermore, the modern American legal system has long provided tools for proper compensation for injuries of all types. While this type of litigation is often misunderstood and sometimes abused, the system provides a crucial tool for people who have suffered harm to be made whole financially, emotionally, and otherwise. Vaccine injuries are no exception, and therefore modern societies have an obligation to ensure that there are proper systems in place to provide accountability, justice, and financial support for those who experience an injury attributable to a vaccination.

In pursuit of these goals, Congress created the VICP as part of the National Childhood Vaccine Injury Act of 1986.¹⁴¹⁷ VICP covers all vaccines which are recommended for routine administration to children and/or pregnant women by the CDC.¹⁴¹⁸ This list currently consists of 16 individual vaccines.¹⁴¹⁹ VICP allows individuals to file a petition for compensation, and these petitions are then adjudicated by the Office of Special Masters [hereinafter "OSM"] which falls within the U.S. Court of Federal Claims.¹⁴²⁰ Awards are paid out from the Vaccine Injury Compensation Trust Fund which gets its funding from a \$0.75 excise tax on each covered vaccine dose.¹⁴²¹

In 2005, Congress passed the PREP Act which contains provisions designed to promote the rapid development of vaccines in the case of a public health emergency, including the CICI.¹⁴²² The CICI has a more limited scope than the VICP and provides compensation only for "covered countermeasures," including vaccines.¹⁴²³ Because the COVID-19 vaccines were purchased and distributed by the federal government under PREP Act authority, any serious injuries caused by them are compensated via CICI rather than VICP. The PREP Act also limits

¹⁴¹⁷ 42 U.S.C. § 300aa-10, et seq.

¹⁴¹⁸ Comparison of Countermeasures Injury Compensation Program (CICI) to the National Vaccine Injury Compensation Program (VICP), HEALTH RESOURCES & SERVICES ADMIN., *available at* <https://www.hrsa.gov/cicp/cicp-vicp>.

¹⁴¹⁹ *Id.*

¹⁴²⁰ Hannah-Alise Rogers, CONG. RESEARCH SERVS., IF 12213, *The National Vaccine Injury Compensation Program and the Office of Special Masters* (Sept. 14, 2022).

¹⁴²¹ *Id.*

¹⁴²² 42 U.S.C. § 247-6d(a)–(b).

¹⁴²³ *Countermeasure Injury Compensation Program (CICI)*, HEALTH RESOURCES & SERVICES ADMIN. *available at* <https://www.hrsa.gov/cicp>.

the liability of the vaccine manufacturers by providing this separate process for adjudicating claims and shielding the manufacturers from lawsuits.¹⁴²⁴

Below is a comparison table between the two programs which is featured HRSA's CICP webpage and has been simplified here.¹⁴²⁵

	CICP	VICP
Filing Deadlines	• One (1) year filing deadline • Filing deadline when a countermeasures injury table is developed or changed	• Injury claim filing deadline • Death claim filing deadline • Filing deadline when changes are made to the Vaccine Injury Table
Process for Adding Covered Vaccines/ Countermeasures	Covered countermeasures are identified by the Secretary of HHS in declarations published under the PREP Act.	For a category of vaccines to be covered by the VICP, the category of vaccines must be recommended for routine administration to children and/or pregnant women by the Centers for Disease Control and Prevention, subject to an excise tax by federal law, and added to the Vaccine Injury Table by the Secretary of Health and Human Services. This has not been done for any U.S. licensed COVID-19 vaccines, which have not been developed to date.
Type of Injury Covered	Serious physical injuries, Deaths	Injuries with effects lasting for more than 6 months after the vaccine was given or resulted in inpatient hospitalization and surgery, or Deaths
Payment of Legal Fees and Costs	Attorneys' fees and costs are not paid by the program.	Attorneys' fees and costs may be available if certain requirements are met (petition filed in good faith and on a reasonable basis)
Process for Filing a Request/Petition	File the Request Form and documentation with the Secretary of HHS.	File petition and documentation with the U.S. Court of Federal Claims and the Secretary of HHS.
Process for Resolving Requests/ Petitions	Administrative Process	Judicial Process
Covered Injury Determinations	HHS makes decision.	Special Masters (or judges) of U.S. of Court of Federal Claims make decision.

¹⁴²⁴ CRS, The National Vaccine Injury Compensation Program and the Office of Special Masters (Sept. 14, 2022).

¹⁴²⁵ Comparison of Countermeasures Injury Compensation Program (CICP) to the National Vaccine Injury Compensation Program (VICP), HEALTH RESOURCES & SERVICES ADMIN. *available at* <https://www.hrsa.gov/cicp/cicp-vicp>.

	CICP	VICP
Appeal Rights	One-step administrative reconsideration possible. No judicial appeal permitted.	Judicial appeal by either party to higher courts possible.
Program Funding	Appropriated Funds	Vaccine Injury Compensation Trust Fund

FINDING: The U.S. Government is Failing to Efficiently, Fairly, and Transparently Adjudicate Claims for COVID-19 Vaccine Injuries.

Because of CICP's limited resources, adjudication of claims is a lengthy process. HRSA reported that 10,226 of the 13,356 total COVID-19 claims are currently pending or are in review.¹⁴²⁶

CICP Claims Data for COVID-19 Vaccines	
Claims filed	13,520
Claims pending or in review	10,082
Decisions	3,438
Claims compensated	18
Claims denied	3,373

These figures indicate that only 25 percent of all COVID-19 countermeasure claims have been adjudicated so far, with a denial rate of more than 98 percent. Worse still, the design differences also mean that it is much more difficult to be compensated via CICP and the payouts are significantly smaller. As of November 30, 2024, CICP only compensated 18 of the 65 COVID-19 claims it determined eligible for compensation, and the average award was only about \$24,514. This also includes one extreme outlier award of more than \$370,000, thus the median award is only \$4,207. On the other hand, the average VICP payout over the last 35 years is approximately \$468,000.¹⁴²⁷ The chart below highlights each of the compensated CICP claims for COVID-19 vaccines.¹⁴²⁸

Countermeasure	Injury	Compensation Amount
COVID-19 Vaccine	Myopericarditis	\$8,962
COVID-19 Vaccine	Myocarditis	\$4,990
COVID-19 Vaccine	Myocarditis	\$4,230
COVID-19 Vaccine	Myopericarditis	\$4,183
COVID-19 Vaccine	Myocarditis	\$1,583

¹⁴²⁶ *Countermeasures Injury Compensation Program (CICP) Data: Aggregate*, HEALTH RESOURCES & SERVICES ADMINISTRATION (Aug. 1, 2024) available at <https://www.hrsa.gov/cicp/cicp-data>.

¹⁴²⁷ *National Vaccine Injury Compensation Program: Monthly Statistics Report*, HEALTH RESOURCES & SERVICES ADMINISTRATION (Last updated Nov 1, 2024) available at <https://www.hrsa.gov/vaccine-compensation/data>.

¹⁴²⁸ *Countermeasures Injury Compensation Program (CICP) Data: Aggregate*, HEALTH RESOURCES & SERVICES ADMINISTRATION (Aug. 1, 2024) available at <https://www.hrsa.gov/cicp/cicp-data>.

COVID-19 Vaccine	Myocarditis	\$1,033
COVID-19 Vaccine	Myocarditis	\$12,403
COVID-19 Vaccine	Anaphylaxis	\$2,020
COVID-19 Vaccine	Myopericarditis	\$3,958
COVID-19 Vaccine	Myocarditis	\$4,919
COVID-19 Vaccine	Myocarditis	\$1,900
COVID-19 Vaccine	Myocarditis	\$4,934
COVID-19 Vaccine	Myocarditis	\$5,392
COVID-19 Vaccine	Myocarditis	\$370,376
COVID-19 Vaccine	Syncope	\$4,493
COVID-19 Vaccine	Myocarditis	\$1,171
COVID-19 Vaccine	Myocarditis	\$1,161
COVID-19 Vaccine	Guillain-Barre Syndrome	\$3,546

As indicated above, claims made under VICP are adjudicated via a judicial process in the US Court of Federal Claims. Meanwhile, claims made under the CICI are adjudicated by an administrative process that is managed by HRSA. The details of each CICI decision are not made public which means that the rationale of each case's compensation award or decision are indiscernible to the public and to other claimants in the program. This opacity means that the rationale of each case's compensation award or decision are indiscernible to the public and to other claimants in the program. For example, the CICI's rationale to award \$370,376 for one myocarditis claim but \$1,033 for another is unclear. While it is likely that the higher award was for a death claim, that is not explicitly noted, nor is it clear how that number was determined when the maximum award for a death claim is \$437,503 for fiscal year 2024.¹⁴²⁹

The nature of CICI being a HRSA administrative process also means that all decision-making authority ultimately lies with the HHS Secretary. Meanwhile, HHS acts as a fervent promotor of vaccines in general, including the COVID-19 vaccine. This situation calls into question whether countermeasures which were mandated by the government can be fairly adjudicated by an executive branch agency that is inexorably connected to such vaccine policy. This arrangement poses an apparent conflict of interest and may undermine public trust in the compensation process and in vaccines broadly.

The Select Subcommittee heard from several claimants to CICI who shared their personal experiences with the program. Mr. Cody Flint, a commercial agriculture pilot from Mississippi, explained how he experienced a severe adverse reaction to the Pfizer COVID-19 vaccine within 30 minutes of receiving it in February 2021. Mr. Flint was diagnosed with left

¹⁴²⁹ Countermeasure Injury Compensation Program, *Request for Benefits Form Instructions* (last updated Apr. 23, 2023) available at <https://www.hrsa.gov/sites/default/files/hrsa/cicp/cicp-request-form-instructions.pdf>; Benefits by the Year, Public Safety Officers Benefits Program available at <https://bja.ojp.gov/program/psob/resources/benefits-by-year#:~:text=Contact%20Us&text=The%20amount%20of%20the%20PSOB,October%201%2C%202023%20is%20%241%2C488.00> (PSOB indicates the FY2024 maximum is \$473,503).

and right perilymphatic fistulas due to increased intracranial pressure and was unable to working as a pilot as a result.¹⁴³⁰

Mr. Flint submitted a claim to CICIP in April 2021 and did not receive any communication until Senator Cindy Hyde-Smith (R-M.S.) brought up his case to HHS Secretary Becerra during a Senate Appropriations Committee hearing on May 4, 2022.¹⁴³¹ Two weeks after the hearing, Mr. Flint received a denial letter for his claim.¹⁴³² The denial letter from HRSA indicated that CICIP was not aware of any links between the Pfizer COVID-19 vaccine and intracranial pressure.¹⁴³³ Mr. Flint immediately submitted a reconsideration package which included a letter from his surgeon, but he received another denial letter a few months later. Unfortunately, unlike the VICP, the CICIP's design does not permit judicial appeal.¹⁴³⁴ Therefore, individuals like Mr. Flint have little recourse if their claim is denied.

This is just one example, but many other people's experiences tell a similar story – that the program does not appear to be working sufficiently to achieve its aims. According to Ms. Renée Gentry, director of The George Washington University's Vaccine Injury Litigation Clinic who has represented numerous claimants, CICIP provides “little more than the right to file and lose.”¹⁴³⁵ If the government wishes to absolve manufacturers of liability and take over the role that the courts would typically provide, then it must facilitate a fair and robust process.

FINDING: The Countermeasures Injury Compensation Program Failed to Handle a Mass-Vaccination Program.

The CICIP was created to provide compensation benefits for injuries associated with countermeasures deployed to combat a public health emergency or security threat. While the U.S. certainly confronted serious public health threats before, the COVID-19 pandemic was a once-in-a-generation event that brought unprecedented challenges to many public health institutions and systems. Unfortunately, CICIP appears to not be designed to handle compensation for a countermeasure which was as widely distributed as the COVID-19 vaccine.

As of August 1, 2024, the total number of claims ever filed to CICIP is 13,920, and COVID-19 claims account for 13,356—more than 97 percent of the total. On February 15, 2024, CDR Reed Grimes testified about the challenges facing the program. CDR Grimes specifically noted this immense uptick in caseload in his opening statement.

CDR George Reed Grimes (February 15, 2024)

¹⁴³⁰ Cody Flint Letter (in possession of the Select Subcommittee)

¹⁴³¹ A Review of the President's FY 2023 Funding Request and Budget Justification for the Dep't of Health and Human Services: Hearing Before U.S. Senate Comm. on Appropriations, 117th Cong. 2 (May 4, 2022).

¹⁴³² Cody Flint Letter (in possession of Select Subcommittee)

¹⁴³³ *Id.*

¹⁴³⁴ *Comparison of Countermeasures Injury Compensation Program (CICIP) to the National Vaccine Injury Compensation Program (VICP)*, HEALTH RESOURCES & SERVICES ADMIN.

¹⁴³⁵ Renée Gentry written testimony, VAERS Part II hearing, March 21, 2024.

While injuries are rare and these claims represent a small fraction of the approximately 676 million total COVID-19 vaccines that have been administered in the United States, the current caseload is of a different order than the previous volume of claims in the Program given the scale of the utilization of COVID-19 covered countermeasures.¹⁴³⁶

Congresswoman Miller-Meeks questioned CDR Grimes about the cause of the backlog. CDR Grimes testified that the backlog was because the CICP only had four staff when the pandemic began.

CDR George Reed Grimes (February 15, 2024)

Q. I understand that there's a current backlog of claims in CICP by about more than 10,000. Why is there a backlog of claims for the COVID-19 vaccines?

A. Thank you for that question. So, at the beginning of the COVID-19 pandemic, we had not had a direct appropriation with the CICP. We also had only four staff. When we received our first direct appropriation in Fiscal Year 2022, we were able to ramp up quickly, and now we have over 35 staff who are assisting to adjudicate claims.¹⁴³⁷

HRSA's updated figures indicate that as of August 1, 2024, there are 10,226 claims pending or under review.¹⁴³⁸ With the staff increased to 35, CDR Grimes testified that the number of claims resolved each month increased from an average of zero per month to an average of more than 90 per month:

CDR George Reed Grimes (February 15, 2024)

We've also implemented other key process improvements to resolve claims at a faster rate. In 2023, we averaged more than 90 claims resolved each month, which is up from zero per month the year before I started in this role.¹⁴³⁹

However, even with this increased rate, the current backlog would take nearly a decade to eliminate without accounting for any new claims.¹⁴⁴⁰ It therefore appears that more must be done to streamline the process to ensure timely decisions. CDR Grimes additionally testified about CICP's efforts to establish an Injury Table for COVID-19 vaccines, which he argued would allow for a streamlined claims review process:

¹⁴³⁶ Commander Grimes written testimony, VAERS Part 1 hearing, February 15, 2024.

¹⁴³⁷ VAERS Part 1 hearing, February 15, 2024, page 35 of transcript

¹⁴³⁸ *Countermeasures Injury Compensation Program (CICP) Data: Aggregate*, HEALTH RESOURCES & SERVICES ADMINISTRATION (Nov. 1, 2024) available at <https://www.hrsa.gov/cicp/cicp-data>.

¹⁴³⁹ VAERS Part 1 hearing, February 15, 2024, Grimes opening statement, page 9 of transcript

¹⁴⁴⁰ 10,226 claims pending review / 90 per month = 113.6 months (Staff math)

CDR George Reed Grimes (February 15, 2024)

The CICP is also in the process of establishing an injury table for COVID-19 vaccine injuries that are presumed to be directly caused by a covered countermeasure. In order to establish this table, HHS must meet the high evidence standards set by Congress. The injury table is another tool that will allow us to streamline the claims review process and more expeditiously address requests.¹⁴⁴¹

This will be an important step toward improving efficiency. For claimants whose injuries appear on the injury table and were sustained within the relevant time window, CICP will automatically assume the injury was the direct result of the countermeasure.¹⁴⁴² Whereas, for non-table injuries, the claimant must prove that the injury was a “direct result” of the countermeasure, based on “compelling, reliable, medical and scientific evidence.”¹⁴⁴³ Thus, non-table injuries are significantly more complicated and time consuming to adjudicate, and act as yet another barrier to compensation for claimants.

As of September 12, 2024, it appears that HRSA has not established an injury table for COVID-19 countermeasures.¹⁴⁴⁴ Yet, since at least June 2021, the federal government acknowledges some conditions, such as Myocarditis, as known side effects of COVID-19 vaccines.¹⁴⁴⁵

FINDING: A Robust and Transparent Vaccine Injury Compensation Program Is Necessary for Promoting Trust in Vaccines.

Regardless of any claims that COVID-19 vaccines are particularly dangerous, it appears that the federal government mandated them without an adequate system in place to adjudicate the inevitable injuries they cause. This may have significant effects on the trust of the public and damage confidence in vaccines. A June 2022 *Politico* article discussed how efforts in Congress to reform the “overwhelmed” system had failed so far and highlighted the fact that this could fuel vaccine hesitancy.¹⁴⁴⁶ According to Dr. Renée Gentry, “the cost of [CICP’s] failing will be like throwing kerosene on the antivax fire.”¹⁴⁴⁷

Dr. Woodcock agreed that adequate vaccine injury compensation is important, particularly in promoting confidence in vaccines:

¹⁴⁴¹ Assessing America’s Vaccine Safety Systems, Part 1, February 15, 2024, transcript page 13, Grimes opening statement

¹⁴⁴² Kevin J. Hickey, et al., Cong. Research Servs., R46982, Compensation for COVID-19 Vaccine Injuries (Updated Mar. 31, 2023).

¹⁴⁴³ *Id.*

¹⁴⁴⁴ CICP aggregate data

¹⁴⁴⁵ Berkeley Lovelace, Jr., *CDC safety group says there’s a likely link between rare heart inflammation in young people after Covid shot*, CNBC (June 23, 2021).

¹⁴⁴⁶ Lauren Gardner, *Vaccine injury compensation programs overwhelmed as congressional reform languishes*, POLITICO, (June 1, 2022).

¹⁴⁴⁷ *Id.*

Dr. Janet Woodcock (May 13, 2024)

Q. [D]o you agree that adequate and comprehensive compensation for individuals who experience rare but serious adverse events relating to vaccines is an important element of promoting confidence in vaccines?

A. I agree with that.

Q. Would you care to elaborate on why that is?

A. Because any medical intervention will cause some harm as well as some as well as major benefit. So the statutes say safe and effective, but safe really means relative to the magnitude of the benefit, not without any harm. So people who are taking vaccines are not only protecting themselves but doing it to protect others, and so forth. And my understanding is the Vaccine Incentive Compensation Act was passed in order to recognize that people can be harmed and to adequately compensate them and protect them.¹⁴⁴⁸

On March 21, 2024, Dr. Gentry testified that the success of America's immunization programs relies on public confidence in vaccines.

Dr. Renee Gentry (March 21, 2024)

A critical part of vaccine confidence is ensuring that those rare individuals who are injured by vaccines have a reasonable and effective forum in which to make their claims.¹⁴⁴⁹

To properly prepare for a future pandemic, it is critical that the federal government and public health officials foster trust in vaccines. No matter how safe a vaccine may be, trust cannot be adequately fostered without efficient and transparent compensation systems. Therefore, it appears that significant reform may be necessary.

FINDING: Debating or Discussing Vaccine Injury Compensation Is Not “Anti-Vax,” and Implications Otherwise Are Counterproductive to Protecting Public Health.

It is paradoxical to imply that the vaccine injured are “anti-vax” since a person must be vaccinated to experience a serious adverse event. This fact was well defined by Dr. Gentry:

Dr. Renee Gentry (March 21, 2024)

¹⁴⁴⁸ Woodcock TI, *supra* note 1171, at 127-128.

¹⁴⁴⁹ Assessing America's Vaccine Safety Systems, Part II, March 21, 2024, Gentry opening statement

[I]t is critical to distinguish the vaccine injured from the anti-vax. All of my clients were vaccinated. They suffered real and often catastrophic injuries that are supported by medical and scientific literature and expert opinion.¹⁴⁵⁰

Dr. Gentry also argued that it is counterproductive to their goals for pro-vaccine advocates to use such pejoratives when discussing vaccine injury compensation, as it bolsters vaccine hesitancy.

Dr. Renee Gentry (March 21, 2024)

The well-meaning, the often dismissive and critical comments of the pro-vaccine side directed at those individuals asserting vaccine injury also creates and bolsters vaccine hesitancy in those individuals who were previously vaccinated and are pro-vaccine. The vaccine injured that I and my colleagues represent are not anti-vax.¹⁴⁵¹

This divisive language was a critical misstep of the COVID-19 vaccination campaign which alienated and dismissed people who had experienced rare but life-altering adverse reactions to the vaccine.

¹⁴⁵⁰ Assessing America's Vaccine Safety Systems, Part II, March 21, 2024, Gentry opening statement

¹⁴⁵¹ Assessing America's Vaccine Safety Systems, Part II, March 21, 2024, Gentry opening statement

VIII. The Erosion of the Doctor-Patient Relationship During the COVID-19 Pandemic

Available data show the relationship between a physician and their patient is a key element to providing high quality care.¹⁴⁵² A doctor knowing their patient and the nuance of the patient's medical history has extraordinary value. It allows the doctor to make informed decisions about patient care and more accurate diagnoses. This relationship is also essential in ensuring mutual trust and respect. Similarly, doctors must have the ability to seize upon the value of this relationship and make critical decisions, without outside interference.

In a 2006 legal opinion, an Illinois court defined the doctor-patient relationship as “a consensual relationship in which the patient knowingly seeks the physician’s assistance and in which the physician knowingly accepts the person as a patient.”¹⁴⁵³ However, this legalistic definition does little to properly explain what is so powerful and important about this dynamic. According to the American Medical Association [hereinafter “AMA”] code of ethics, this relationship is described as:

The practice of medicine, and its embodiment in the clinical encounter between a patient and a physician, is fundamentally a moral activity that arises from the imperative to care for patients and to alleviate suffering. The relationship between a patient and a physician is based on trust, which gives rise to physicians’ ethical responsibility to place patients’ welfare above the physician’s own self-interest or obligations to others, to use sound medical judgment on patients’ behalf, and to advocate for their patients’ welfare.¹⁴⁵⁴

A paper published in 2015 titled *Impact of the Doctor-Patient Relationship* identified the four elements that form the doctor-patient relationship as trust, knowledge, regard, and loyalty.¹⁴⁵⁵ The paper notes some of the powerful benefits a strong relationship can yield, including that “a physician’s knowledge of the patient’s ailments and emotional state is associated positively with whether or not those physical ailments resolve.” The paper’s conclusion was as follows:

As our vignettes intended to illustrate, the doctor-patient relationship is a powerful part of a doctor’s visit and can alter health outcomes for patients. Therefore, it is important for physicians to recognize when the relationship is challenged or failing. If the relationship is challenged or failing, physicians should be able to recognize the causes for the disruption in the relationship and implement solutions to improve care.¹⁴⁵⁶

Since a doctor’s direct relationship with their patient is such a significant aspect of health care delivery, it is therefore important that health care policies and systems prioritize and

¹⁴⁵² R. Henry Olaisen, et al., *Assessing the Longitudinal Impact of Physician-Patient Relationship on Functional Health*, ANNALS OF FAMILY MEDICINE (Sept. 18, 2020).

¹⁴⁵³ *QT, Inc. v. Jacksonville*, No. 05 C 6387 (N.D. Ill. May 15, 2006).

¹⁴⁵⁴ American Medical Association, Code of Medical Ethics, Opinion 1.1.1, Patient-Physician Relationships.

¹⁴⁵⁵ Fallon E. Chipidza, et al., *Impact of the Doctor-Patient Relationship*, PUBMED CENTRAL (Oct. 22, 2015).

¹⁴⁵⁶ *Id.*

preserve this relationship. As the American health care system continues to evolve to confront new challenges and utilize new technologies, it is ever important that we consider the human-to-human element and its direct link with positive health outcomes. This was a component of the global response to COVID-19 that was tragically ignored in favor of short-sighted one-size-fits-all solutions.

FINDING: Pandemic-Era Policy Often Disregarded or Outright Violated the Sanctity of the Doctor-Patient Relationship.

Some of the most consequential decisions of the COVID-19 pandemic were surrounding COVID-19 vaccines. Unfortunately, government policy on this front violated the sanctity of the doctor-patient relationship and may have permanent consequences. One glaring example is the fact that hundreds of millions of doses of COVID-19 vaccines were distributed under a regulatory arrangement that did not ensure the same standards of informed consent that fully approved drugs are subject to.

Generally, informed consent is “the process in which a health care provider educates a patient about the risks, benefits, and alternatives of a given procedure or intervention.”¹⁴⁵⁷ The American Medical Association (AMA) states “the process of informed consent occurs when communication between a patient and a physician results in the patient’s authorization or agreement to undergo a specific medical intervention.”¹⁴⁵⁸ The FDA’s guidance documents indicate that informed consent “is not required for administration or use of an EUA product.”¹⁴⁵⁹ During a transcribed interview conducted by Select Subcommittee staff, Dr. Woodcock readily admitted this fact:

Dr. Janet Woodcock (May 13, 2024)

Q. Does an EUA necessitate informed consent from individuals?

A. My understanding is it does not.¹⁴⁶⁰

Dr. Woodcock testified that instead of the typical legal documents, there were “information sheets” given out for COVID-19 vaccines and therapeutics which “spelled out...what the parameters were.”¹⁴⁶¹ The CDC produces Vaccine Information Statements [hereinafter “VIS”] which Federal law requires healthcare staff provide to a patient, parent, or legal representative before each dose of certain vaccines.¹⁴⁶² Because COVID-19 vaccines are

¹⁴⁵⁷ Parth Shah, *et al.*, *Informed Consent*, STATPEARLS (Oct. 15, 2024).

¹⁴⁵⁸ American Medical Association, Code of Medical Ethics, Opinion 2.1.1, Informed Consent.

¹⁴⁵⁹ *Guidance Document: Emergency Use Authorization of Medical Products and Related Authorities*, U.S. FOOD & DRUG ADMIN. (Jan. 2017).

¹⁴⁶⁰ Woodcock TI, *supra* note 1171, at 871-873.

¹⁴⁶¹ Woodcock TI, *supra* note 1171, at 37.

¹⁴⁶² *Vaccines & Immunizations*, Ctrs. For Disease Control and Prevention, *available at* https://www.cdc.gov/vaccines/hcp/about-vis/?CDC_AAref_Val=https://www.cdc.gov/vaccines/hcp/vis/about/facts-vis.html.

not covered under the National Childhood Vaccine Injury Act, healthcare staff are not required to provide VISs before administering COVID-19 vaccines.¹⁴⁶³

Additionally, during the pandemic, vaccines were often administered at pharmacies rather than in a doctor's office or hospital. In August 2020, HHS issued an amendment to the PREP Act declaration which permitted State-licensed pharmacists to administer vaccines under certain circumstances.¹⁴⁶⁴ A January 2023 report on trends in vaccine administration found that "across all vaccines for adults in-scope, a large majority off the administration took place at the pharmacy level compared to a non-pharmacy medical setting..."¹⁴⁶⁵ This report did not factor in locations where claims wouldn't be generated, including mass vaccination centers which administered a significant share of COVID-19 vaccine doses early in the rollout.

While these flexibilities were ostensibly put in place to increase access to vaccines, they may have also served to further erode the role of doctors in these important medical decisions. Following HHS' August 2020 amendment, representatives of the AMA publicly urged HHS to "reconsider the negative health repercussions of funneling children away from their primary care physicians and rescind this declaration."¹⁴⁶⁶

Worse, mandatory vaccination policies represented a direct assault on the doctor-patient relationship. Vaccine mandates may be the most salient example of pandemic-era policies where governments and other political entities inserted themselves in a decision that should be between each patient and their doctor. This is inherently incompatible with the definition of the doctor-patient relationship.

The COVID-19 vaccine mandates also largely ignored the notion of naturally acquired immunity. By the standard of most COVID-19 vaccine mandates, there was no exception made for those who had previously contracted the virus. The mandates also left no room for women who were pregnant or trying to get pregnant. If left up to doctors, who are familiar with their patients and their health, individual patient risk and benefit could have been much better assessed. During the Select Subcommittee's July 27, 2023 hearing on vaccine mandates, Chairman Wenstrup shared his own experience dealing with this one-size-fits-all approach:

Chairman Brad Wenstrup (July 27, 2023)

I got vaccinated, Pfizer, both doses. Six months later, I got COVID. The only reason I knew is I couldn't smell garlic salt. I was told I needed a booster to travel. I said I would like to check my T-cells and antibodies. The lab here couldn't do the T-cells. I got my antibodies. Strong number was 40. My number was 821. Should I get a booster? That is a legitimate question. I don't want a hyperimmune response.¹⁴⁶⁷

¹⁴⁶³ Id.

¹⁴⁶⁴ Notice, Third Amendment to Declaration Under the Public Readiness and Emergency Preparedness Act for Medical Countermeasures Against COVID-19, U.S. DEP'T OF HEALTH & HUMAN SERVICES (Aug. 24, 2020).

¹⁴⁶⁵ *Trends in Vaccine Administration in the United States*, THE IQVIA INSTITUTE (Jan. 13, 2023).

¹⁴⁶⁶ Andis Robeznieks, *AMA opposes HHS move to expand pharmacists' scope of practice*, AMA (Aug. 24, 2020).

¹⁴⁶⁷ Vaccine Mandates Hearing Transcript Page 42

Relatedly, some pandemic policies also created intense political pressure for doctors to toe the line and recommend COVID-19 vaccines and boosters regardless of whether they believed the evidence supported such a recommendation. Doctors saw colleagues who spoke out being ridiculed and silenced and many likely chose to keep their own dissent to themselves.¹⁴⁶⁸ Some medical boards and state governments took things even further and pursued legal and administrative action against doctors who were perceived to be purveying misinformation.¹⁴⁶⁹

Dr. Azadeh Khatibi testified during the Select Subcommittee's September 14, 2023 hearing about the rigid anti-free speech policies which were imposed on medical providers in California during the pandemic.¹⁴⁷⁰ One such example was a California law signed by Governor Gavin Newsom in August 2022 which declared it to be "unprofessional conduct" for doctors to disseminate misinformation or disinformation about COVID-19, which was state-defined as "contradicted by contemporary scientific consensus contrary to the standard of care."¹⁴⁷¹ Dr. Khatibi testified that, as a doctor in California, she believed that scientific consensus is "always behind the cutting edge" which is why doctors have historically "had liberty to contradict consensus opinion."¹⁴⁷² California eventually repealed this law in October 2023 after mounting legal pressure, however significant damage had already been done.¹⁴⁷³

Vaccine mandates were not only ineffective, but they were also harmful. As was established during the Select Subcommittee's July 2023 hearing on the subject, vaccine mandates caused significant collateral damage.¹⁴⁷⁴ A BMJ paper from May 2022 titled *The unintended consequences of COVID-19 vaccine policy: why mandates, passports and restrictions may cause more harm than good* found that COVID-19 vaccine mandates caused significant collateral damage. The paper's summary stated:

Our analysis strongly suggests that mandatory COVID-19 vaccine policies have had damaging effects on public trust, vaccine confidence, political polarization, human rights, inequities and social wellbeing. We question the effectiveness and consequences of coercive vaccination policy in pandemic response and urge the public health community and policymakers to return to non-discriminatory, trust-based public health approaches.¹⁴⁷⁵

¹⁴⁶⁸ Timothy Bella, *A vaccine scientist's discredited claims have bolstered a movement of misinformation*, THE WASH. POST (Jan. 24, 2022).

¹⁴⁶⁹ Alicia Ault, *ABIM Revokes Certification for Two Physicians Accused of COVID Misinformation*, MEDSCAPE (Aug. 15, 2024).

¹⁴⁷⁰ Oh Doctor, Where Art Thou? Pandemic Erosion of the Doctor-Patient Relationship: Hearing Before Select Subcomm. on the Coronavirus Pandemic, 118th Cong. 1 (Sept. 14, 2023).

¹⁴⁷¹ Steven Lee Myers, *California Approves Bill to Punish Doctors Who Spread False Information*, THE N.Y. TIMES (Aug. 29, 2022).

¹⁴⁷² Oh Doctor, Where Art Thou? *Supra* note 18.

¹⁴⁷³ Sean Salai, *California repeals COVID misinformation law, bowing to legal pressure*, The Wash. Times (Oct. 2, 2023).

¹⁴⁷⁴ Because I Said So, *supra* note 1348.

¹⁴⁷⁵ Bardosh, *The unintended consequences of COVID-19 vaccine policy*, *supra* note 891.

The paper also asserts that these vaccine mandates may “be in tension with” bioethical principles:

Denying individuals education, livelihoods, medical care or social life unless they get vaccinated—especially in light of the limitation with current vaccines—is arguably in tension with constitutional and bioethical principles, especially in liberal democracies. While public support consolidated behind these policies in many countries, we should acknowledge that ethical frameworks were designed to ensure that rights and liberties are respected even during public health emergencies.¹⁴⁷⁶

During the Select Subcommittee’s hearing on September 14, 2024, Dr. Jeffrey Singer—a surgeon who has written about medical ethics—testified that vaccine mandates hindered trust and undermined the doctor-patient relationship:

Dr. Jeffrey Singer (September 14, 2024)

Q. Finally, very simply, do you feel that vaccine mandates facilitate fracturing the patient-doctor relationship?

A. I think mandating does because, first of all, it’s a natural tendency for people to recoil when they’re mandated even if what’s being mandated is actually a good idea. People don’t like being told they have to do things. And so, when you have somebody who it’s important that they have a very trusting relationship, the doctor and the patient, and the patient understands that they’re being compelled to do something, I think it just undermines the relationship of trust between the doctor and the patient.¹⁴⁷⁷

Dr. Singer published a study titled “A Hippocratic Oath for a Free Society,” wherein he argues that doctors must always “prioritize the autonomy and rights of individual patients” and should take an oath which declares:

I will respect the crucial scientific advances in medicine but will always question the assumptions my profession has inherited and will judge them in the light of the latest evidence. I will gladly share any knowledge I have gleaned from years of research, study, and clinical experience with health professionals in all disciplines. I will respect my patients’ autonomy, thoroughly explain all the diagnostic possibilities and therapeutic options as I understand them, offer my best opinion and advice from among these options, and accept their decisions.¹⁴⁷⁸

¹⁴⁷⁶ *Id.*

¹⁴⁷⁷ Because I Said So, *supra* note 1348, at 35.

¹⁴⁷⁸ *Id.* (statement by Jeffrey A. Singer, M.D.).

When asked to expand on his study on the Hippocratic Oath, Dr. Singer testified that he did not believe the government had the right to compel vaccination:

Dr. Jeffrey Singer (September 14, 2024)

- Q. Dr. Singer, you have written about the ethical questions of COVID-19 vaccine mandates, and you have often said, and I'm quoting at this point, as a medical doctor, I enthusiastically endorse COVID-19 vaccine, and you personally had been vaccinated and will encourage others to be vaccinated. But you continued brilliantly by saying, but I will use persuasion, not coercion. Your words. Dr. Singer, do you believe that vaccine mandates without exemption are incompatible with the Hippocratic oath or the tenets of the basic doctor-patient relationship?
- A. Representative Joyce, Dr. Joyce, yes, I do. I think it's actually you have no right to force someone to be vaccinated. Obviously, I believe that the vaccines saved hundreds of thousands of lives, and I got vaccinated. I got the first two shots, and I got the booster shortly thereafter, and I'm glad I did. But my role is to recommend to people, not to force people, not to compel people. In addition, there are some people who have very good reasons to not be vaccinated. They may have allergies. They may have already had COVID, and they have natural immunity, and they are concerned about getting a reaction to a vaccine that is of a new technology and hadn't been subjected to clinical trials because there was an emergency use authorization. These are not unreasonable concerns. I need to respect those concerns.¹⁴⁷⁹

Overall, Americans would be better served by a health care system that encouraged patients to seek out advice of a trusted doctor regarding their individual medical history and the risks and benefits of being vaccinated. This relationship between doctor and patient is a crucial cornerstone of overall trust in medicine and could have helped prevent some of the anti-science rhetoric and misinformation that erupted during the COVID-19 pandemic.

FINDING: The Use of Off-Label Prescriptions Was Unjustly Demonized and Further Eroded the Doctor-Patient Relationship.

The COVID-19 pandemic deepened political and social divides and opened new wounds in the public discourse. Unfortunately, health care was no exception. The onslaught of controversy, polarization, shame, and censorship damaged the profession as well as the health care system more broadly. One extremely common and important tool at the disposal of doctors is prescribing an FDA-approved medication for a use which the drug is not specifically approved

¹⁴⁷⁹ *Id.* at 34.